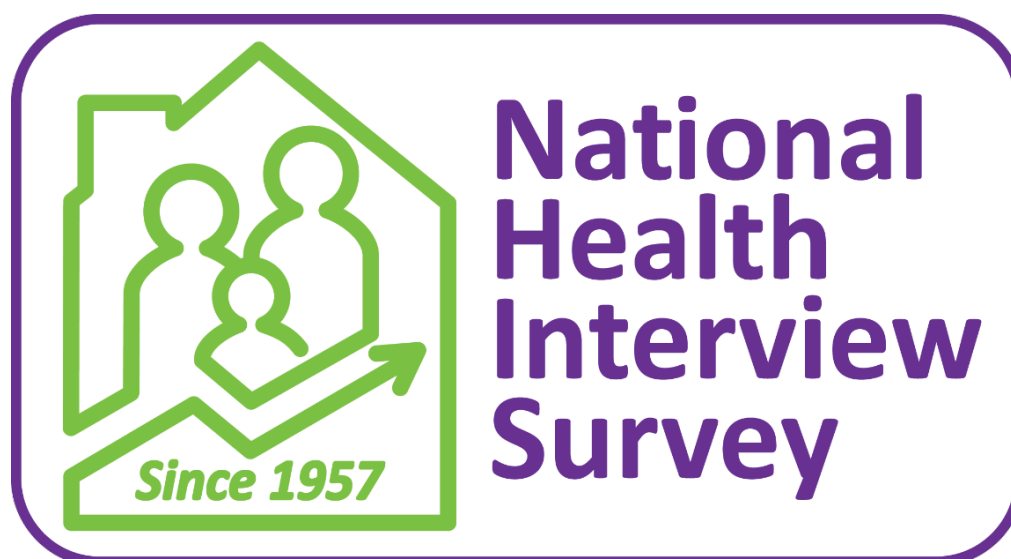




2025 SURVEY DESCRIPTION

Division of Health Interview Statistics
National Center for Health Statistics
Hyattsville, Maryland

Centers for Disease Control and Prevention
U.S. Department of Health and Human Services

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NCHS Website

Data users can obtain the latest information about the National Health Interview Survey (NHIS) by periodically checking our website: <https://www.cdc.gov/nchs/nhis.htm>

The website features downloadable data and documentation for the 2025 NHIS and previous years, as well as important information about any modifications or updates to the data or documentation. Published reports from previous years' surveys are also available, as are updates about future surveys and datasets. Data files and documentation can be found at: <https://www.cdc.gov/nchs/nhis/documentation/index.html>

NHIS Electronic Mail List

Data users are encouraged to join the NHIS Listserv, an electronic mailing list. The Listserv is made up of over 3,200 NHIS data users located around the world who receive news about NHIS surveys (e.g., new releases of data or modifications to existing data), and related products. To join, go to https://www.cdc.gov/nchs/products/nchs_listservs.htm, and select "National Health Interview Survey (NHIS) Researchers" as one of your options, or select "subscribe now" at the bottom of the NHIS annual release page.

Questions about NHIS?

The staff of the Division of Health Interview Statistics at the National Center for Health Statistics respond to data users' questions about NHIS. Users may e-mail us at nhislist@cdc.gov. A response may take 1-2 business days.

Guidelines for Citation of Data Source

With the goal of mutual benefit, the National Center for Health Statistics (NCHS) requests that recipients of NHIS data files cooperate in certain actions related to their use.

Any published material derived from the 2025 NHIS data should acknowledge “National Center for Health Statistics, National Health Interview Survey” as the original source. The full spelling of the source without the use of acronyms is preferred.

The suggested citation to appear at the bottom of all tables and graphs is as follows:

Data Source: National Center for Health Statistics, National Health Interview Survey, 2025.

In a bibliography, the suggested citation for this document is:

National Center for Health Statistics. National Health Interview Survey, 2025 survey description. 2026. Available from:

https://ftp.cdc.gov/pub/Health_Statistics/NCHS/Dataset_Documentation/NHIS/2025/srvydesc-508.pdf

The suggested citation for 2025 NHIS survey data and other documentation is:

National Center for Health Statistics. National Health Interview Survey, 2025. Public-use data file and documentation. <https://www.cdc.gov/nchs/nhis/documentation/index.html>. 2026.

The published material should also include a disclaimer that credits the author’s analyses, interpretations, and conclusions to the author (recipient of the data file) and not to NCHS, which is responsible only for the initial data. Users who wish to publish a technical description of the data should make a reasonable effort to ensure that the description is consistent with that published by NCHS.

NHIS questionnaires are in the public domain and no permission is required to use them. Citation as to source, however, is appreciated.

Information on how to cite NCHS publications and electronic media is available at:

<https://www.cdc.gov/nchs/products/citations.htm>.

Data User Agreement

Please Read Carefully Before Using the National Health Interview Survey

The National Health Interview Survey (NHIS) is conducted by the National Center for Health Statistics (NCHS), Centers for Disease Control and Prevention (CDC).

NCHS, CDC conducts statistical and epidemiological activities under the authority granted by the Public Health Service Act (42 U.S.C. § 242k). NCHS survey data such as NHIS are protected by Federal confidentiality laws including Section 308(d) Public Health Service Act [42 U.S.C. 242m(d)] and the Confidential Information Protection and Statistical Efficiency Act (CIPSEA) of 2018 (44 U.S.C. 3561-3583). These confidentiality laws state the data collected by NCHS may be used only for statistical reporting and analysis. Any effort to determine the identity of individuals and establishments violates the assurances of confidentiality provided by federal law.

Terms and Conditions

NCHS does all it can to assure that the identity of individuals and establishments cannot be disclosed. All direct identifiers, as well as any characteristics that might lead to identification, are omitted from the dataset. Any intentional identification or disclosure of an individual or establishment violates the assurances of confidentiality given to the providers of the information. Therefore, users will:

- Use the data in this dataset for statistical reporting and analysis only.
- Make no attempt to learn the identity of any person or establishment included in these data.
- Not link this dataset with individually identifiable data from other NCHS or non-NCHS datasets.
- Not engage in any efforts to assess disclosure methodologies applied to protect individuals and establishments or any research on methods of re-identification of individuals and establishments.

By using these data, you signify your agreement to comply with the above-stated statutorily based requirements.

Sanctions for Violating NCHS Data Use Agreement

Anyone willfully disclosing any information that could identify a person or establishment in any manner to a person or agency not entitled to receive it, shall be guilty of a class E felony and imprisoned for not more than 5 years, or fined not more than \$250,000, or both.

What's New in 2025?

- Starting in 2025, a modified sample design based on the 2020 Census that uses a stratified, two-stage sample was implemented. Primary sampling units (PSUs) were selected during the first stage with household units drawn annually at the second stage. This design incorporates the latest decennial census population counts and uses enhanced sample design requirements and improvements. Notably, it provides a better balance between interviewer or field representative (FR) travel costs and the geographic clustering of sampled households, while enabling the production of more reliable state-level estimates.
- Analysts interested in combining or pooling public-use NHIS data files from 2025 and 2024 (or prior years since 2019) do not need to account for the different sample design in variance estimation. Analysts using the restricted variance estimation variables through the Research Data Center should differentiate the strata values between 2025 and prior years. Options include adding or multiplying one of the annual strata by a constant value to make the strata values distinct between 2025 and the prior years, or by specifying two designs when declaring the variance variables in the analysis.
- Due to the lapse in appropriations between October 1 and November 12 of 2025, data collection by the U.S. Census Bureau was paused between October 1 and November 16, with NHIS data collection restarting on November 18. This pause in operations resulted in a reduction in the number of interviews completed in 2025 and slight delays in the processing of data that were collected. Specific data collection and data file impacts are described below:
 - The interview period for the October, November, and December sample shifted. FRs were instructed to work October cases from November 18 to December 7, November cases from December 1 to January 4, and December case from January 1 to January 31, 2026. This resulted in a slight overlap in October and November sample interview periods.
 - Data collected from January 1 to January 31, 2026, are excluded from 2025 annual file.
 - The month of interview (INTV_MON2) and quarter of interview (INTV_QRT2) variables have been revised to reflect the shift and overlap in interview periods. October and November sample cases are coded as “99” in INTV_MON2 and coded as “9” in INTV_QRT2.
 - In addition to data collection changes due to a lapse in appropriations, data collection was also impacted by limitations on FRs' travel, hiring, and retention in 2025.
- Continuing since 2019, the NHIS questionnaire includes four types of content: 1) Annual core is content that is included in the NHIS every year; 2) Rotating core is scheduled to appear on a periodic basis of every other year, one out of every three years, or two out of every three years; 3) Sponsored content includes questions that are funded by other agencies; and 4) Emerging content is content that is sponsored by NCHS on new health topics. A visualization that summarizes 2019–2027 NHIS content by year is available for the Sample Adult (<https://www.cdc.gov/nchs/data/nhis/Sample-Adult-Questionnaire.pdf>) and the Sample Child (<https://www.cdc.gov/nchs/data/nhis/Sample-Child-Questionnaire.pdf>).
- Annual core content remains largely the same in 2025 except:
 - The “read if necessary” text to the question “Is everyone in this household a member of the same family?” in the FAM section of the Roster was reworded to make it clear that respondents should include unmarried couples living together as family.
 - Six new questions were added to the sample adult and sample child health insurance section (INS) in Quarter 4 to capture changes to a person’s health care coverage over the past 12 months. The six questions ask if a person had health insurance purchased through an employer, exchange or marketplace such as Healthcare.gov, or had Medicaid, medical assistance, or coverage through the Children’s Health Insurance Program in the past 12 months. Data for these questions are only available

through the [Research Data Center](#).

- Additional information on the 2025 rotating, sponsored and emerging content is described below:

Sample Adult Rotating Core
Aspirin use*, chronic pain, dental care, hysterectomy, physical and other therapeutic care, and preventive screening (clinical checks for blood pressure, cholesterol, and blood glucose, and screening for breast, cervical, colorectal** cancer)—All previously fielded in 2023. GAD-7 anxiety scale and PHQ-8 depression scale— All previously fielded in 2022. <i>*In 2025, only two of the four low-dose aspirin questions fielded in the previous rotations were included.</i> <i>**In 2025, the age of Sample Adults asked about colorectal cancer screening was lowered to 35 years from age 40 years used in the previous rotations.</i>
Sample Child Rotating Core
Physical and other therapeutic care and stressful life events — All previously fielded in 2023. Strength and difficulties questionnaire (SDQ) — Previously fielded in 2022.
Sample Adult Sponsored Content
Age of disability onset, attention-deficit/hyperactivity disorder, caregiving, chronic pain management, diabetes distress, epilepsy, immunization (COVID-19, respiratory syncytial virus (RSV), shingles, and tetanus, diphtheria, and acellular pertussis (Tdap)), insulin use, life satisfaction, Long COVID, social support and loneliness, menopausal symptoms, non-cigarette tobacco products, non-insulin injectable medications to lower blood sugar or lose weight, pregnancy in the past 12 months that ended in live birth, preventive screening (colon, prostate, breast, and cervical cancer), social connectedness and isolation, traumatic brain injury, voice, swallowing, speech, and language communication disorders, and whole person health.
Sample Child Sponsored Content
COVID-19 vaccination, Long COVID, traumatic brain injury, and voice, swallowing, speech, and language communication disorders.
Sample Adult Emerging Content
Civic engagement, internet access and health information technology, language spoken at home, social and emotional parenting support, and volunteer activities.

About NHIS

NHIS is the principal source of information on the health of the civilian noninstitutionalized population of the United States and is one of the major data collection programs of the NCHS. The National Health Survey Act of 1956 provided for a continuing survey and special studies to secure accurate and current statistical information on the amount, distribution, and effects of illness and disability in the United States and the services rendered for or because of such conditions. The survey referred to in the Act, now called the National Health Interview Survey, was initiated in July 1957. Since 1960, the survey has been conducted by NCHS, which was formed when the National Health Survey and the National Vital Statistics Division were combined.

The main objective of the NHIS is to monitor the health of the United States population through the collection and analysis of data on a broad range of health topics. A major strength of this survey lies in the ability to categorize these health characteristics by many demographic and socioeconomic characteristics.

NHIS data are used widely throughout the Department of Health and Human Services (HHS) to monitor trends in illness and disability and to track progress toward achieving national health objectives. The data are also used by the public health research community for epidemiologic and policy analysis of such timely issues as characterizing those with various health problems, determining barriers to accessing and using appropriate health care, and evaluating Federal health programs.

Since 1957, the content of the survey has been updated about every 10–15 years to incorporate advances in survey methodology and coverage of health topics. In January 2019, NHIS launched a redesigned content and structure that differs from the 1997–2018 NHIS.

Overview of 2025 Survey Methods

NHIS is a nationally representative household survey of the U.S. civilian noninstitutionalized population. It is conducted continuously throughout the year by the National Center for Health Statistics (NCHS). However, due to a lapse in federal appropriations, NHIS data were not collected from October 1 to November 16, 2025. Interviews are typically initiated in respondents' homes, but follow-ups to complete interviews may be conducted over the telephone. In 2025, 53.1% of the Sample Adult interviews and 54.3% of the Sample Child interviews were conducted at least partially by telephone.

Information about the Sample Adult is self-reported, unless physically or mentally unable to do so (and then a knowledgeable proxy can answer for the Sample Adult). Information about the Sample Child is collected from a parent or adult who is knowledgeable about and responsible for the health care of the Sample Child. In 2025, there were 24,215 Sample Adult interviews and 6,049 Sample Child interviews. Starting in 2025 (and available retrospectively from 2019-2024), weighted response rates are reported. In 2025, the weighted final Sample Adult response rate was 41.3% and the weighted final Sample Child response rate was 42.0%. The response rates in 2025 were impacted by the cessation of data collection due to the lapse in federal appropriations, the overall challenges hiring and retaining field representatives (FR), and the moratorium on federal travel that prevented FRs' travel to areas with low field coverage.

NHIS includes annual content that appears on the survey every year. The survey also includes rotating core content, sponsored content, and emerging content that appears periodically. Visit <https://www.cdc.gov/nchs/nhis/about/2019-questionnaire-redesign.html> for a description of content in any given year.

For more information about NHIS, visit: <https://www.cdc.gov/nchs/nhis/index.html>. An itemized checklist is also available that summarizes the information necessary for researchers to implement the Preferred Reporting Items for Complex Sample Survey Analysis (PRICSSA) reporting guidelines (Available from: <https://www.cdc.gov/nchs/nhis/documentation/2025-nhis.html>).

NHIS Methods

I. Sample Design

NHIS is a cross-sectional household interview survey. The target population for the NHIS is the civilian noninstitutionalized population residing within the 50 states and the District of Columbia at the time of the interview. The NHIS universe includes residents of households and noninstitutional group quarters (e.g., homeless shelters, rooming houses, and group homes). Persons residing temporarily in student dormitories or temporary housing are sampled within the households where they usually reside (e.g., their parents' household). Persons excluded from the universe are those with no fixed household address (e.g., homeless and transient persons not residing in shelters), active duty military personnel and civilians living on military bases, persons in long-term care institutions (e.g., nursing homes for the elderly, hospitals for the chronically ill or physically or intellectually disabled, and wards for abused or neglected children), persons in correctional facilities (e.g., prisons or jails, juvenile detention centers, and halfway houses), and U.S. nationals living in foreign countries. While active-duty Armed Forces personnel are not sampled for inclusion in the survey, any civilians residing with Armed Forces personnel in non-military housing are eligible to be sampled.

NHIS is typically initiated in a face-to-face interview format. However, the costs of interviewing a large simple random sample of households and noninstitutional group quarters would be prohibitive, and randomly sampled dwelling units would be too dispersed throughout the nation for cost-effective interviewing. To keep survey operations manageable, cost-effective, and timely, the NHIS uses geographically clustered sampling techniques to select the sample of dwelling units for the NHIS. The sample is designed in such a way that each month's sample is nationally representative. Data collection on the NHIS is continuous, i.e., from January to December each year.

Every ten years, the sample design for the NHIS is redrawn to align with updated population counts from the most recent decennial census. This is also an opportunity to change sampling requirements and to incorporate design improvements. The 2025 NHIS sample redesign, which aligns with the 2020 decennial census, was implemented in 2025 and was driven by the need to have a nationally representative sample providing coverage in all 50 states and the District of Columbia (DC). Additionally, a new requirement was to include the ability to generate state-level estimates based on three years of pooled data. The redesigned sample retained the ability to pool data over time. To meet these requirements, the 2025 NHIS sample design uses a stratified, two-stage sample. The first stage PSUs were selected at the start of the design, while the second stage household units are selected on an annual basis.

The process of defining PSUs involves combining all the counties and county-equivalents in the nation into over 1500 PSUs. This nationwide universe of PSUs is the first-stage sampling frame used when selecting sample PSUs for the design. The 2025 NHIS design implements a novel method of cycling non-self-representing (NSR) PSUs in which sample NSR PSUs are cycled in and out of sample during the ten-year design period. Cycling ensures more geographic dispersion and degrees of freedom for each of the 50 states and D.C. when pooling three years of data. NSR PSUs remain in sample for two or three consecutive years before rotating out. The NSR PSUs staying in sample for at least two years helps to ease the burden on field operations in managing staff. Highly populated PSUs are designated as self-representing (SR) and are in-sample every year of the design. The sample size is set to annually achieve 30,000 complete adult interviews. While the baseline goal is to achieve 30,000 complete adult interviews each year, budget constraints can require a reduction of this goal in a particular year. This was the case in 2025, the first year of this sample design. The minimum sample size in each state is set to achieve the estimated number of complete adult interviews necessary to ensure that an estimate of approximately 10 percent has a 95 percent confidence interval width no larger than 1.3 times its proportion (assuming a design effect of three).

The current design also marks the beginning of annual sampling. In previous designs, the bulk of the sample was selected at the start of the ten-year period, with new growth added over time. Beginning with units interviewed in the field in 2025, all housing units are now selected annually in the within-PSU sampling processes. Similar to the 2010 NHIS sample design, the majority of the NHIS sampling frame is based on an address list provided by a vendor. This portion of the frame is referred to as the Unit Frame. The remainder of the NHIS sample frame is referred to as the Area Frame and is composed of the results of listings carried out by FRs. For the 2025 NHIS sample, 5.5% of the sampled housing units were from the area frame and 94.5% of the sampled housing unit were from the unit frame. A report on the new NHIS sample design starting in 2025 is forthcoming.

II. Interviewing Procedures

Data Collection

The U.S. Census Bureau, under a contractual agreement, is the data collection agent for the National Health Interview Survey. Typically, NHIS data are collected continuously throughout the year by Census FRs.

2025

Due to a lapse in federal appropriations, NHIS data were not collected from October 1 to November 16, 2025. The 2025 data collection also faced unique challenges in addition to the cessation of data collection. Restrictions in federal hiring and staff retention led to a severe shortage in FRs which impacted response rates. The moratorium on federal travel also meant that FRs could not travel to areas with inadequate numbers of FRs to complete interviews.

In 2025, 636 FRs conducted NHIS interviews nationwide. FRs are trained and directed by health survey supervisors in the U.S. Census Bureau Regional Offices. FRs are observed by supervisors periodically, and their work is monitored by the Census Bureau's PANDA system, a performance and data analysis program that provides monthly checks on response rates, completion rates, item response times, item nonresponse, telephone usage rates, and other data quality indicators. FR performance is also assessed with the Field Quality Monitoring (FQM) system, a near real-time monitoring that integrates performance metrics across all surveys worked by an FR. Metrics assessed include patterns in the rate of vacant and usual residence elsewhere, actual versus billed distance between home and personal visits, number of personal visit attempts per day, and variation in the days and times of attempts. The supervisors responsible for the NHIS are career Civil Service employees who are selected through an examination and testing process. FRs receive thorough refresher training annually and other training during the year in basic interviewing procedures and in the concepts and procedures unique to the NHIS.

Each household address selected for participation in the NHIS is mailed a letter prior to the FR's visit. The "Advance letter" is mailed one week prior to the start of the interview period (one week before the 1st of the month) with the goal that it might be fresh on people's mind when the FR makes contact the first few days of the month. This "Advance letter" contains information about the purpose of the NHIS and the amount of time the interview will require, and it assures potential respondents that participation in the NHIS is voluntary. It also informs respondents that the information they provide is protected by law and details how the information will be used. When the FR arrives at the household address, he/she provides another copy of the "Advance letter" to each respondent and obtains verbal consent for survey participation. A copy of the current "Advance letter" and other NHIS materials available for distribution by FRs in the field are available at the NHIS participants' page: <https://www.cdc.gov/nchs/nhis-participants/what-to-expect/index.html>.

The NHIS is conducted using computer-assisted personal interviewing (CAPI). The CAPI data collection method employs Blaise computer software that presents questions on computer screens to each FR. The instrument guides the FR through the questionnaire, automatically routing the FR to appropriate questions based on answers to previous questions. FRs are trained to read the questions as written in the instrument and probe if necessary. Unless noted in the introduction to the question or in the question text, “refuse” and “don’t know” responses are not read to respondent but accepted as a response to any survey question. FRs enter survey responses directly into the computer, and the CAPI program determines if the selected response is within an allowable range, checks it for consistency against some of the other data collected during the interview, and saves the responses into a survey data file. The computer contains help facilities to aid FRs in administering the CAPI questionnaire. This data collection technology reduces the time required for transferring, processing, and releasing data, and it ensures the accurate flow of the questionnaire.

Typically, face-to-face interviews are initiated in respondents’ homes, but follow-ups to complete interviews may be conducted over the telephone. A telephone interview may also be conducted when the respondent requests a telephone interview or when road conditions or travel distances would make it difficult to schedule a personal visit before the required completion date.

Due to the closure of the federal government between October 1 and November 12 of 2025, data collection by the U.S. Census Bureau was paused between October 1 and November 16. The October NHIS sample was fielded between November 18 and December 7, and the November NHIS sample was fielded between December 1 and January 4 of 2026. Data collected using the October and November samples were included in the 2025 data files. The December NHIS sample was fielded between January 1st and 31st. Data collected using the December sample was not included in the 2025 data files. Therefore, the 2025 NHIS data files contain data from 11 months of sample. Variables for month of interview and quarter of interview were revised to categorize sample months 10 and 11 as 99 and quarter 4 as 9 to differentiate these variables from previous years. Weights were adjusted to account for 11 months of sample—see Weighting section for more information

Field Results

In 2025, 53.1% of the Sample Adult interviews and 54.3% of the Sample Child interviews were conducted at least partially by telephone. This is consistent with the percentage of telephone interviews that were conducted at least partially by telephone in 2024 (53.2% for Sample Adult interviews and 54.8% for Sample Child interviews), but still considerably higher than interviews in 2019 (pre-pandemic) for both the Sample Adult (34.3%) and Sample Child (31.7%).

The Paradata data file, available with the annual data release, has information collected about the interview process.

Rostering and Respondents

For the Household Roster section of the questionnaire, any responsible household member age 18 years and older is identified to act as the “Household Respondent.” The Household Respondent provides names, age, sex, race, and ethnicity for all household members. The highest level of education completed, and active military status is asked for all adult household members age 18 years and older. In addition to collecting this basic demographic information, the Household Roster interview also identifies whether all persons in the household are members of the same or different family. Note that in a multi-family household, a single “Household

Respondent” provides household information for all families.

NHIS has consistently defined a family as an individual or a group of two or more people residing together who are related by birth, marriage, or adoption. A family additionally includes any unrelated children who are cared for by the family (such as foster children) and any unmarried cohabiting partners and their children. After the Household Roster is completed, data are collected on one adult and child per household.

A “Sample Adult” is randomly selected by the computer from each household with at least one household member age 18 years and older and is asked more detailed health related questions. The Sample Adult responds for him/herself to the questions in that section unless he/she is physically or mentally unable to do so, in which case a knowledgeable proxy may answer for the Sample Adult. Students age 18 and older living away at college, trade, or commercial schools in on-campus housing are eligible to be interviewed in the location they consider to be their usual residence, such as their parent’s or other family member’s household. Students living away at school or college in off-campus housing will not be included as members of the household, since they could be sampled at their off-campus location.

A “Sample Child” is randomly selected by the computer from each household with at least one child 17 years of age or younger. An adult respondent who was previously indicated to be knowledgeable and responsible for the Sample Child’s health is asked questions about that child. In 2025, 94.6% of the Sample Child respondents were the child’s parent, either a biological, adoptive, or stepparent, while 3.6% were a grandparent, 0.3% were an aunt or uncle, 0.6% were an adult sibling, 0.8% were another relative or other non-relative, and 0.05% were not ascertained. For each sampled household address, FRs also maintain electronic documentation about the NHIS interview process, including contact attempts, observed characteristics about the exterior of the sample unit or vicinity, and descriptive information about the interview outcome.

Confidentiality

All information collected by the NHIS that would permit identification of the individual is held strictly confidential, seen only by persons who work on the NHIS (including related studies carried out by the Public Health Service) with a need to know, and such information is not disclosed or released to anyone for any other purpose without the consent of the respondent. NCHS must adhere to Section 308(d) of the Public Health Service Act (42 U.S.C. 242m(d)), which forbids the disclosure of any information that may compromise the confidentiality promised to survey respondents. In addition, confidentiality protections are also mandated by the Privacy Act of 1974 (5 U.S.C. § 552a), and the Confidential Information Protection and Statistical Efficiency Act of 2018 (44 U.S.C. 3561-3583). In addition to the above cited laws, NCHS complies with the Federal Cybersecurity Enhancement Act of 2015 (6 U.S.C. §§ 151 and 151 note) which protects Federal information systems from cybersecurity risks by screening their networks.

Further information about data collection procedures is available in the *National Health Interview Survey: 2025 CAPI Manual for NHIS Field Representatives* available on the NHIS website, https://ftp.cdc.gov/pub/Health_Statistics/NCHS/Survey_Questionnaires/NHIS/2025/2025-NHIS-CAPI-Manual.pdf.

III. NHIS Design

In 2019, the structure and content of the NHIS questionnaire was redesigned to better meet the needs of data users, CDC, and HHS. The goals of the redesign were to reduce respondent burden by shortening the length of the questionnaire, harmonize overlapping content with other federal health surveys, establish a long-term structure of ongoing and periodic topics, and incorporate advances in survey methodology and measurement.

For more information about the 2019 Questionnaire Redesign, visit:

<https://www.cdc.gov/nchs/nhis/about/2019-questionnaire-redesign.html>. For information about the previous NHIS design conducted during 1997–2018, refer to year-specific NHIS documentation:

https://archive.cdc.gov/www_cdc.gov/nchs/nhis/1997-2018.htm

Structure of NHIS

The redesigned NHIS questionnaire consists of three modules: (1) Household Roster; (2) Sample Adult Interview; and (3) Sample Child Interview. Following a brief household rostering interview that collects basic demographics of everyone who usually lives or stays in the household and identifies which members of the household are in the same or a separate family, one “Sample Adult” age 18 years and older and one “Sample Child” age 17 years or younger (if any children live in the household) are randomly selected from each household. Information about the household members is collected from a person age 18 years and older residing at the sample address. Information about the Sample Adult is collected from the Sample Adult themselves unless they are physically or mentally unable to do so, in which case a knowledgeable proxy can answer for the Sample Adult. Information about the Sample Child is collected from a parent or adult who is knowledgeable and responsible for the health care of the Sample Child. The respondent for the Household Roster, Sample Adult and Sample Child modules may or may not be the same person.

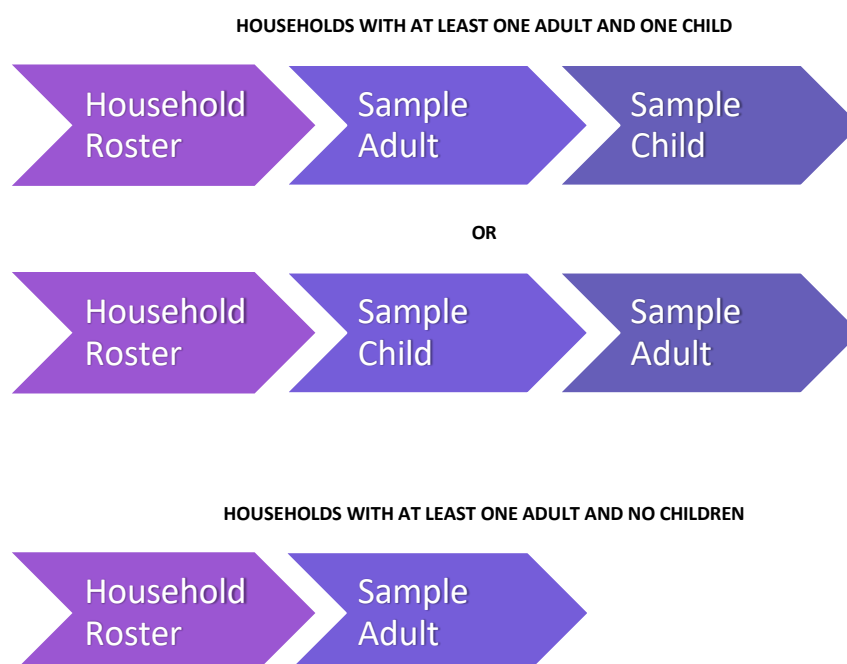
Figure 1 (on the next page) illustrates the interviewing flow of the Household Roster, Sample Adult and Sample Child interviews or modules in the NHIS. The Sample Adult and Sample Child may be part of the same family or be part of different families in the household. Family-level content (e.g., family income, food insecurity) is collected in the Sample Adult or Sample Child questionnaire module.

Content of the Household Roster

In the Household Roster, an adult (age 18 years and over) living in the household provides basic information about themselves and other people living in the household. The name, age, sex, race, and ethnicity of everyone in the household are collected. Additionally, the parents of all children are identified. The instrument then randomly selects one adult (Sample Adult) and one child (Sample Child), if any children live in the household, to be given follow-up questions. The Sample Adult is selected randomly among persons age 18 years and older living in the household, and the Sample Child is selected randomly among those age 17 years or younger. Questions are asked to determine who is in the family of the Sample Adult and Sample Child. The Sample Adult and Sample Child do not need to be in the same family. No health information is collected in this section. When the Household Roster is complete, the FRs can then proceed with the Sample Adult or Sample Child interview (if a child lives in the household).

Figure 2 (on the next page) illustrates how topics or content in the NHIS are organized by interviewing modules (i.e., Household Roster, Sample Adult and Sample Child).

Figure 1. NHIS Structure



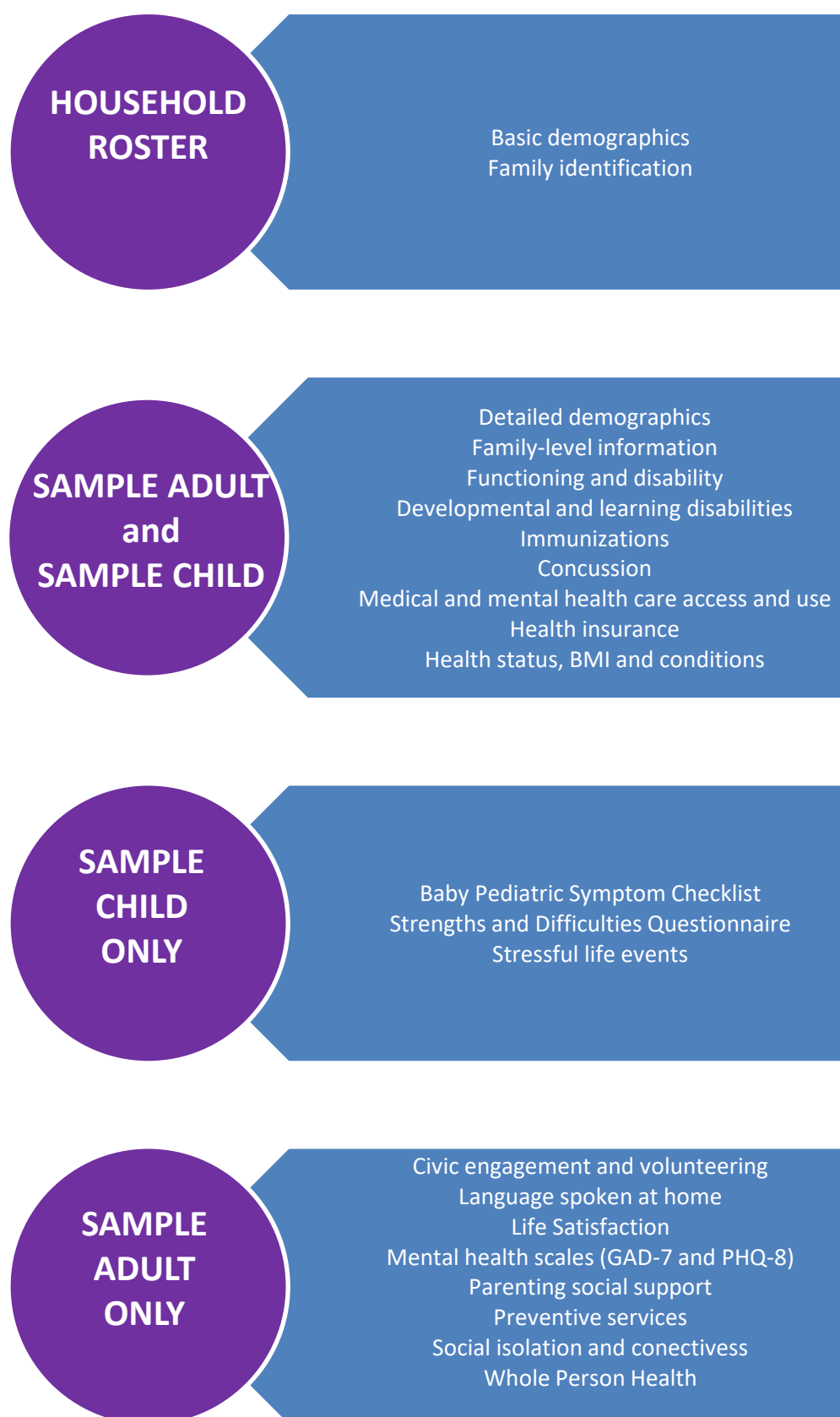
In households where there is an eligible Sample Adult and Sample Child, either the Sample Adult or Sample Child interview is administered first. Once both the Sample Adult and Sample Child interviews have been completed the interview is complete.

The NHIS is a survey of the civilian noninstitutionalized population, and active-duty military personnel are not included. In the rare case where a child lives in a household consisting of only active-duty military personnel, the Household Roster would be completed followed by a Sample Child interview.

Content of the Sample Child Interview

An adult knowledgeable and responsible for the health of the child is asked a set of questions about the Sample Child. Some questions asked of the Sample Child vary by age, as younger and older children have different health needs. Additional demographic information is also collected about the child and their family.

Figure 2. Topic organization in the Household Roster, Sample Adult and Sample Child modules: NHIS 2025.



Content of the Sample Adult Interview

The Sample Adult is asked a series of health questions about themselves. Some questions vary by age or sex of the Sample Adult, but most are the same for all Sample Adults. Additional demographic information is also collected about the Sample Adult and their family.

Types of Questions

The redesigned NHIS incorporates a long-term structure of fixed and periodic content. The long-term structure for the Sample Adult and Sample Child questionnaires organizes question topics by year and by type of content for the survey years 2019–2027. Additional information about periodicity of question topics for 2019–2027, see, <https://www.cdc.gov/nchs/nhis/about/2019-questionnaire-redesign.html>. There are four types of content: (1) Annual core; (2) Rotating core; (3) Sponsored content; and (4) Emerging topics.

Annual core are consistent questions that are asked every year.

Rotating core are questions that are asked some, but not all years. These questions are scheduled to appear on a periodic basis of every other year, one out of every three years, or two out of every three years.

Sponsored content are questions funded by other federal agencies or other centers within CDC about topics of interest to the sponsor. Sustaining sponsors fund content every year, whereas other sponsors fund content periodically.

Emerging topics are questions about areas of interest to NCHS, CDC, or HHS. These are newer subject areas that have generally not been researched in the general population.

Questionnaire Sections

The NHIS is divided into many questionnaire sections within each module, each with a different focus. The sections may include any combination of annual core, rotating core, sponsored content, or emerging topics. When the same questions or same types of questions are asked in Sample Adult and Sample Child interviews, the sections are given the same name for both interviews.

The names of the questions asked of the Sample Adult or pertaining to the Sample Adult's family all end in “_A” whereas those asked of the Sample Child or about the Sample Child's family end with “_C.” Section names have a 3-letter abbreviation (e.g., INS for Health Insurance), and questions are grouped by module and section.

Summary of the 2025 Content

A description of the 2025 topics and type of questions are described in this report under Sample Adult's Health, Sample Child's Health, and Health Insurance, while all demographic information has been portioned into four sets of characteristics: 1) those about the Sample Adult and Sample Child; 2) those about the parents or guardian residing in the household with the Sample Child; 3) those about the spouse or partner residing in the

household with the Sample Adult (if married or cohabiting); and 4) those about the family and household of the Sample Adult and Sample Child. In this document, multiple questionnaire sections with annual, rotating, emerging and sponsored content are described in each of the health topics included under Sample Adult's Health, Sample Child's Health, and sets of demographic characteristics.

Sample Adult health topics for 2025

I. Health Status and Conditions

self-reported health status, height, weight, pregnancy status, and the following health conditions: angina pectoris, anxiety disorder, arthritis, rheumatoid arthritis, gout, lupus, or fibromyalgia, asthma, attention deficit or hyperactivity disorder, cancer and cancer type, chronic fatigue syndrome, communications disorders, chronic obstructive pulmonary disease, coronary heart disease, dementia including Alzheimer's disease, depression, diabetes and type, epilepsy, gestational diabetes, hyperlipidemia, hypertension, immunosuppression, history of COVID and Long COVID, live birth in the past 12 months, menopausal symptoms and oophorectomy history, myocardial infarction, pre-diabetes, stroke, and whole person health. It also includes medication use for hyperlipidemia, hypertension, and diabetes including insulin initiation and GLP-1 injectables, emergency care for asthma, whether Long COVID symptoms limited usual activities, and whether overwhelmed by demands of living with diabetes.

II. Functioning and Disability

anxiety, cognition, communication and use of sign language, depression, hearing, mobility, self-care and upper body, social functioning (participation), vision, and age of disability onset.

III. Chronic Pain

frequency and severity of pain, pain impact, pain management techniques, pain in various body locations: back; hands, arms, or shoulders; hips, knees, or feet; headache or migraine; abdominal, pelvic, or genital pain; and toothache or jaw pain.

IV. Health Care Access and Health Service Utilization

Caregiving; difficulty paying for health care; health care use—medical care and wellness visit, dental care, mental health counseling, eye specialist, physical, speech, rehabilitate or occupational therapy, emergency care, hospitalization, retail health clinic, and urgent care center; immunizations (COVID-19, flu, pneumonia, RSV, shingles and Tdap); internet access and use of the internet for health and medical purposes; prescription medication; telehealth; usual place for medical care; unmet medical needs (dental, medical, mental health, and prescriptions) due to cost.

V. Preventive Care and Services

aspirin use; breast exam by a doctor, clinical checks for arterial blood pressure, blood glucose, A1C level, and cholesterol; history of hysterectomy; screenings for breast, cervical, colorectal, and prostate cancer; main reason for breast exam, colonoscopy, mammogram, PSA test, and for no up-to-date cervical and breast screening; received information about dense breast tissue at last mammogram; told if had PAP or HPV test at last cervical screening; payment for colonoscopy; a; doctor or health care professional recommended colorectal screening if no up-to-date colorectal screening; and number of PSA tests.

VI. Health-Related Behaviors

cigarette smoking; electronic cigarette use; use of other tobacco products (cigars and smokeless tobacco).

VII. Mental Health

General Anxiety Disorder-7 (GAD-7); Patient Health Questionnaire-8 (PHQ-8); life satisfaction; social parenting support; and social isolation and connectiveness.

VIII. Injuries

traumatic brain Injury.

IX. Community Engagement and Social Context

language spoken at home; participation in local elections and public meetings; and volunteer activities.

*Sample Child health topics for 2025***I. Health Status and Conditions**

reported health status of the Sample Child, and the following health conditions: asthma, communication disorders, developmental conditions (Autism, Asperger's disorder, pervasive development disorder, autism spectrum disorder, attention deficit hyperactivity disorder, intellectual disability, and learning disabilities), prediabetes or diabetes, history of COVID and Long COVID. It also includes emergency care for asthma.

II. Functioning and Disability

anxiety, behavior, cognition, communication, depression, hearing, mobility, self-care and upper body, and vision.

III. Health Care Access and Health Service Utilization

difficulty paying for health care; health care use—medical care and wellness visit, dental care, mental health counseling, eye specialist, physical, speech, rehabilitate or occupational therapy, emergency care, hospitalization, retail health clinic, and urgent care center; immunizations (COVID-19, and flu); prescription medication; telehealth; usual place for medical care; and unmet medical needs (dental, medical, mental health, and prescriptions) due to cost.

IV. Behavioral and Mental health

Baby Pediatric Symptom Checklist (BPSC), and Strengths and Difficulties Questionnaire (SDQ).

V. Stressful Life Events

had a parent who was incarcerated after birth, lived with someone with a mental illness, lived with someone with a drug or alcohol problem, and experienced or witnessed neighborhood violence.

VI. Injuries

traumatic brain injury.

Health insurance

coverage status at the time of interview, continuity and type of coverage in the past year, sources of coverage, characteristics of coverage, and reasons for no coverage.

Characteristics of Sample Adult and Sample Child

Adult and Child: age; Hispanic origin and race; place of birth; missed school days due to disability or injury; and sex.

Adult: education level, and currently in school; employment; marital status; parental status; sexual orientation; use of proxy for interview; and veteran status.

Child: special education or early intervention plan.

Characteristics of Sample Child's parents or guardian

Parent 1 and Parent 2: age; employment status; marital status; relationship to Sample Child; and sex.

Summary of all parents in household: highest level of education; parents of same or opposite sex; parents of same or different Hispanic ethnicity, and race; parents of same or different Hispanic ethnicity, and race, as Sample Child.

Characteristics of Sample Adult's spouse or partner

age; education; employment status; sex; same or different Hispanic ethnicity, and race as Sample Adult.

Characteristics of family and household of the Sample Adult and Sample Child

family income as a percent of federal poverty level with imputed income when missing; highest education level of adult family members; household has multiple families; housing tenure; Household Respondent is also the Sample Adult or Sample Child respondent; number of adults in family; number of children in family; family includes at least one person age 65 and over; number of adult family members working and working full-time; number of children and adults in the household; participation in food related programs (SNAP, WIC, reduced-price meals at school); receipt of housing assistance; region; telephone use; and urbanization level.

IV. Sponsors

Some 2025 NHIS content is sponsored by other federal agencies or other centers within CDC. . Sponsored content may be used to collect data on new topics or to go into more depth about subjects already on the NHIS. Sustaining sponsors add content every year. Other sponsors add content for selected years.

NHIS Sustaining Sponsors

Cancer Control and Prevention



The National Cancer Institute at the National Institutes for Health (NIH/NCI) and the National Center for Chronic Disease Prevention and Health Promotion at the Centers for Disease Control and Prevention (CDC/NCCDPHP) sponsored 22



questions asked of Sample Adults about breast, cervical, colon, and prostate cancer screening.

Immunization



The National Center for Immunization and Respiratory Diseases at the Centers for Disease Control and Prevention (CDC/NCIRD) sponsored seven Sample Adult questions about pregnancy in the past 12 months that ended in a live birth, and vaccination for COVID-19, RSV, shingles, and Tdap. NCIRD also sponsored two Sample Child questions about COVID-19 vaccination.

Insulin Use



The National Institute of Diabetes and Digestive and Kidney Diseases at the National Institutes for Health (NIH/NIDDK) and the National



Center for Chronic Disease Prevention and Health Promotion

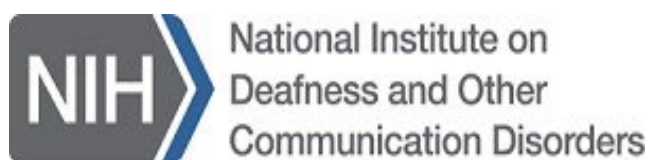
at the Centers for Disease Control and Prevention (CDC/NCCDPHP) sponsored three Sample Adult questions about insulin initiation among adults with diabetes who take insulin.

Noncigarette Tobacco Product Use and Menthol Cigarette Use



The Center for Tobacco Products at the Food and Drug Administration (FDA) sponsored five Sample Adult questions about the use of cigars and smokeless tobacco.

Voice, Swallowing, Speech, and Language



The National Institute on Deafness and Other Communication Disorders at the National Institutes of Health (NIH/NIDCD) sponsored 17 questions for the Sample Adult and 17 questions for the Sample Child on voice, swallowing, speech, and language.

Other NHIS Sponsors

ADHD

The National Center on Birth Defects and Developmental Disabilities (CDC/NCBDDD) sponsored five Sample Adult questions on attention deficit/hyperactivity disorder (ADHD).

Age of Disability Onset

The Administration for Community Living at the Department of Health and Human Services (ACL/HHS) sponsored one Sample Adult question about the age of disability onset.

Caregiving

The National Center for Chronic Disease Prevention and Health Promotion at the Centers for Disease Control and Prevention (CDC/NCCDPHP) sponsored one Sample Adult question about caregiving.

Diabetes

The National Center for Chronic Disease Prevention and Health Promotion at the Centers for Disease Control and Prevention (CDC/NCCDPHP) sponsored two Sample Adult questions on A1C testing, one question on diabetes stress, and one question on non-insulin injectable medications to lower blood sugar or lose weight.

Epilepsy

The National Center for Chronic Disease Prevention and Health Promotion at the Centers for Disease Control and Prevention (CDC/NCCDPHP) sponsored four Sample Adult questions about epilepsy.

Long COVID

The National Center for Immunization and Respiratory Diseases at the Centers for Disease Control and Prevention (CDC/NCIRD) sponsored four Sample Adult and four Sample Child questions on Long COVID.

Menopause

The Office of Research on Women's Health at the National Institutes of Health (NIH/ORWH) sponsored five Sample Adult questions on menopausal symptoms.

Pain Management

The National Center for Injury Prevention and Control at the Centers for Disease Control and Prevention (CDC/NCIPC) sponsored 11 Sample Adult questions on chronic pain management medications and therapies.

Satisfaction With Life

The National Center for Chronic Disease Prevention and Health Promotion at the Centers for Disease Control and Prevention (CDC/NCCDPHP) and the Office of Disease Prevention at the National Institutes of Health (NIH/ODP) sponsored one Sample Adult question on satisfaction with life.

Social Connectivity and Isolation

The National Center for Chronic Disease Prevention and Health Promotion at the Centers for Disease Control and Prevention (CDC/NCCDPHP) sponsored four Sample Adult questions on social connectivity and isolation.

Social Support and Loneliness

The National Center for Chronic Disease Prevention and Health Promotion at the Centers for Disease Control and Prevention (CDC/NCCDPHP) sponsored two Sample Adult questions on getting needed social support and feeling lonely.

Traumatic Brain Injury (TBI) -Concussion

The National Center for Injury Prevention and Control at the Centers for Disease Control and Prevention (CDC/NCIPC) sponsored five Sample Adult and five Sample Child questions on traumatic brain injury (concussion).

Whole Person Health

The National Center for Complementary and Integrative Health at the National Institutes of Health (NIH/NCCIH) sponsored eight Sample Adult questions asking respondents to rate different areas of their life.

V. Sample Sizes and Response Rates

When the NHIS sample was redesigned for 2025, the goal was to produce about 30,000 Sample Adult interviews a year, with expected adult completion rates by state used to allocate the annual sample. This would require about 34,500 households with completed rosters, which, in turn, would yield about 8,000 Sample Child interviews. A total of 66,935 addresses were sampled for 2025; this number also took into account expected ineligible addresses (e.g., vacant, demolished, converted to a permanent business).

Table 1 provides a breakdown of sample sizes for the various components of the 2025 NHIS public-use data release. As discussed in the section on data collection, the final 2025 sample sizes and response rates were impacted by the government shut down and the limitations on FR travel, hiring and retention.

Table 1. Final sample sizes for the 2025 NHIS public-use data release

Interview Unit	2025 Sample
Households	25,182
Sample Adults*	24,215
Sample Children	6,049

* For 465 of the 24,215 Sample Adults on the 2025 sample, a knowledgeable proxy answered for the Sample Adult because he or she was mentally or physically incapable of answering for himself or herself.

Response Rate Method

Beginning with the 2025 sample redesign, which supports state-level inference using at least three years of data, unweighted and weighted response rates are reported for the NHIS. According to AAPOR Standard Definitions (2023), surveys such as the NHIS that use complex, multi-stage sample designs with unequal probabilities of selection should report both weighted and unweighted response rates. Unweighted response rates are useful operationally because they provide a straightforward indicator of data collection performance and fieldwork success, allowing for comparisons of response performance across regions, FRs, or time periods. Weighted response rates better reflect the proportion of the target population represented by respondents and are particularly useful for evaluating the potential for nonresponse bias in survey estimates (Federal Committee on Statistical Methodology, 2001; Office of Management and Budget, 2006).

The weighted household response and Household Roster Completion rates presented below utilize the household base weight, which reflects the inverse of the household probability of selection. Weighted conditional and unconditional Sample Adult and Sample Child response rates utilize the Sample Adult base weight and Sample Child base weight, respectively. The Sample Adult base weight is defined as: household base weight multiplied by the inverse of the Sample Adult probability of selection. Similarly, the Sample Child base weight is defined as: household base weight multiplied by the inverse of the Sample Child probability of selection.

Unweighted and weighted response rates presented below conform to the American Association of Public Opinion Research (AAPOR) Response Rate Definition # 2, or AAPOR RR2 (AAPOR, 2023). “Interviewed Households,” “interviewed Sample Adults,” and “interviewed Sample Children” include those with completed interviews or acceptable “sufficient partial” interviews.

In the NHIS, a Sample Adult or Sample Child interview is considered fully complete when respondents complete all sections, and therefore questions, for which they are eligible. Conversely, an interview is considered a “partial” when all sections are not completed. The most common reason for a partial is a “break-off,” which occurs when a respondent stops the interview in-progress before completion and the FR fails to complete the interview during the allotted assignment period. The partial interview rate is the percent of all sufficiently complete interviews that are not entirely complete. These “sufficient partials” are counted as interviews in the computation of response rates. Partial interviews that are not far enough along in the interview, known as “insufficient partials,” are considered refusals and therefore included as eligible, non-respondents in the computation of response rates.

2025 Sample

Household Response Rates

For the 2025 Sample, the weighted household response rate was calculated by dividing the weighted number of interviewed households (N=56,824,682) by the sum of the weighted number of interviewed households (N=56,824,682) and the weighted number of nonresponding households (N=71,369,304). Nonresponding households are eligible households that were not interviewed for a variety of reasons, including language barriers, no one home after repeated contact attempts, refusal, household records rejected for insufficient data, or other reasons for no interview. Calculation of the unweighted household response rate follows the same logic: number of interviewed households (n=25,182) divided by the sum of the number of interviewed households (n=25,182) and the number of nonresponding households (n=32,420).

The weighted Household Response Rate for the 2025 Sample was 44.3%.

The unweighted Household Response Rate for the 2025 Sample was 43.7%.

The weighted Household Response Rate should be reported if one is performing a household-level analysis using the public-use paradata file.

Household Roster Completion

The completion of the Household Roster is defined as the enumeration of all persons in an eligible household, with basic demographic information collected about each household member. The weighted Household Roster Completion Rate is calculated by dividing the weighted number of eligible households with a completed Household Roster (N=62,845,485) by the weighted number of eligible households (N=128,193,986). For the 2025 Sample, the weighted Household Roster Completion Rate was 49.0%. The unweighted Household Roster Completion Rate was 48.4%: number of eligible households with a completed Household Roster (n=27,867) divided by the number of eligible households (n=57,602). Based on demographic information obtained from completed Household Rosters, there were 7,167 (weighted=29,707,804) eligible Sample Children and 27,831 (weighted=119,040,844) eligible Sample Adults in the 2025 Sample.

Sample Child Response Rates

Sample Child response rates can be computed two ways, resulting in either a conditional or final response rate. The weighted Conditional Sample Child Response Rate is calculated by dividing the weighted number of interviewed Sample Children (N=25,433,474) by the weighted number of eligible Sample Children from households with completed rosters (N=29,707,804). For the 2025 Sample, the weighted Conditional Sample Child Response Rate was 85.6%. The unweighted Conditional Sample Child Response rate was 84.4%: number of interviewed Sample Children (n=6,049) divided by the number of eligible Sample Children from households with completed rosters (n=7,167).

The weighted Final Sample Child Response Rate accounts for the weighted Household Roster Completion Rate and is calculated by dividing the weighted number of interviewed Sample Children (N=25,433,474) by the weighted number of eligible Sample Children (N=29,707,804) from households with completed rosters, and then multiplying this quotient by the weighted Household Roster Completion Rate (49.0%). Here again, the unweighted Final Sample Child Response Rate follows the same logic: number of interviewed Sample Children (n=6,049) divided by the number of eligible Sample Children from households with completed rosters (n=7,167) multiplied by the unweighted Household Roster Completion Rate (48.4%). In 2025, 1.4% (weighted=1.5%) of Sample Child interviews were sufficient partials.

The weighted Final Sample Child Response Rate for the 2025 Sample was 42.0%.

The unweighted Final Sample Child Response Rate for the 2025 Sample was 40.8%.

Data users should report the weighted Final Sample Child Response Rate when reporting on analyses performed with the Sample Child data file.

Sample Adult Response Rates

As with Sample Children, both a conditional and final response rate can be computed for Sample Adults. The weighted Conditional Sample Adult Response Rate is calculated by dividing the weighted number of interviewed Sample Adults (N=100,307,488) by the weighted number of eligible Sample Adults from households with completed rosters (N=119,040,844). For the 2025 Sample, the weighted Conditional Sample Adult Response Rate was 84.3%. The unweighted Conditional Sample Adult Response rate was 87.0%: number of interviewed Sample Adults (n=24,215) divided by the number of eligible Sample Adults from households with completed rosters (n=27,831).

The weighted Final Sample Adult Response Rate is calculated by dividing the weighted number of interviewed Sample Adults (N=100,307,488) by the weighted number of eligible Sample Adults from households with completed rosters (N=119,040,844) and then multiplying this quotient by the weighted Household Roster Completion Rate (49.0%). The unweighted Final Sample Adult Response Rate uses the same formula: number of interviewed Sample Adults (n=24,215) divided by the number of eligible Sample Adults from households with completed rosters (n=27,831) multiplied by the unweighted Household Roster Completion Rate (48.4%). In the 2025 Sample, 3.4% (weighted=3.4%) of Sample Adult interviews were sufficient partials.

The weighted Final Sample Adult Response Rate for the 2025 sample was 41.3%.

The unweighted Final Sample Adult Response Rate for the 2025 sample was 42.1%.

Data users should report the weighted Final Sample Adult Response Rate when reporting on analyses performed with the Sample Adult data file.

Note that unweighted and weighted numbers of households, Sample Children, and Sample Adults eligible and interviewed were used for the calculations of response rates shown and rounding discrepancies may occur when using the percentages.

Reporting Household, Sample Adult, and Sample Child Response Rates

As noted above, data users should report the weighted Final Sample Adult Response Rate when reporting on analyses performed with the Sample Adult data file. Similarly, data users should report the weighted Final Sample Child Response Rate on analyses performed with the Sample Child data file. The weighted Household Response Rate should be reported if one is performing a household-level analysis using the public-use paradata file. The weighted Household Roster Completion Rate and the weighted Conditional Response Rates may be reported, in addition to weighted Final Response Rates, when data users want to include additional information about the effectiveness of each stage of the data collection process. If one is analyzing multiple years of data, say 2024 and 2025, Tables 2–5 provide weighted and unweighted counts and response rates from 2019–2025.

Response rates obtained during 1997–2018 NHIS and 2019–2025 NHIS should be presented separately. First, the definition of an interviewed household differs from the past questionnaire design (1997–2018). Previously, an interviewed household was defined as one where at least one family in the household completed a substantial portion of the family interview. With the family interview removed from the redesigned NHIS, an interviewed household is now defined as one where the Household Roster and a substantial portion of either the Sample Adult interview or the Sample Child interview (if one or more children reside in the household) is completed. In addition, Final Sample Adult and Sample Child Response Rates during 1997–2018 NHIS were the product of the Conditional Sample Adult or Sample Child Response Rate and the Final Family Response Rate. Starting in 2019, Final Sample Adult and Sample Child Response Rates are the product of the Conditional Sample Adult or Sample Child Response Rate and the Household Roster Completion Rate.

Table 2. Weighted number of eligible and interviewed households, Sample Adults and Sample Children, National Health Interview Survey, 2025

Year	Household			Sample Adult		Sample Child	
	Eligible	Roster Complete	Interviewed	Eligible	Interviewed	Eligible	Interviewed
2019	119,500,083	77,019,611	72,928,888	145,332,636	130,018,970	42,103,768	38,602,204
2020	77,469,942	41,767,591	39,110,171	80,150,631	70,604,757	21,775,997	19,476,397
2021	122,055,914	70,194,916	64,612,277	134,325,018	115,635,969	37,243,871	32,704,406
2022	123,033,572	66,931,037	61,043,324	127,240,488	107,786,219	35,395,596	30,247,802
2023	123,177,437	66,348,188	60,345,513	126,232,509	107,085,069	33,787,626	28,671,137
2024	125,627,995	68,561,791	62,184,219	130,951,675	110,633,891	34,480,809	29,389,455
2025	128,193,986	62,845,485	56,824,682	119,040,844	100,307,488	29,707,804	25,433,474

Summary of Household, Sample Adult, and Sample Child Response Rates Since 2019

Weighted counts for eligible and interviewed sample units used in the calculation of weighted response rates for combined data years are shown in Table 2. The corresponding unweighted counts are presented in Table 3. Table 4 presents weighted and unweighted conditional response rates for the Sample Adult and Sample Child modules from 2019–2025, while Table 5 presents weighted and unweighted final response rates for the Household, Sample Adult, and Sample Child modules for 2019–2025.

Table 3. Unweighted number of eligible and interviewed Households, Sample Adults and Sample Children, National Health Interview Survey, 2025

Year	Household			Sample Adult		Sample Child	
	Eligible	Roster Complete	Interviewed	Eligible	Interviewed	Eligible	Interviewed
2019	54,231	35,404	33,138	35,365	31,997	10,155	9,193
2020	43,280	23,694	21,930	23,694	21,153	6,626	5,790
2021	58,068	33,325	30,673	33,264	29,482	9,511	8,261
2022	58,215	31,654	28,854	31,579	27,651	8,879	7,464
2023	62,800	33,742	30,670	33,692	29,522	9,202	7,692
2024	68,279	37,371	33,917	37,294	32,629	9,975	8,360
2025	57,602	27,867	25,182	27,831	24,215	7,167	6,049

Table 4. Weighted and unweighted conditional response rates, National Health Interview Survey, 2019–2025

Year	Sample Adult		Sample Child	
	Weighted	Unweighted	Weighted	Unweighted
2019	89.5	90.5	91.7	90.5
2020	88.1	89.3	89.4	87.4
2021	86.1	88.6	87.8	86.9
2022	84.7	87.6	85.5	84.1
2023	84.8	87.6	84.6	83.6
2024	84.5	87.5	85.2	83.8
2025	84.3	87.0	85.6	84.4

Notes: The weighted Conditional Response Rate is calculated by dividing the weighted number of interviewed Sample Adults or Sample Children by the weighted number of eligible Sample Adults or Sample Children from households with completed rosters. The unweighted conditional response rate is calculated the same way using unweighted counts.

Table 5. Weighted and unweighted unconditional response rates, National Health Interview Survey, 2019–2025

Year	Household		Sample Adult		Sample Child	
	Weighted	Unweighted	Weighted	Unweighted	Weighted	Unweighted
2019	61.0	61.1	57.7	59.1	59.1	59.1
2020	50.5	50.7	47.5	48.9	48.2	47.8
2021	52.9	52.8	49.5	50.9	50.5	49.9
2022	49.6	49.6	46.1	47.7	46.5	45.8
2023	49.0	48.8	45.7	47.0	45.5	44.9
2024	49.5	49.7	46.1	47.9	46.5	45.9
2025	44.3	43.7	41.3	42.1	42.0	40.8

Notes: The weighted Final Household Response Rate is calculated by dividing the weighted number of interviewed households by the weighted number of eligible households. Weighted Final Sample Adult and Sample Child response rates, respectively, are calculated by dividing the weighted number of interviewed Sample Adults or Sample Children by the weighted number of eligible Sample Adults or Sample Children and then multiplying this quotient by the weighted Household Roster Completion rate. The unweighted response rate is calculated the same way using unweighted counts.

VI. Weighting

NHIS is a sample survey. That is, only a sample (subset) of the civilian noninstitutionalized population is selected to participate in the survey. Additionally, not everyone selected to participate agrees to participate, which can affect the representativeness of the sample. To account for these two factors, sampling weights are created.

These sampling weights are used to produce representative national estimates. The data must be weighted to obtain population estimates for survey outcomes in the population represented by the NHIS. The value of the weight for a given respondent can be interpreted as the number of persons in the NHIS target population represented by that respondent. The sum of the weights over all respondents is used to estimate the size of the total target population. The weights reflect several steps of adjustments starting with a base weight, which is inverse to the probability of selection. Households and persons that are more likely to be selected are given lower weights so that the final estimates are not biased by their increased likelihood of being selected. For example, in a household of two eligible adults, the Sample Adult has a selection probability of one-half, and therefore their base weight will be increased two-fold. However, in a household of four eligible adults, the Sample Adult has a selection probability of one-fourth, and therefore their base weight will be increased four-fold, since roughly speaking they represent more people from the household. The base weights are then adjusted for nonresponse patterns, that is, the different response rates among different household and person-level subgroups. Since the 2025 NHIS data files contain data from 11 months of sample, a 12/11 adjustment was applied to the household base weight. That is, in 2025 all household base weights for the 11 months included were inflated by a factor of 1.09.

The 2019 questionnaire redesign provided an opportunity to evaluate the methods through which weights are adjusted to account for differential nonresponse that had been in place since 1997. For 1997-2018, the adjustment approach was based on geography; the weights for households and persons in geographic areas with lower response rates were increased more than for those in areas with higher response rates. That way, final estimates were not biased by the latter group's increased likelihood of participation. More sophisticated methods to decrease potential nonresponse bias are now available (Olson, 2013; Valliant et al., 2018), and based on the evaluation, the weighting process for 2019 was updated. In 2019 the updated approach for nonresponse adjustment used

multilevel regression models that include paradata variables that are predictive of both survey response and selected key health outcomes, the key criteria for effective bias reduction. The paradata used in the regression models included FR observations of the sample unit and neighborhood, and aggregate, contextual variables (e.g., percentage of adults age 65 and older in the Census tract) taken from sources such as the Census Planning Database (CPD) and Area Health Resource File (AHRF). For 2021 and 2022, Recursive Partitioning for Modeling Survey Data (RPMS) was used within R programming language to build classification trees of household, Sample Adult, and Sample Child response (Toth, 2019). Since 2023, the function `ctree()`, a conditional inference tree method, in the R package `partykit` has been used to build the classification trees of response (Hothorn and Zeileis, 2015). The inverse of the response rate within terminal nodes of each resulting tree serves as the nonresponse adjustments to Household, Sample Adult, and Sample Child base weights. A major advantage of classification tree analysis is the automatic consideration and estimation of complex interactions.

Finally, the nonresponse adjusted weights are typically calibrated to U.S. Census Bureau population projections and American Community Survey (ACS) one-year estimates. Starting in 2019, calibration is performed with iterative proportional raking using the following raking dimensions: age by sex (population projections), age by race and ethnicity (population projections), educational attainment (ACS one-year estimate), and Census division by Metropolitan Statistical Area (MSA) status (ACS one-year estimates). In 2020, housing tenure (ACS one-year estimate) was added to the calibration step, in 2022, Census division was replaced with region, and in 2025, household size was added to the calibration. Note that for the 2021 survey year, the U.S. Census Bureau did not release single-year ACS estimates by housing tenure, education level, and MSA by Division. Therefore, substitute calibration totals for these variables were obtained from the 2021 Current Population Survey (CPS) March Annual Social and Economic (ASEC) Supplement. ACS one-year estimates were once again used for raking dimensions starting in 2022. These changes to the nonresponse adjustment approach and the calibration methods have the potential to impact comparisons of the weighted survey estimates over time.

Weights

The Sample Adult and the Sample Child file each have unique and separate final annual weights. The Final Annual Weight should be used to generate national estimates. This weight includes the design, ratio, nonresponse and calibration adjustments.

The final annual weight in the Sample Adult file is WTFA_A.

The final annual weight in the Sample Child file is WTFA_C.

Further information on how to implement sampling weights is found in the section “Analyzing 2025 NHIS.”

The paradata files do not contain weights. Depending on the analysis of the paradata, if weights are needed, they can be pulled from public-use data files.

Interim weights are those sampling weights that do not include the final calibration adjustments to population control totals for age by sex, age by race and ethnicity, education, housing tenure, and region by MSA status. Since 2020, interim weights are not included in the public use files and are available through the NCHS Research Data Center (RDC): <https://www.cdc.gov/rdc/>.

Variance Estimation

In a data collection, estimates based on different samples will vary and can differ from the true population values. The estimated difference between the true target population value and the estimate from a random sample is the sampling error. Sampling error cannot be directly calculated because the true target population value is unknown. Rather, sampling error is estimated and expressed as a standard error (SE), the average degree to which estimates based on random samples differ from each other and the true target population value due to sampling. This measure is incorporated in common statistical methods such as significance testing and estimating confidence intervals. Because of the complex nature of the NHIS sampling design (specifically, the use of stratified cluster sampling), key nesting variables were created to capture explicit stratification and to identify clustering for a more accurate estimation of the sampling error.

For both the Sample Adult and the Sample Child files, the stratum and primary sampling unit (PSU) variable names are PSTRAT and PPSU.

PSTRAT and PPSU are simplified versions of the true NHIS sample design variables created for the public-use files in order to protect the identity of survey respondents. The strata identifier is not directly related to state or density strata. When using the publicly available data files for estimation purposes, strata and PSU identifiers provided by NCHS are required to properly estimate variances. The use of these publicly available variance estimation variables may provide slightly different standard errors than the use of the confidential variance estimation variables used by analysts at NCHS. Data users who want access to the confidential variance estimation variables used by analysts at NCHS may apply to the RDC.

A new sample design began in 2025. Data users combining or pooling 2025 NHIS data with prior years, using the restricted variance estimation variables, should modify the stratum variable to uniquely differentiate between current and prior sample design. Analysts combining or pooling public-use NHIS data files with prior years since 2019 do not need to modify the stratum variable.

Analysts should be aware that the use of standard statistical procedures that are based on the assumption that data are generated via simple random sampling (SRS), instead of a complex sample design, generally will produce incorrect estimates of variances and standard errors when used to analyze data from the NHIS. Analysts who apply SRS techniques to NHIS data generally will produce standard error estimates that are, on average, too small, and are likely to produce results that are subject to excessive Type I error.

Degrees of Freedom

The number of degrees of freedom is used to determine the t-statistic, its associated percentage points, p-values, standard error, and confidence intervals. A rule of thumb to calculate the number of degrees of freedom to associate with a standard error is the quantity (number of PSUs - number of strata). Typically, this rule is applied to a design with at least two PSUs per stratum and when the variance components by stratum are roughly the same magnitude. This rule of thumb is not directly applicable to the NHIS design. The applicability of this rule of thumb depends upon the variable of interest and its interaction with the design structure (for additional information, see Chapter 5 of Korn and Graubard, 1999). As the number of degrees of freedom becomes large, the distribution of the t-statistic approaches the standard normal distribution. For example, with 120 degrees of freedom, the 97.5 percentage point of the t distribution is 1.980, while the 97.5 percentage point of the standard normal distribution is 1.960. If a variable of interest is distributed across most of the NHIS address clusters, a normal distribution assumption may be adequate for analysis since the number of degrees of

freedom would be large. The user should consult a mathematical statistician for further discussion.

VII. Editing the Data During and After the Interview

Edits to Protect Confidentiality

NCHS (including its contractors and agents) collects personally identifiable NHIS and other survey data under a pledge of confidentiality and a promise that the data will be used only for statistical purposes. Section 308d of the Public Health Service Act (42 U.S.C. 242m(d)) and Section 302 of the Foundations for Evidence-Based Policymaking Act of 2018 (44 U.S.C. 3561-3583, also known as the Confidential Information Protection and Statistical Efficiency Act (CIPSEA)) require that confidentiality be maintained without exception. Violations of CIPSEA are a class E felony, punishable by imprisonment for not more than 5 years, a fine of not more than \$250,000, or both. Strict procedures in survey operations and data dissemination are used by NCHS, its data collection contractors, and other agents to prevent disclosure of survey subjects' identities.

The risk of inadvertent disclosure of confidential information regarding individual respondents is higher when there exists a publicly released data set having detailed geography variables and a detailed and extensive set of survey observations. For this reason, the NHIS does not publicly release state identifiers and some other geographic variables, and the original design strata and primary sampling units (PSUs) are masked when the data are publicly released. NHIS data sets may also be coarsened by suppressing survey variables, collapsing multiple variables into one, and collapsing response categories. In addition, statistical noise at both the variable level and record level may occasionally be added to protect confidentiality.

Notes fields in the Codebook report may include information about edits and data suppression that were done to protect the confidentiality of NHIS participants. However, one important edit is worth noting here because it applies to multiple variables across the survey. To protect confidentiality among the oldest adults, all age variables were top-coded to "85 years and older" (85+). For example, survey questions related to age at diagnosis for a type of cancer (e.g., LUNGAGE_A) and diabetes (DIBAGE_A) ("How old were you when you were diagnosed with [this condition]?") are top-coded to 85+ years.

To further protect confidentiality, detailed information for some variables is not available on the NHIS public-use data files. For a list of questions not available on the public-use file, see the annual restricted-use codebook available on the data release webpage, and the Appendix in this document.

Analysts interested in working with data that were suppressed or edited to protect confidentiality may apply to access selected unmodified data files through the NCHS RDC. The RDC is a data enclave established to provide a mechanism whereby researchers can access detailed data files in a secure environment without jeopardizing the confidentiality of survey participants. Information about RDC access options and application procedures is available at: <https://www.cdc.gov/rdc/>.

Family-Level Replicate

In the field, the FR can conduct either the Sample Adult or Sample Child interview first in households where both eligible adults and children reside. In instances where the Sample Adult and the Sample Child belong to the same family, the instrument is optimized to only ask family level questions in the first interview. This helps to minimize respondent burden by eliminating repetition for family level questions, such as family income. If,

however, the respondent of the first interview refuses or doesn't know the answer to a significant number of questions within a family-level section, that section is repeated in the second interview when the respondent of the second interview is not the same individual.

The family level data collected are then replicated (i.e., copied) to the other interview to a replicate variable of the same name (but a different suffix) in a post-processing step. For example, if the adult interview preceded the child interview and they are in the same family, the question about whether anyone in the family had problems paying medical bills is collected in the adult variable PAYBLL12M_A and replicated to the child variable PAYBLL12M_C. The Questionnaire report identifies a variable as being replicated in the "Replicate to:" field. Searching the document for the string "Replicate" will identify the variables that underwent replication.

When the Sample Adult and the Sample Child are in different families within the household, both the Sample Adult and the Sample Child respondent will be asked family level questions about their respective families. In households where there are no children or there are no eligible adults (e.g., all active-duty military personnel), there is no replication involved.

Annual core sections of the questionnaire with instrument optimizations and replicated variables include Family Income (INC), Family Employment (FEM), Difficulty Paying for Health Care (PAY), Food-Related Programs (FOO), Housing (HOU), and Telephone Use (TEL). They can also be found in some sponsored sections of the questionnaire in 2019–2024.

Replicate measures are used in analyses the same way as any other measure available in the Sample Adult or Sample Child files. Analyses of replicate measures can be interpreted as an estimate of Sample Adults, Sample Children, or persons who are in a family meeting a specific outcome or characteristic (e.g., percentage of persons ages 0–64 years who are in a family that is having problems paying medical bills).

Hard and Soft Edits

To help prevent both interviewer data entry error and respondent error, range values and consistency checks may be programmed into the CAPI system. During the interview, if an FR enters an out-of-range value (such as 180 years instead of 18 for age), an error message instructs the FR to enter a new value. Such an interruption of the interview is called a "hard edit" if the interview cannot continue without an acceptable response being entered, and a "soft edit" if the interview may continue with or without a new response being entered. Soft edits may apply to questions for which the response entered is plausible (such as an extreme height value).

Even with such checks built into the CAPI system, data cleaning (data "editing") is still necessary. The first step in the data cleaning process is verification of the valid number of cases in the data file and the review of frequencies for reasonableness. Each variable is examined to determine if its values are within its range of permissible values. Values not in that range are verified as missing if they are not in the universe due to legitimate skip patterns in the questionnaire or set to the special value of "not ascertained" if there was a break-off in the interview.

Question-Specific Replication

An optimization edit is an edit that fills-in values for variables that were skipped in the instrument because the information could be inferred from the Sample Adult or Sample Child interview, whichever went first. For

example, the marital status of the Sample Adult would be known if the Sample Child interview preceded the Sample Adult interview, if the Sample Adult and Sample Child were in the same family, and if the Sample Adult was also a parent of the Sample Child. Select questions in health insurance relating to detailed characteristics of shared private plans between the Sample Child and Sample Adult in the same family were also filled-in from responses of the interview that came first.

Recode into Different Variable

Recodes have been created for select questions to make the data more analytically useful. One example of this is a recode that converts a single variable allowing selection of as many answers as are applicable into a series of variables (one for each possible response) with yes/no or mentioned/not mentioned responses. Other recodes have been created to summarize information obtained from multiple questions available to the public (e.g., summary scores of validated scales), or to combine information from which some information may not be publicly available (e.g., multiple race categories).

Data users are recommended to review the description of 'Major Recodes' for the topic of interest in this document, and the codebook documentation for additional recode information.

Orientation to How to Use NHIS Data

All datasets and associated documentation for 2025 are available on the NHIS website:

<https://www.cdc.gov/nchs/nhis/documentation/2025-nhis.html>

Overview

The NHIS website has information to help users understand, analyze and download the 2025 NHIS data and documentation. The following description provides a walkthrough of the website and information about the files.

Using our data

Survey Description document (PDF): A description of NHIS methods, year-specific response rates and content, examples on how to use NHIS, and other useful resources for NHIS data users.

Imputed income technical document (PDF): A description of the methodology for creating the 2025 NHIS imputed income variables.

Preferred Reporting Items for Complex Sample Survey Analysis (PRICSSA) (PDF): A year-specific itemized checklist to guide researchers publishing analyses using complex sample survey data, intended to help eliminate analytic and reporting errors and increase transparency and reproducibility.

Data file metadata (PDF): A year-specific list summarizing the number of records, file size, record length, and reference values for each of the ASCII and CSV data files released that allows data users to verify the integrity of downloaded files.

Restricted data

These links provide more information on accessing restricted NHIS data through the Research Data Center at NCHS.

The main access page for the RDC is available here: <https://www.cdc.gov/rdc/>

Restricted-use variable access: A link to information about NHIS restricted data available through the RDC.

Restricted-use geography data provides information on the lower levels of geography variables (not in the public-use files) that are available through the RDC.

Questionnaires and survey materials

Questionnaires and content

Survey Questionnaire – English (PDF) and Spanish (PDF): Year-specific NHIS questions fielded.

The 2025 survey questionnaire (PDF) lists the questions in the survey and descriptive information about them. The information in the document is organized in two panels: a hierarchical bookmarks panel on the left for navigation, and a main panel on the right for displaying detailed content. The bookmarks themselves are organized as:

- a) Link to a contents page that explains the document’s overall structure.
- b) Hierarchical section index that lists the sections, their descriptions, the content type (Annual Core, Rotating Core, Sponsored Content or Emerging Content) and the page range in the PDF for each section for ease of printing
- c) Hierarchical questionnaire organized by module, section and variable and appearing in the order that the questions are asked.
- d) A glossary of variables mentioned in the skip and fills instructions of some survey questions that are used for programming the interview.

Selecting a bookmark for a module or a section navigates to the first variable in the module or section, respectively. When selecting a variable of interest, detailed information for that variable is displayed in the main panel. For each variable, the main panel heading has the year and title of the survey along with the section abbreviation and description. The body of the main panel starts with a header (in blue background) with the Question ID (used for ordering questions in the questionnaire), the variable name, the interview module and the content type. This is followed by the question text as it appears in the instrument. The question text may contain one or more context-sensitive fills, indicated by text with a leading caret (^) symbol. A fill is text that is conditionally generated to modify the question text to make it more suitable to the context of the interview. For example, the fill whose name is ^heshe_C will expand to “he” if the Sample Child respondent indicated that the Sample Child is male, “she” if female or “they” if sex was not provided. All fills in the question text appear in the fills table in order along with their description and rule-based instructions on how the fill text is generated in the instrument. For some variables, as part of the question text, there may be interviewer instructions in bolded blue text with any optional text appearing in italics and gray font. Below the fills table, there is another table with valid response categories and their descriptions, followed by the universe description, and if present, any skip instructions, hard or soft edits.

Both English and Spanish versions of the questionnaire are available on the NHIS website. The Spanish version of the questionnaire has Spanish translations for the question text, the fills and the response choices.

NHIS Sponsored Content (PDF)

A year-specific list of sponsoring agencies and associated sponsored questions in NHIS. The information in this document is organized in two panels: a bookmarks panel on the left listing the sponsoring organization names alphabetically (short form), and a main panel on the right listing the variables and some descriptive information about them such as question ID, question text and the universe description. When content is sponsored by multiple organizations, the names of all the

sponsors are shown on the bookmarks panel. Selecting an organization's bookmark takes you to the content sponsored by that organization. The document's page header lists the sponsoring organizations' full name(s).

NHIS Instrument Flowchart(s) (PDF)

A graphical view of the questionnaire sections within each module during the year. The chart uses different colors to identify sections that include annual, rotating, sponsored and emerging content, and a different shape to identify sections that include family-level questions. A description for each section is listed in page 2.

Participant materials

Advance Letter (PDF): A letter mailed to sampled households prior to FR contact and provided to survey respondents at the time of the FR's visit that explains the NHIS.

Other survey materials

Field Representative Manual (PDF): The manual on Computer-Assisted Personal Interview (CAPI) for NHIS interviewers.

Sample Adult interview

Sample Adult interview files contain data from the interview of the randomly selected adult in the household. The Documentation tab includes a PDF of the Summary, Codebook, and Codebook for restricted use variables. The Public-use data tab includes data files as CSV and ASCII in a compressed format or zip, and input statements to read the ASCII file for computer software using SAS, SPSS, and Stata.

Table 6 lists the names of data files, programs and documentation in the 2025 NHIS data release. The summary PDF contains the variable name, a brief description, and variable type (e.g., character or numeric) while the codebook provides more detailed description including the question universe (i.e., respondents that received the question), responses options, and unweighted frequencies. The restricted use variables PDF provides a more detailed description of the variables that are restricted, and not available on the public-use file.

Table 6. Data release files names for the Sample Adult, Sample Child, Imputed Income and Paradata files: NHIS 2025.

Type of file	Sample Adult Annual files	Sample Child Annual files	Imputed Income files	Paradata files
Data in column-delimited ASCII format	adult25.dat	child25.dat	Adultinc25.dat Childinc25.dat	paradata25.dat
Data in comma separated values (CSV) format	adult25.csv	child25.csv	Adultinc25.csv Childinc25.csv	paradata25.csv
SAS program with input statements	adult.sas	child.sas	adultinc.sas childinc.sas	paradata.sas
STATA program with input statements	adult.do	child.do	adultinc.do childinc.do	paradata.do
SPSS program with input statements	adult.sps	child.sps	adultinc.sps childinc.sps	paradata.sps
Summary*	Adult-summary.pdf	Child-summary.pdf	Adultinc-summary.pdf Childinc-summary.pdf	Paradata_summary.pdf
Codebook*	Adult-codebook.pdf	Child-codebook.pdf	Variables included in the Adult and Child Codebooks	Paradata_codebook.pdf
Codebook for restricted-use variables	Adult-nofreq-codebook.pdf	Child-nofreq-codebook.pdf	Not available	Paradata-nofreq-codebook.pdf

*More information about the documentation is provided below.

Sample child interview

Sample child interview files contain data from the interview of the randomly selected child in the household. The Documentation tab includes a PDF of the Summary, Codebook, and Codebook for restricted use variables. The Public-use data tab includes data files as CSV and ASCII in a compressed format or zip, and input statements to read the ASCII file for computer software using SAS, SPSS, and Stata.

Parent-child pair data

NHIS parent-child pair data files contain dyad or pair weights that can be used for studies using parent-child pairs in NHIS. Adult-child pair sampling weights were developed for use in the analysis of mother-child and father-child pair data of NHIS. Documentation, including a Readme document, detailed report with sample analyses, and public-use data files (CSV, SAS) are provided in the parent-child pair data section of the webpage. See the section “Parent-Child Pair Analysis” later in this document for additional information.

Imputed income

NHIS Imputed income files contain data about respondents’ family income. Family income variables, which are based on total family income in the previous calendar year, are collected on the NHIS interview. Family income is known to have a high percentage of nonresponse; therefore, multiple imputation is used to impute missing data on the family income variable. Since 2019, NHIS imputed income files have contained 10 multiply imputed family income values. Documentation, including an imputed income technical document, Sample Adult summary, Sample child summary, public-use data files (CSV, ASCII), and input statements for computer software (SAS, SPSS, Stata) are provided in the imputed income section of the webpage.

Paradata

NHIS paradata files contain data about the interview process, including information collected through the Contact History Instrument (CHI) regarding FR contact attempts, strategies for gaining participation, and reasons for respondent reluctance. The Back Section of the NHIS gathers details on the mode of interview, location, and reasons for partial interviews or breakoffs. Additionally, date and time variables are recorded for each module (Household, Sample Adult, Sample Child), and the paradata file can be linked with the NHIS regular public-use health data file when cases are sufficiently complete.

Documentation, including a paradata survey description document, summary, codebook, codebook for restricted-use variables, public-use data files (CSV, ASCII), and input statements for computer software (SAS, SPSS, Stata) is provided in the paradata section of the webpage. The summary PDF contains the variable name, a brief description, and variable type (e.g., character or numeric) while the codebook provides more detailed description including the question universe (i.e., respondents that received the question), responses options, and unweighted frequencies. The restricted use variables PDF provides a more detailed description of the variables that are restricted and not available on the public-use file.

Documentation

Codebook (PDF)

The Codebook report combines all the detailed information for a variable with the unweighted frequencies (counts and percentages) found in the data. The report has a navigational bookmarks panel on the left with expandable module and section bookmarks in questionnaire order. The main panel on the right contains variable details. When a variable bookmark is selected, the detailed display includes its module, section, file, data type, question text (if present), question fill information, universe and universe description, the variable description or label, question ID, keywords, and notes. This is followed by a table that provides the unweighted frequencies and percentages for the variable. All response categories are shown in the table, including those with a zero count in the data files. For continuous variables, a range of values is provided, but frequency counts for each response is not included. This allows users to see a complete list of response categories with frequencies for each variable without referring to additional documentation. In addition, the “frequency missing” label will be shown if a variable has cases that are not in the universe.

In the NHIS, the same codes are used across all files to designate “refused” (RF) and “don’t know” (DK) responses: refusals are coded as 7 (with leading 9’s added to the length of the field, as in 7, 97, 997, etc.), while “don’t know” responses are coded as 9 (with leading 9’s added to the length of the field, as in 9, 99, 999, etc.). For partially completed interviews (e.g., Sample Adult interviews where the respondent discontinued the interview before reaching the question), the responses will appear as 8’s for “not ascertained,” again with leading 9’s added to the length of the field, as in 8, 98, 998, etc., for the remaining variables in the file. A code of 8 is also used to indicate “not ascertained” responses when the field was blank or contained an impossible code. Lastly, in some limited situations (primarily recodes), the “Refused,” “Don’t know,” and “Not ascertained” categories may be collapsed into a single category called “Unknown,” which is typically designated with an 8 or 9 (with leading 9’s to fill out the field, if necessary). Data users are advised to read the notes in the data release documentation for further information about the variables of interest.

Codebook definitions

The following defines some terms used in the different reports:

F1: Indicates there is a ‘help screen’ available to the interviewers with definitions about the question topic that can be used for answering respondents’ questions.

Fills: Text that modifies the question, based on previously collected information and using conditional logic. Fills are indicated by a caret (^) symbol followed by the name of the fill, e.g., ^SCNAME.

Keywords: Descriptive words or phrases relevant to the topic of the variable; these can be used for word searches.

Notes: Additional information that analysts need to know about a variable, such as assumptions, limitations, caveats, and differences between instrument versions. Analysts are encouraged to read the notes pertaining to variables of interest. Notes may contain cross-references to other pertinent variables.

Recode: A variable derived from the reordering, collapsing, or verbatim coding of another variable. Alternatively, a recode may be constructed from two or more variables. All variables used to construct a recode are listed as a cross reference in Sources. Examples of recodes based on annual core content include MARSTAT_A (current marital status of the Sample Adult) and SMKCI GST_A (cigarette smoking status). Additionally, a recode may be created when a continuous variable needs top- and/or bottom-coding for confidentiality reasons (for example, SCHDYMSSTC_C, school days missed due to illness or injury in the past 12 months). In some years, the NHIS may also contain content on the frequency of a health behavior in two parts: number and type of time unit. Recodes are then created to combine and standardize responses into a single time unit.

Sources: If the variable is a recode, then all variables that were used to make that recode are listed as sources.

Universe: The group of adults or children to whom a specific question applies. For example, the universes for most Sample Adult variables are adults who were age 18 or over. This universe is specified on the Codebook report as HHSTAT_A=1. Sample Adults who are not eligible to answer a given question are considered to be not-in-universe. For example, Sample Adults who reported that they never had high cholesterol — CHLEV_A having a value of 2 or RF or DK — would not be eligible for a follow-up question CHL12M_A about whether they had high cholesterol in the past 12 months. Universes for many questions are often age specific. In the redesigned NHIS, missingness in the Sample Adult or Sample Child's age is possible, and in those few cases the individuals would be ineligible for the question. Note that during rostering, when a person's age is not known, there are age-related follow-up questions to get at whether they are adults or children so the Sample Adult or Sample Child selections can be made. If the age is still not known, the interview terminates. Similarly, the sex variable (SEX_A or SEX_C) also allows for missing values, but the interview can proceed. Sex-specific questions for the Sample Adult and the Sample Child are not in universe when sex is unknown.

Summary (PDF)

The summary report is a PDF document with a bookmarks panel on the left organized by module and section in questionnaire order, and a main panel that displays the variable list by section. When the section is selected in the bookmarks panel, the following information for all variables in that section is displayed in the main panel in tabular form. The lead-in header has the module name, the 3-letter section abbreviation, and the section description. The table has rows with the following information:

Question ID: if the variable is in the questionnaire, the unique ID for that variable is displayed. For recodes, the word "Recode" is displayed, and for any that are neither (e.g., identifiers), this column is blank. Question ID may change by survey year and should not be used for data management purposes across years.

Variable name: the name of the variable in the data. Generally, the variable name in the questionnaire has the same variable name in the dataset.

Source variables: for recodes, this column lists the names of variables used to create the recode.

Description: the variable label

Type: the data type for this variable, i.e., character or numeric

Location: the column range in the ASCII file (column numbers) where this variable is stored

Length: the length of the variable as a character data type

Codebook for restricted-use variables (PDF)

This document lists the restricted-use (or inhouse) variables that are available to analysts in the RDC. It does not include any variables that are in the public data files. The format is similar to the codebook, except that no frequencies are shown.

Public use data

The 2025 NHIS data release includes files for the annual Sample Adult, annual Sample Child, Imputed Income (as a poverty ratio) for the Sample Adult and Sample Child, Parent-child pair data, and Paradata.

For Sample Adult, Sample Child, Paradata, and Imputed Income, these files are available as both a column-delimited text (ASCII) file and a comma-separated values (CSV) file. The Parent-child pair data are available as CSV or SAS file.

Programs that contain input statements in SAS, Stata and SPSS environments are provided to help load the ASCII files into datasets with proper data formats and labels in the respective computing environments. Refer to Table 6 for lists the names of data files, programs and documentation in the 2025 NHIS data release.

Files corresponding to the 2025 NHIS have a two-digit suffix at the end that represents the survey year, e.g., 25 for 2025, or adult25. In years prior to 2019, separate files were available for household, family, and person level information due to the different survey design. In 2020, additional files were available for the Sample Adult (Partial and Longitudinal) due to the longitudinal component implemented that year but not continued thereafter. Imputed income files for Sample Adult and Sample Child can be merged with their respective Sample Adult and Sample Child files to create a single dataset (see, “Merging Survey Data and Imputed Income Files” in next section). Sample Adult and Sample Child files can also be merged with Paradata.

Variable Conventions. Variable labels are restricted to 80 characters due to limits in some programming languages. All variable names have a length of 15 characters or less. Variables names in the Sample Adult file have the suffix _A, e.g., DIBEV_A, to indicate that they refer to the Sample Adult or were asked of the Sample Adult’s family. Variables associated with the Sample Child will analogously have the suffix _C, e.g., DIBEV_C. Variables that do not have these suffixes are household or family level variables or identifiers, e.g., HHX.

Analyzing 2025 NHIS and Software Examples

To appropriately analyze NHIS data, it is necessary to utilize weights and variance estimation variables. This is because the NHIS uses a complex sample design involving stratification and clustering designed to represent the civilian noninstitutionalized population of the United States and not all sampled respondents respond. If data are not weighted, severely biased estimates may result, such as producing estimates that are not representative of the NHIS target population. If the correct variance estimation variables are not used, then estimates of precision, such as standard errors, will likely be smaller than they should be. This will make the data appear to be more precise and will result in more statistically significant differences between estimates and in other analyses that are subject to excessive Type I error (rejection of a true null hypothesis).

- **Analysts interested in combining or pooling public-use NHIS files from 2025 and 2024 (or another prior year since 2019) do not need to account for the different sample design in variance estimation.**
- Analysts interested in combining or pooling restricted files from 2025 and 2024 (or another prior year since 2019) and using restricted variance estimation variables (through the RDC) do need to account for the different survey design in variance estimation. Analysts using restricted variance estimation variables should differentiate the strata values (ISTRAT) between 2025 and the prior data year. Options include adding or multiplying one of the annual strata by a number or constant (e.g., ISTRAT+10,000) to make the strata values distinct by year or specifying two designs when declaring the variance variables in the analysis.

Applying Sample Weight and Variance Estimation Variables in Analysis

Several software packages are available for analyzing complex samples. Below are examples of computer code for specifying sample weight and variance estimation variables for standard error calculation code of means, percentages and totals with the NHIS data using SUDAAN, Stata, SPSS, SAS, and R software packages for illustrative purposes. The examples below use the Sample Adult sampling weight (WTFA_A) or for Sample Child analysis, use the Sample Child sampling weight (WTFA_C).

The limited public release design information requires a mathematical simplification that the PSUs be treated as if they were sampled with replacement (WR). The simplified design structure can be specified for the file with the following statements in selected software packages.

Example using SUDAAN

```
PROC <DESCRIPT, CROSSTAB, ...> ... DESIGN = WR;
  NEST PSTRAT PPSU;
  WEIGHT WTFA_A;
```

Note that SUDAAN requires that the input file be sorted by the variables listed on the NEST statement (i.e., PSTRAT and PPSU). Design statements for other data files should use the appropriate weight variables found on these files.

Example using SAS

*Examples for SAS proc surveymeans (for averages) or surveyfreq (for percentages/proportions);

```
PROC SURVEYMEANS DATA=<name of file> ;
STRATA PSTRAT;
CLUSTER PPSU;
WEIGHT WTFA_A;
VAR <name of variable to be analyzed>;
RUN;
```

```
PROC SURVEYFREQ DATA=<name of file>;
STRATA PSTRAT;
CLUSTER PPSU;
WEIGHT WTFA_A;
TABLES <name of variable to be analyzed>;
RUN;
```

Example using STATA

*Use Stata svy. *Stata is case-sensitive.

```
svyset [pweight=wtfa_a], strata(pstrat) psu(ppsu)
svy: mean <name of variable to be analyzed for average>
```

*or

```
svy: proportion <name of variable to be analyzed for percentage/proportion>
```

Example using SPSS

SPSS csdescriptives (for averages) or cstabulate (for percentages/proportions):

One needs first to define a “plan file” with information about the weight and variance estimation, and then refer to the plan file when using csdescriptives or cstabulate.

```
CSPLAN ANALYSIS
/PLAN FILE=“< file name >”
/PLANVARS ANALYSISWEIGHT=WTFA_A
/DESIGN STRATA=PSTRAT CLUSTER=PPSU
/ESTIMATOR TYPE=WR.
```

```
/PLAN FILE=“< file name >”
/SUMMARY VARIABLES =<name of variable to be analyzed>
/MEAN. CSTABULATE
/PLAN FILE=“< file name >”
```



```
/TABLES VARIABLES =<name of variable to be analyzed>  
/CELLS TABLEPCT.
```

Example using R

```
#R (including the “survey” add-on package). Note that R syntax is case-sensitive.  
# load survey package required (“survey”)  
# create data frame with NHIS design information, using existing data frame of NHIS data  
  
nhissvy <- svydesign(id=~PPSU, strata=~PSTRAT,  
  nest = TRUE,  
  weights=~WTFA_A,  
  data=< data file name>)  
  
svymean(~name of variable to be analyzed, design=nhissvy)  
  
# Note that svymean will produce proportions for “factor variables.” For details, consult the R  
documentation.
```

Examples of Analysis and Weighting Procedures for Sample Adults and Sample Children

Table 7 includes examples of types of research questions for which analysts may use 2025 Sample Adult and Sample Child NHIS data, and guidance on additional adjustments needed to the sampling weights when analyzing in combination with multiple NHIS years. Some of these analytic examples are interchangeable between the Sample Adult and Sample Child files, but analysts must use the respective Sample Adult and Sample Child weight.

Table 7. Examples of Analysis and Weighting Procedures for Sample Adults and Sample Children—2025 NHIS

Analytic Goal	Example	Weight Variable and Modifications Needed	Variance Estimation
Produce official estimates for 2025	Percentage of adults ever told by doctor or other health professional that they had diabetes	Use WTFA_A	Use standard variance estimation variables PSTRAT and PPSU for 2025
Compare estimates between 2025 and 2023	Percentage of children who ever live with anyone who had a problem with alcohol or drugs in 2025 compared with 2023	Use WTFA_C for both 2025 and 2023. Concatenate the 2025 and 2023 files. Weights do not require modification. Make sure to include the variable SRVY_YR (survey year) in your data files so that the two years can be compared.	Use standard variance estimation variables PSTRAT and PPSU for 2025 and 2023
Pooling 2025 and 2024 to increase sample size*	Percentage of children ages 2–17 who have Attention-Deficit/Hyperactivity Disorder or ADD or Attention-Deficit Disorder.	Concatenate the 2025 and 2024 files. Create a new weight using WTFA_C and divide it by 2	Use standard variance estimation variables PSTRAT and PPSU for 2025 and 2024
Produce estimates for topics available from January 2023 through December 2025 as 6-month intervals. Compare semi-annual intervals (trends).† The variable name for interview quarter changed in 2025.	Percentage of adults who smoke cigarettes everyday: January–June 2023, July–December 2023, January–June 2024, July–December 2024, January–June 2025, July–December 2025	For each 6-month interval, multiply the weight (WTFA_A) by 2.	Use standard variance estimation variables PSTRAT and PPSU for 2023, 2024 and 2025

* See scenario 1 for example using SAS, STATA, or R.

† See scenario 2 for example using SAS, STATA, or R.

Examples are not supported using SPSS. Consult SPSS documentation about analyzing complex survey data.

Additional examples are available in the 2022 Survey Description Document, section “Analyzing 2022 NHIS and Software Examples” available from https://ftp.cdc.gov/pub/Health_Statistics/NCHS/Dataset_Documentation/NHIS/2022/srvydesc-508.pdf.

Scenario 1: Concatenating 2025 and 2024 (pooling) Sample Child data to increase sample size

This example illustrates how to create a dataset that pools data from 2024 and 2025 to increase sample size and produce a cross-sectional estimate (e.g., Attention-Deficit/Hyperactivity Disorder or ADHD or Attention-Deficit Disorder or ADD). This question was asked about children ages 2–17. An analyst can pool 2024 and 2025 data when interested in increasing the precision of an estimate among a population that might have a small sample size with a single year of data. For this example, a new file called CHILD24_25 will include all Sample Children in 2024 and 2025. It will be derived from the data files, child24 and child25. This new file will only contain the

variables of interest: a doctor or other health professional diagnosed the Sample Child with ADHD/ADD, age of the child, variance structure variables, and sampling weights from the child24 and child25 files. The new weight WTFA_ADJ will be created to account for 2 years of data (i.e., that is dividing each sample weight by the number of years that are being pooled).

Analysts interested in pooling three or more years of data can follow the same steps for concatenating the additional data year(s). The new sampling weight variable MUST account for number of years of data pooled (i.e., that is dividing the sample weight by the number of years that are being pooled).

Example using SAS

```
*Create temporary datasets of 2025 and 2024 data by selecting weight, analytic variables and variance
structures (PSTRAT and PPSU);
DATA TEMPCHILD25;
SET CHILD25; *2025 Sample Child file;
KEEP WTFA_C PSTRAT PPSU ADHDEV_C AGEP_C;
RUN;

DATA TEMPCHILD24;
SET CHILD24; *2024 Sample Child file;
KEEP WTFA_C PSTRAT PPSU ADHDEV_C AGEP_C;
RUN;

*Concatenate the temporary 2025 and 2024 datasets;
DATA CHILD25_24;
SET TEMPCHILD25 TEMPCHILD24;
WTFA_ADJ=WTFA_C/2; *Divide the weight by 2;
RUN;
```

Example using Stata

```
*create temporary datasets of 2025 and 2024 data by selecting weight, analytic variables and variance
structures (pstrat and ppsu)
*child25 – 2025 sample child file
use child25
keep wtfa_c pstrat ppsu adhdev_c agep_c
save tempchild25

*child24 – 2024 sample child file
use child24
keep wtfa_c pstrat ppsu adhdev_c agep_c
save tempchild24

*concatenate temporary 2025 and 2024 datasets
append using tempchild25
gen wtfa_adj=.
```

```
*divide the weight by 2;
replace wtfa_adj=wtfa_c/2
save child25_24
```

Example using R

```
# keep only the selected variables
tempchild25 <- subset(child25, select = c(WTFA_C, PSTRA_T, PPSU, ADHDEV_C, AGEP_C))

# keep only the selected variables
tempchild24 <- subset(child24, select = c(WTFA_C, PSTRA_T, PPSU, ADHDEV_C, AGEP_C))

# concatenate the two temporary datasets
child25_24 <- rbind(tempchild25, tempchild24)

# create a new variable by dividing wtfa_c by 2
child25_24$wtfa_adj <- child25_24$WTFA_C / 2
```

Scenario 2: Comparing semi-annual intervals (trends):

Produce estimates of daily cigarette smokers from January 2023 through December 2025 as 6-month intervals

This example illustrates how to create one dataset to examine semi-annual intervals (trends) over a 36-month period of data collection. Data from 2023, 2024 and 2025 will be used in this example to produce cross-sectional 6-month interval estimates (e.g., daily cigarette smokers).

For this example, one new file will be created called NEWADULT23_25. This new file will only contain the variables of interest: smoking status of the Sample Adult (current every day smoker, current some day smoker, former smoker, never smoker, smoker with current status unknown, and unknown), variance structure variables, modified sampling weights from the adult23, adult24, and adult25 files and an indicator for each 6-month interval (TIME6M). For each 6-month interval, a NEWWT will be created by multiplying the weight (WTFA_A) by 2 to account for only using 6-months of data. Estimates in this dataset should only be examined by 6-month intervals, which is indicated by TIME6M variable (i.e., value 1 is the first 6-month period, value 2 is the second 6-month period, value 3 is the third 6-month period, value 4 is the fourth 6-month period, value 5 is the fifth 6-month period, and value 6 is the sixth 6-month period). The annual weight has been adjusted to treat 6 months of data so that weighted frequencies for a particular measure of interest will reflect the number of adults who were daily smokers in the U.S. civilian noninstitutionalized population. Using WTFA_A without adjustment for a 6-month analysis will result in population estimates that are too low and will not reflect the number of people in the U.S. civilian noninstitutionalized population with the outcome of interest.

Example using SAS

```
*Create temporary dataset of 2023 data by selecting weight, analytic variables and variance structures
(PSTRAT and PPSU);
DATA NEWADULT23;
SET ADULT23; *2023 Sample Adult file;
KEEP PSTRAT PPSU SMKCIGST_A NEWWT TIME6M;
IF INTV_QRT IN (1, 2) THEN TIME6M=1; *the first and second quarter of 2023, corresponding to the first 6-
month time period;
IF INTV_QRT IN (3, 4) THEN TIME6M=2; *the third and fourth quarter of 2023, corresponding to the second
6-month time period;
NEWWT = WTFA_A *2; *For 6-months of data, multiply the weight by 2;
RUN;
```

```
*Create temporary dataset of 2024 data by selecting weight, analytic variables and variance structures
(PSTRAT and PPSU);
DATA NEWADULT24;
SET ADULT24; *2024 Sample Adult file;
KEEP PSTRAT PPSU SMKCIGST_A NEWWT TIME6M;
IF INTV_QRT IN (1, 2) THEN TIME6M=3; *the first and second quarter of 2024, corresponding to the third 6-
month time period;
IF INTV_QRT IN (3, 4) THEN TIME6M=4; *the third and fourth quarter of 2024, corresponding to the fourth
6-month time period;
NEWWT = WTFA_A *2; *For 6-months of data, multiply the weight by 2;
RUN;
```

```
*Create temporary dataset of 2025 data by selecting weight, analytic variables and variance structures
(PSTRAT and PPSU);
DATA NEWADULT25;
SET ADULT25; *2025 Sample Adult file;
KEEP PSTRAT PPSU SMKCIGST_A NEWWT TIME6M;
/*In 2025, interview quarter is named INTV_QRT2 and quarter 4 has the value of 9*/
IF INTV_QRT2 IN (1, 2) THEN TIME6M=5; *the first and second quarters of 2025, corresponding to the fifth
6-month time period;
IF INTV_QRT2 IN (3, 9) THEN TIME6M=6; *the third and fourth quarters of 2025, corresponding to the sixth
6-month time period;
NEWWT = WTFA_A *2; *For 6-months of data, multiply the weight by 2;
RUN;
```

```
*Concatenate temporary 2023, 2024 and 2025 datasets;
```

```
DATA NEWADULT23_25;
SET NEWADULT23 NEWADULT24 NEWADULT25;
KEEP PSTRAT PPSU SMKCIGST_A NEWWT TIME6M;
RUN;
```

Example using Stata

```
*create temporary dataset of 2023 data by selecting weight, analytic variables and variance structures
(pstrat and ppsu)
use adult23
keep pstrat ppsu smkcigst_a intv_qrt wtfa_a
gen time6m=.
*the first and second quarter of 2023, corresponding to the first 6-month time period
replace time6m=1 if intv_qrt == 1 | intv_qrt == 2
*the third and fourth quarter of 2023, corresponding to the second 6-month time period
replace time6m=2 if intv_qrt == 3 | intv_qrt == 4
*for 6-months of data, multiply the weight by 2
gen newwt = wtfa_a * 2
drop wtfa_a intv_qrt
save newadult23
```

```
*create temporary dataset of 2024 data by selecting weight, analytic variables and variance structures
(pstrat and ppsu)
use adult24
keep pstrat ppsu smkcigst_a intv_qrt wtfa_a
gen time6m=.
*the first and second quarter of 2024, corresponding to the third 6-month time period
replace time6m=3 if intv_qrt == 1 | intv_qrt == 2
*the third and fourth quarters of 2024, corresponding to the fourth 6-month time period
replace time6m=4 if intv_qrt == 3 | intv_qrt == 4
*for 6-months of data, multiply the weight by 2
gen newwt = wtfa_a * 2
drop wtfa_a intv_qrt
save newadult24
```

```
*concatenate temporary 2023 and 2024 datasets
append using newadult23
keep pstrat ppsu smkcigst_a newwt time6m
save newadult23_24
```

```
*create temporary dataset of 2025 data by selecting weight, analytic variables and variance structures
(pstrat and ppsu)
*In 2025, interview quarter is named INTV_QRT2 and quarter 4 has the value of 9.
use adult25
keep pstrat ppsu smkcigst_a intv_qrt2 wtfa_a
gen time6m=.
*the first and second quarter of 2025, corresponding to the fifth 6-month time period
replace time6m=5 if intv_qrt2 == 1 | intv_qrt2 == 2
*the third and fourth quarter of 2025, corresponding to the sixth 6-month time period
replace time6m=6 if intv_qrt2 == 3 | intv_qrt2 == 9
*for 6-months of data, multiply the weight by 2
gen newwt = wtfa_a * 2
```

```
drop wtfa_a intv_qrt
save newadult25
```

```
*concatenate temporary 2023, 2024 and 2025 datasets
append using newadult23_24
keep pstrat ppsu smkcigst_a newwt time6m
save newadult23_25
```

Example using R

```
# Create temporary datasets of 2023 data
NEWADULT23 <- adult23[, c("PSTRAT", "PPSU", "SMKCIGST_A", "WTFA_A", "INTV_QRT")]
NEWADULT23 <- transform(NEWADULT23, NEWWT = WTFA_A * 2, TIME6M = ifelse(INTV_QRT %in% c(1,2),
1, ifelse(INTV_QRT %in% c(3,4), 2, NA)))
NEWADULT23 <- NEWADULT23[, c("PSTRAT", "PPSU", "SMKCIGST_A", "NEWWT", "TIME6M")]

# Create temporary dataset of 2024 data
NEWADULT24 <- adult24[, c("PSTRAT", "PPSU", "SMKCIGST_A", "WTFA_A", "INTV_QRT")]
NEWADULT24 <- transform(NEWADULT24, NEWWT = WTFA_A * 2, TIME6M = ifelse(INTV_QRT %in% c(1,2),
3, ifelse(INTV_QRT %in% c(3,4), 4, NA)))
NEWADULT24 <- NEWADULT24[, c("PSTRAT", "PPSU", "SMKCIGST_A", "NEWWT", "TIME6M")]

# Create temporary dataset of 2025 data
#In 2025, interview quarter is named INTV_QRT2 and quarter 4 has the value of 9.
NEWADULT25 <- adult25[, c("PSTRAT", "PPSU", "SMKCIGST_A", "WTFA_A", "INTV_QRT2")]
NEWADULT25 <- transform(NEWADULT25, NEWWT = WTFA_A * 2, TIME6M = ifelse(INTV_QRT2 %in%
c(1,2), 5, ifelse(INTV_QRT2 %in% c(3,9), 6, NA)))
NEWADULT25 <- NEWADULT25[, c("PSTRAT", "PPSU", "SMKCIGST_A", "NEWWT", "TIME6M")]

# Concatenate temporary 2023, 2024 and 2025 datasets
NEWADULT23_25 <- rbind(NEWADULT23, NEWADULT24, NEWADULT25)
NEWADULT23_25 <- NEWADULT23_25[, c("PSTRAT", "PPSU", "SMKCIGST_A", "NEWWT", "TIME6M")]
```

Merging Survey Data and Paradata Files

Data users can merge the Paradata file with the Sample Adult file (or the Sample Child file) to explore associations between a wide range of methodological measures and survey data. To merge 2025 files, use variable HHX as the unique identifier between the two files. Sample code in SAS, STATA and R is provided below to illustrate merging the Sample Adult file with the Paradata file.

Note that data files should be merged within the same year before combining (pooling) data files for multiple years.

Example using SAS

```
PROC SORT DATA=ADULT25;
BY HHX;
RUN;
PROC SORT DATA=PARADATA25;
BY HHX;
RUN;

/* creates a new file with Sample Adult and Paradata variables for each household */
DATA ADULT25_PLUS_PARA;
MERGE ADULT25 PARADATA25; BY HHX;
RUN;
```

Example using STATA

```
*change working directory to location of data files cd c:\nhis2025\

use adult25
sort hhx
save adult25, replace

use paradata25
sort hhx
save paradata25, replace

merge 1:1 hhx using adult25
save adultpara25
* creates a new file with sample adult and paradata variables for each household
```

Example using R

```
library(dplyr)

#Reading data from ADULT25 and sorting it by HHX
ADULT25 <- read.csv("ADULT25.csv")
ADULT25 <- arrange(ADULT25, HHX)

#Reading data from PARADATA25 and sorting it by HHX
PARADATA25 <- read.csv("PARADATA25.csv")
PARADATA25 <- arrange(PARADATA25, HHX)

#Merging the sorted datasets by HHX and creating a new dataset ADULT25_PLUS_PARA
ADULT25_PLUS_PARA <- right_join(ADULT25, PARADATA25, by = "HHX")
```

Merging Survey Data and Imputed Income Files

Data users can merge the Sample Adult file (or the Sample Child file) with their respective Imputed Income file to apply the imputation variable to the analyses for the appropriate calculation of standard error of the imputed variable. Variable HHX is the unique identifier between the two files.

The following code (in SAS, SUDAAN, STATA, and R) is for merging the Sample Adult data file and the Sample Adult imputed income file and conducting an analytic procedure (i.e., logistic regression using survey data). The variables used in this example were recoded as illustrated in Table 8. The analytic example, using the 2025 Sample Adult data file and the Sample Adult imputed income file, will examine the association of the variables RATCAT_A (the ratio of family income to the poverty threshold [*imputed*]), and WELLCHK (last doctor's visit was a wellness visit, recoded from WELLNESS_A) with HEALTH (having good-to-excellent health, recoded from PHSTAT_A).

Table 8. Variables in the imputed income example.

Survey question	Original variable name	Original values	Recoded variable Name	Recoded values
Imputation number	IMPNUM_A	Values 01-10	(not recoded)	(not recoded)
Randomly assigned household number unique to household	HHX	Range of unique of values	(not recoded)	(not recoded)
Would you say your health in general is excellent, very good, good, fair, or poor?	PHSTAT_A	1. Excellent 2. Very good 3. Good 4. Fair 5. Poor 7. Refused 8. Not ascertained 9. Don't know	HEALTH	1. Excellent, very good or good 0. Fair or poor . (missing)
Was the last doctor's visit a wellness visit, physical, or general-purpose check-up?	WELLNESS_A	1. Yes 2. No 7. Refused 8. Not ascertained 9. Don't know	WELLCHK	1. Yes 2. No . (missing)
Ratio of income to poverty threshold	RATCAT_A	1. 0.00–0.49 2. 0.50–0.74 3. 0.75–0.99 4. 1.00–1.24 5. 1.25–1.49 6. 1.50–1.74 7. 1.75–1.99 8. 2.00–2.49 9. 2.50–2.99 10. 3.00–3.49 11. 3.50–3.99 12. 4.00–4.49 13. 4.50–4.99 14. 5.00 or greater	(not recoded)	(not recoded)

Example using SAS

In SAS, analysis of multiple imputed data is conducted in two stages:

1. Analysis: each of the M imputed datasets is analyzed separately using any method that would have been selected had there been a single complete dataset. This includes analytical procedure in SAS, such as PROC GLM, PROC MIXED, PROC LOGISTIC, PROC FREQ, etc. In SAS, analysis of multiply imputed data is invoked with a "BY _IMPUTATION_" statement, to indicate that the same analysis is performed within each of the imputed datasets. Users need to rename the NHIS imputation number identifier IMPNUM_A to

_IMPUTATION.

2. Pooling: analysis results from M imputed datasets obtained from step 1 are combined into one overall result. This step can be carried out using SAS PROC MIANALYZE.

Import data files into SAS. See SAS input statements provided on the 2025 NHIS Data Release page.

This example uses the libname 'NHIS.'

/*The sample code below illustrates renaming the IMPNUM_A variable to _IMPUTATION_ for analyses in a new SAS dataset. */

```
DATA IMPINC;  
SET NHIS.ADULTINC25;  
RENAME IMPNUM_A= _IMPUTATION_; *SAS identifies imputed datasets by imputation_;  
RUN;
```

/* Next, merge the Sample Adult file and Sample Adult imputed Income file. Data files must be sorted by the common ID before they can be merged*/

```
PROC SORT DATA= IMPINC;  
BY HHX;  
RUN;
```

```
PROC SORT DATA=NHIS.ADULT25 OUT=ADULT25;  
BY HHX;  
RUN;
```

```
DATA NHIS25;  
MERGE ADULT25 (IN=A) IMPINC; *Merging the imputed income and the main dataset;  
BY HHX;  
IF A;  
RUN;
```

/* Sort the new dataset by imputation prior to analysis. Otherwise, your analyses will only show the first category of each variable for imputed analyses */

```
PROC SORT DATA= NHIS25; BY _IMPUTATION_;  
RUN;
```

/*The survey analytic procedure (PROC SURVEYLOGISTIC) is used to account for the complex sampling design of NHIS. */

/*WELLCHK is a recode from WELLNESS_A and HEALTH is a recode from PHSTAT_A*/

```
PROC SURVEYLOGISTIC DATA=NHIS25;  
STRATUM PSTRT;  
CLUSTER PPSU;  
WEIGHT WTFA_A;  
CLASS RATCAT_A WELLCHK (DESC);
```

```
MODEL HEALTH(EVENT='1') = RATCAT_A WELLCHK;
ODS OUTPUT PARAMETERESTIMATES=LGSPARMS ODDSRATIOS=LGSODDS;
BY _IMPUTATION_;
RUN;
```

/*The ODS datasets from the code above will contain a set of estimates for each imputed dataset identified by the variable _imputation_ included in each of them.

The MIANALYZE procedure combines the results of the analyses of imputed data and generates valid statistical inferences.*/

```
PROC MIANALYZE PARMS(CLASSVAR=CLASSVAL)=LGSPARMS;
CLASS RATCAT_A WELLCHK;
MODELEFFECTS RATCAT_A WELLCHK;
ODS OUTPUT PARAMETERESTIMATES=MIAN_LGSPARMS; *COMBINES THE RESULTS OF PREVIOUS
ANALYSES;
RUN;
```

/*The mian_lgsparms output shows parameters from the pooled imputed datasets.*/

Example using SAS-callable SUDAAN

SUDAAN reads in separate imputed datasets. To conduct analyses in SAS-callable SUDAAN, the following steps are taken:

1. Separate the multiply imputed NHIS.ADULTINC25 SAS data set into 10 individual imputed income datasets impinc1-impinc10.
2. Merge each imputed dataset with the main NHIS data file.

Import data files into SAS. See SAS input statements provided on the 2025 NHIS Data Release page.

This example uses the libname 'NHIS.'

```
PROC SORT DATA=nhis.ADULT25; *Sorting by HHX;
BY HHX;
RUN;
```

/* The following macro creates 10 separate imputation datasets from the NHIS.ADULTINC25 multiply imputed file. It then sorts them by the merge variable HHX and merges each with the NHIS.ADULT25 dataset. Ultimately 10 separate datasets are created for imputed analyses in SUDAAN */

```
%MACRO SEPARATE;
%DO I= 1 %TO 10; *Instructs SAS to do the procedure for 10 iterations;
DATA IMPINC&I; *CREATING 10 SEPARATE IMPUTED DATASETS;
SET NHIS.ADULTINC25;
WHERE IMPNUM_A= &I;
RUN;
```

```
PROC SORT DATA= IMPINC&I; *Sorting the 10 imputed datasets by HHX;  
BY HHX;  
RUN;
```

```
DATA NHIS25_&I; *Creating 10 separate analytic datasets;  
MERGE NHIS.ADULT25 IMPINC&I;  
BY HHX;
```

/* SUDAAN requires analytic datasets be sorted by the design/nest variables. These variables are PSTRAT and PPSU on the NHIS. */

```
PROC SORT DATA= NHIS25_&I; *SORTING THE IMPUTED DATASETS BY DESIGN VARIABLES;  
BY PSTRAT PPSU;  
RUN;  
  
%END;  
%MEND;  
%SEPARATE;
```

/*In SUDAAN, the option MI_COUNT indicates use of multiple imputed datasets. The associated numeral indicates the number of imputed datasets to be used in the analysis. Note that the data name used after the DATA= is the name of the first imputed dataset. This dataset name ends with the number 1. Variable HEALTH in this example is coded 0/1 */

```
PROC RLOGIST DATA = NHIS25_1 FILETYPE=SAS DESIGN=WR MI_COUNT=10 ;  
NEST PSTRAT PPSU / MISSUNIT;  
WEIGHT WTFA_A;
```

```
SUBGROUP WELLCHK ;  
LEVELS 2 ;
```

```
REFLEVEL WELLCHK=1 ;
```

```
MODEL HEALTH= RATCAT_A WELLCHK ;  
EFFECTS WELLCHK= (2 -1)/EXP NAME="WELLCHK Yes vs No" ;
```

```
PRINT / betas=default risk=default tests=default expcntrst=default t_betafmt=f7.2 waldffmt=f8.2  
dffmt=f10.0 orfmt=f5.2 loworfmt=f5.2 uporfmt=f5.2 exp_cntrstfmt=f13.2 low_cntrstfmt=f5.2  
up_cntrstfmt=f5.2;  
SETENV COLWIDTH=15 DECWIDTH=4 LABWIDTH=25 COLSPCE=1 TOPMGN=0;  
RLABEL HEALTH="In good-excellent health";  
RTITLE "Using LOGISTIC to Model good-excellent health";  
RUN;
```

Example using STATA

Import data files into Stata. See the sample Stata *.do* statements provided on the 2025 NHIS Data Release page.

```
*Change working directory to location of data files cd c:\nhis2025\

use adult25
sort hhx
* rename or drop the variable impnum_a included in this file because the imputed income file also has it
rename impnum_a impnumcopy1
save nhis25

use adultinc25
sort hhx
save impinc

use nhis25
merge 1:m hhx using "impinc"
append using "nhis25"

save nhis25_mi

*set data to mi svyset
mi import flong, m(impnum_a) id(hhx)

*set dataset to multiple imputation for survey

mi svyset [pweight=wtfa_a], strat(pstrat) psu(ppsu) singleunit(centered)
save nhis25_mi_dat

*mi describe will list the registration status of the variables.
*mi varying will report the varying*and super-varying variables.
*verify that all varying variables are registered as imputed or passive.

mi describe
mi varying
* logistic regression. the dependent variable, health, is coded as 0 or 1.
mi estimate: svy: logistic health ratcat_a wellchk
*odds ratios
mi estimate, or: svy: logistic health ratcat_a wellchk
```

Example using R

```
#Import the data and rename the variable
library(dplyr)
IMPINC <- ADULTINC25 %>% rename(IMPNUM_A = IMPNUM_A)

#Recode Variables
```

```

ADULT25$HEALTH <- ifelse(ADULT25$PHSTAT_A %in% c(1, 2, 3), 1,
  ifelse(ADULT25$PHSTAT_A %in% c(4, 5), 0, NA))

ADULT25$WELLCHK <- ifelse(ADULT25$WELLNESS_A %in% c(1,2), ADULT25$WELLNESS_A, NA)

#Sort the data by HHX
IMPINC <- IMPINC[order(IMPINC$HHX),]
ADULT25 <- ADULT25[order(ADULT25$HHX),]

#Merge the data by HHX
NHIS25 <- merge(ADULT25, IMPINC, by = "HHX", all.x = TRUE)

#Sort the data by IMPUTATION
NHIS25 <- NHIS25[order(NHIS25$IMPUTATION),]

# Survey design object
NHIS25 <- NHIS25 %>% rename(RATCAT_A = RATCAT_A.x)
survey_design <- svydesign(ids = ~PPSU, strata = ~PSTRAT, weights = ~WTFA_A, data = NHIS25, nest = TRUE)

# Survey logistic regression
logistic_model <- svyglm(HEALTH ~ RATCAT_A + WELLCHK, design = survey_design, family = binomial(link =
"logit"))

# Extract parameter estimates and odds ratios
lgsparms <- coef(logistic_model)
print (lgsparms)
lgsodds <- exp(lgsparms)
print (lgsodds)

# Multiple imputation (if applicable)
mi_results <- with(data = NHIS25, expr = glm(HEALTH ~ RATCAT_A + WELLCHK, family = binomial(link =
"logit")))
print(mi_results)

```

Appending Sample Adult and Sample Child Files

The 2025 Sample Adult and Sample Child can be appended to one another (i.e., add observations from different persons) to facilitate the analysis of measures that are common to both adults and children. An example of the need to combine observations or concatenate the Sample Adult and Sample Child files would be if the user is interested in generating an estimate of the U.S. civilian noninstitutional population of both children and adults or any subset of age ranges that includes both children and adults (e.g., ages 0 to 64 years).

To do so, data from the Sample Adult file and the Sample Child file should have comparable measures available. Since the names of the Sample Adult variables end in “_A” and the names of the Sample Child variables end in “_C”, comparable measures should be renamed to a common variable name. The Sample Adult and Sample Child weights should also be renamed to have the same variable name. The variance estimation variables have the same name for both the Sample Adult and Sample Child files, and no additional recoding and renaming is needed.

The following code illustrates the concatenation of the Sample Adult and Sample Child files for the purpose of generating an estimate of the U.S. civilian noninstitutional population who are uninsured, by age. The example code illustrates keeping variables of interest for the analysis and recoding them in order to generate the estimate of interest. The code does not include analytic procedures for generating the estimate. The variables used in this example are illustrated in Table 9.

Table 9. Variables in example concatenating the Sample Adult file and the Sample Child

Variable description	Variable name in the Sample Adult file	Variable name in the Sample Child file	Variable name in new file
Indicates person is the Sample Adult; Indicates person is the Sample Child	HHSTAT_A	HHSTAT_C	HHSTAT_A and HHSTAT_C
Pseudo-stratum for public-use file variance estimation	PSTRAT	PSTRAT	PSTRAT
Pseudo-PSU for public-use file variance Estimation	PPSU	PPSU	PPSU
Weight - Final Annual	WTFA_A	WTFA_C	WTFA_NEW
Coverage status as used in Health United States	NOTCOV_A	NOTCOV_C	NOTCOV
Age of Sample Adult (top coded); Age of Sample Child	AGEP_A	AGEP_C	AGE

Example using SAS

Create a new file with all Sample Adult and all Sample Child records. Keep variance and sample weights, common variables in both files and new recodes combining key variables.

```
DATA PERSON25;
SET ADULT25 CHILD25;
KEEP PSTRAT PPSU WTFA_A WTFA_C HHSTAT_A HHSTAT_C NOTCOV_A AGEP_A NOTCOV_C AGEP_C
WTFA_NEW NOTCOV AGE;

/* recodes*/
IF HHSTAT_A=1 THEN DO;
WTFA_NEW=WTFA_A;
NOTCOV=NOTCOV_A;
AGE = AGEP_A;
END;

IF HHSTAT_C=1 THEN DO;
WTFA_NEW=WTFA_C;
NOTCOV=NOTCOV_C;
AGE = AGEP_C;
END;
RUN;
```


Example using STATA

*Create new separate files for the Sample Adult and Sample Child with the variables for variance, sample weight, and common variables of interest.

*Combine files and recode combining key variables.

```

use child25
keep notcov_c hhstat_c ppsu pstrat wtfa_c agep_c
save childvars

use adult25
keep notcov_a hhstat_a ppsu pstrat wtfa_a agep_a
save adultvars

append using childvars

*recodes
gen notcov=.
replace notcov=1 if notcov_c==1 | notcov_a==1
replace notcov=2 if notcov_c==2 | notcov_a==2

gen age=.
replace age=agep_c if hhstat_c==1
replace age=agep_a if hhstat_a==1

gen wtfa_new=.
replace wtfa_new= wtfa_c if hhstat_c==1
replace wtfa_new= wtfa_a if hhstat_a==1
save vars_child_adult

```

Example using R

```

# read in data from ADULT25 and CHILD25
PERSON25 <- merge(ADULT25, CHILD25, all=TRUE)

# keep the desired variables
PERSON25 <- subset(PERSON25, select = c("PSTRAT", "PPSU", "WTFA_A", "WTFA_C",
    "HHSTAT_A", "HHSTAT_C", "NOTCOV_A",
    "AGEP_A", "NOTCOV_C", "AGEP_C"))

# create new variables based on the HHSTAT_A and HHSTAT_C variables

PERSON25 <- PERSON25 %>%mutate(NOTCOV = case_when(HHSTAT_A == 1 ~ NOTCOV_A, HHSTAT_C == 1 ~
    NOTCOV_C, TRUE ~ NA))

```

```

PERSON25 <- PERSON25 %>%mutate(AGE = case_when(HHSTAT_A == 1 ~ AGEP_A,HHSTAT_C == 1 ~
AGEP_C,TRUE ~ NA))

PERSON25 <- PERSON25 %>%mutate(WTFA_NEW = case_when(HHSTAT_A == 1 ~ WTFA_A,HHSTAT_C == 1 ~
WTFA_C,TRUE ~ NA))

# keep the desired variables
PERSON25 <- subset(PERSON25, select = c("PSTRAT", "PPSU", "WTFA_NEW", "NOTCOV", "AGE"))

```

Parent-Child Pair Analysis

A growing interest in the simultaneous analysis of NHIS data collected from parent-child pairs necessitated the development of appropriate analytic weights (Zhang et al, 2024). Sampling weights for adult-child pairs were created for domain estimation methods (that is, statistical analysis for subpopulations or subgroups) to produce estimates separately for “mother-child” or “father-child” pairs since the NHIS does not collect data from parents-child groups, i.e., triad data.

Available sampling weights for adult-child pairs are year-specific and found in a separate file in the annual release page (<https://www.cdc.gov/nchs/nhis/documentation/index.html>), under “Parent-child pair data.” Data users interested in analyzing parent-child pair data need to link the file “sampling weights for adult-child pairs” to the Sample Adult and Sample Child datasets for the same data year, using the Household ID, named HHX. After linking the files, data users can categorize mother-child, father-child, and non-parent-child pairs using variables SAPARENTSC_A (Sample Adult relationship to Sample Child) and SEX_A (Sex of Sample Adult).

All analyses of parent-child pairs need to be of either the mother-child or the father-child pair in a subsetted data analysis. “Parent-child” analyses are not feasible using the NHIS pair data because only dyads were sampled—and a parent-child analysis necessitates knowledge of both parents (in two-parent households). When categorizing mother-child, father-child, and non-parent-child pairs, the category “non-parent-child pairs” should be included in the dataset for analysis, but it may not be meaningful to examine outcomes for this group. This is because the non-parent-child pair group is highly heterogenous, as it may include a mix of relatives (e.g., adult sibling, grandparent, uncle) and nonrelatives.

The pair weight variable is called FINAL_PAIR_WEIGHT and should be used in the analysis of NHIS mother-child or father-child pair data. These weights need to be used when data from a mother or father (obtained from the Sample Adult file) are being incorporated in a child-level analysis as an exposure or independent variable. Similarly, these weights need to be used when data from a child (obtained from the Sample Child file) are being incorporated in an adult-level analysis as an exposure or independent variable. Finally, these weights need to be used when the analytic focus is that of joint child-mother or child-father outcomes.

Table 10 describes different research scenarios using NHIS data and the appropriate weight to use.

Table 10. Examples of research scenarios and the appropriate weight to use in your analysis.

	Sample Adult Weight	Sample Child Weight	Pair Weight
Prevalence of Sample Adult variable (for example, diabetes) that is adjusted for family-level characteristics (such as income, number of children)	✓		
Prevalence of Sample Child variable (for example, autism) that is adjusted for family-level characteristics (e.g. income, number of children)		✓	
Prevalence of Sample Adult variable (for example, depression) that is adjusted by whether the Sample Child has a health outcome (e.g. diabetes)			✓
Prevalence of Sample Child Health variable (for example, anxiety) that is adjusted by whether the Sample Adult has a health outcome (e.g. cancer)			✓
Prevalence of Mother-Child Pairs where both individuals have a health condition (e.g. asthma)			✓
Prevalence of Mother-Child Pairs where both individuals have a health condition (e.g. asthma), adjusting for family-level characteristics			✓
Prevalence of Mother-Child Pairs where both individuals have a health condition (e.g. asthma), adjusting for characteristics of both the Sample Adult and Sample Child			✓

Note. The same guidance holds when the analytic subgroup of interests are father-child pairs.

The pair weights should not be used in statistical analyses focusing exclusively on *all* Sample Adults or *all* Sample Children; such analyses require the use of the sampling weights included in the Sample Adult or Sample Child files (WTFA_A or WTFA_C, respectively). Although the Sample Adult (and Sample Child) sampling weights are correlated with the pair weights, **pair-level statistical analyses should use the pair-level sampling weights (FINAL_PAIR_WEIGHT) as this incorporates the sampling probabilities of both the Sample Adult and the Sample Child and are adjusted for pair-level nonresponse.** Using the WTFA_A or WTFA_C sampling weight in parent-child pair analysis is incorrect and can lead to biased results. **Use the stratum and primary sampling unit variables PSTRAT and PPSU for variance estimation.** Variables PSTRAT and PPSU can be retained from either the Sample

Adult or Sample Child file as these values will be same in parent–child pairs. For a report about the development of the sampling weights, with example analyses, see Zhang et al, 2024, and available at https://www.cdc.gov/nchs/data/series/sr_02/sr02-207.pdf.

The following code (adapted from Zhang et al, 2024) illustrates the file creation of an analysis that requires the use of the “pair-weight,” with an example to examine health status of parent-child pairs.

Example using SAS

```
*Create files and prepare to merge them;
*file with parent-child weights ;
DATA FINAL_WEIGHT25;
SET FINAL_PAIR_WEIGHT25;
ELIGIBLE_FAMILYID=1;
KEEP HHX FINAL_PAIR_WEIGHT ELIGIBLE_FAMILYID;
RUN;

PROC SORT DATA=FINAL_WEIGHT25;
BY HHX;
RUN;

*Sample Adult file for 2025;
DATA ADULT25new;
SET ADULT25;
KEEP HHX PSTRAT PPSU SAPARENTSC_A PHSTAT_A SEX_A;
RUN;

PROC SORT DATA=ADULT25new;
BY HHX;
RUN;

*Sample Child file for 2025;
DATA CHILD25new;
SET CHILD25;
KEEP HHX PHSTAT_C;
RUN;

PROC SORT DATA=CHILD25new;
BY HHX;
RUN;

*Merge the sample adult, sample child and pair weight files together;
DATA ADULTCHILDPAIR25;
MERGE FINAL_WEIGHT25 ADULT25new CHILD25new;
BY HHX;
IF ELIGIBLE_FAMILYID=1;
IF SAPARENTSC_A=1 THEN PARENT_CHILD=1;
```

```

ELSE PARENT_CHILD=0;

*create pairs;
IF PARENT_CHILD=1 THEN DO;
    IF SEX_A=1 THEN Z=2; *father-child;
    IF SEX_A=2 THEN Z=1; *mother-child;
END;

IF PARENT_CHILD=0 THEN Z=3; *non-parent-child;

IF PHSTAT_A in (7 9 ) then PHSTAT_A=.;
IF PHSTAT_C in (7 9 ) then PHSTAT_C=.;

IF PHSTAT_A ^= . and PHSTAT_C ^= . then do;
IF PHSTAT_A in (1 2 3) and PHSTAT_C in (1 2 3 ) then HEALTH_COMPOSITE =1 ; *
Both members of dyad in at least good health;
ELSE HEALTH_COMPOSITE =2 ; *at least one member has fair, poor health;
END;

RUN;

*This is an example of generating health status of the parent-child pairs;

PROC SURVEYFREQ DATA= ADULTCHILDPAIR25;
STRATUM PSTRAT;
CLUSTER PPSU;
WEIGHT FINAL_PAIR_WEIGHT;
TABLE Z*HEALTH_COMPOSITE/ ROW CL NOCELLPERCENT;
RUN;

```

Example using STATA

```

*Create files and prepare to merge them
*File with parent-child weights ;
use final_pair_weight2025
generate eligible_familyid=1
sort hhx
save final_weight25.dta

*Sample Adult file;
use adult25.dta
keep hhx pstrat ppsu saparentsc_a phstat_a sex_a
sort hhx
save adult25new.dta

*Sample Child file;
use child25.dta
keep hhx phstat_c

```

```

sort hhx
save child25new.dta

*Merge the sample adult, sample child and pair weight files together
merge 1:1 hhx using final_weight25
drop _merge
merge 1:1 hhx using adult25new.dta
*delete unmatched pairs
keep if _merge==3
drop _merge

*create pairs
generate parent_child=1 if saparentsc_a==1
replace parent_child=0 if saparentsc_a~=1
label variable parent_child "parent-child pair"

generate z=1 if parent_child==1 & sex_a==2
*mother-child pair
replace z=2 if parent_child==1 & sex_a==1
*father-child pair
replace z=3 if parent_child==0
*nonparent-child pair
label variable z "type of parent-child pair"

generate chealth=1 if phstat_c==1 | phstat_c==2 | phstat_c==3
replace chealth=0 if phstat_c==4 | phstat_c==5
label variable chealth "child in at least good health"

generate ahealth=1 if phstat_a==1 | phstat_a==2 | phstat_a==3
replace ahealth=0 if phstat_a==4 | phstat_a==5
label variable ahealth "adult in at least good health"

generate health_composite=1 if chealth==1 & ahealth==1
replace health_composite=0 if chealth==0 | ahealth==0
label variable health_composite "both members of the family are in at least good health"

*mom-child pair indicator
gen mom_child_pair=1 if parent_child==1 & sex_a==2
replace mom_child_pair=0 if parent_child==1 & sex_a==1
label variable mom_child_pair "child pair "

*dad-child pair indicator
gen dad_child_pair =1 if parent_child==1 & sex_a==1
replace dad_child_pair =0 if parent_child==1 & sex_a==2
label variable dad_child_pair "father-child pair"
save adultchildpairs25.dta

svyset [pweight=final_pair_weight], psu(ppsu) strata(pstrat) singleunit(centered)

svy, subpop(mom_child_pair): proportion health_composite

```

svy, subpop(dad_child_pair): proportion health_composite

*or

svy: tab z health_composite, ci row

Example using R

```
library(dplyr)
library(survey)
library(haven)

# load data and create a file with pair weights

final_weight25 <- final_pair_weight2025 %>% mutate(eligible_familyid = 1) %>% select(hhx,
final_pair_weight, eligible_familyid)

# sort by hhx
final_weight25 <- final_weight25 %>% arrange(hhx)

# create adult25 and child25
adult25 <- adult25 %>% select(hhx, pstrat, ppsu, saparentsc_a, phstat_a, sex_a)
child25 <- child25 %>% select(hhx, phstat_c)

# sort by hhx for adult25 and child25
adult25 <- adult25 %>% arrange(hhx)
child25 <- child25 %>% arrange(hhx)

# merge final_weight25, adult25, and child25 by hhx and filter eligible_familyid = 1
adultchildpair25 <- final_weight25 %>% left_join(adult25, by = "hhx") %>% left_join(child25, by = "hhx")
%>% filter(eligible_familyid == 1)

# create parent_child variable based on saparentsc_a
adultchildpair25 <- adultchildpair25 %>% mutate(parent_child = ifelse(saparentsc_a == 1, 1, 0))

# create z variable based on parent_child and sex_a
adultchildpair25 <- adultchildpair25 %>% mutate(z = case_when(
  parent_child == 1 & sex_a == 1 ~ 2, # father-child
  parent_child == 1 & sex_a == 2 ~ 1, # mother-child
  parent_child == 0 ~ 3 # non-parent-child
))

# replace phstat_a and phstat_c values of 7 and 9 with NA
adultchildpair25 <- adultchildpair25 %>%
  mutate(phstat_a = ifelse(phstat_a %in% c(7, 9), NA, phstat_a), phstat_c = ifelse(phstat_c %in% c(7, 9), NA,
phstat_c))

# create health_composite variable based on phstat_a and phstat_c
adultchildpair25 <- adultchildpair25 %>%
```

```

mutate(
  health_composite = case_when(
    !is.na(phstat_a) & !is.na(phstat_c) &
    phstat_a %in% c(1, 2, 3) &
    phstat_c %in% c(1, 2, 3) ~
    "both members of dyad in at least good health",

    !is.na(phstat_a) & !is.na(phstat_c) ~
    "at least one member has fair, poor health",

    TRUE ~ NA_character_
  )
)

# handle cases where phstat_a or phstat_c is na

# ensure z and health_composite are factors
adultchildpair25$z <- factor(adultchildpair25$z)
adultchildpair25$health_composite <- factor(adultchildpair25$health_composite)
adultchildpair25 <- adultchildpair25 %>% filter(!is.na(health_composite))

survey_design <- svydesign(ids = ~ppsu, strata = ~pstrat, weights = ~final_pair_weight, data =
adultchildpair25, nest = TRUE)

# generate the frequency table for z and health_composite
freq_table <- svytable(~z + health_composite, design = survey_design)
row_percentages <- prop.table(freq_table, margin = 1) * 100
print(row_percentages)

# calculate the weighted total for each category in z
total_z <- svytotal(~z, design = survey_design)
print(total_z)

# calculate the weighted total for health_composite
total_health_composite <- svytotal(~health_composite, design = survey_design)

print(total_health_composite)

```

Variance Estimation for Subsetted Data Analysis

Frequently, analyses using NHIS data are restricted to specific population subgroups (e.g., persons age 65 and older). NCHS recommends that subpopulation analyses be carried out using the full data file and the SUBPOPN statement in SUDAAN, or an equivalent procedure with another complex design variance estimation software package.

Some users delete all records outside of the domain of interest (e.g., persons less than 65 years) in order to work with smaller data files and run computer jobs more quickly. This procedure of keeping only selected records (and list-wise deleting other records) is called subsetting the data. With a subsetted dataset that is appropriately

weighted, correct point estimates (e.g., estimates of population subgroup means) can be produced. However, in general, software packages that correctly analyze complex survey data cannot compute accurate standard errors for subsetting data. When complex survey data are subsetted, often the sample design structure available to the software is incomplete; subsetting data deletes important design information needed for variance estimation.

Analyses of large NHIS subgroups usually produce reliable estimates, but analyses of small subgroups may yield unreliable estimates, as indicated by their larger variances. The analyst should pay attention to the coefficient of variation (relative standard error) for estimates of means, proportions, and totals. In addition, small sample sizes, or small numbers of primary sampling units containing targeted data, may be an indication of estimates lacking precision.

Below are examples for subsetting NHIS data using SUDAAN, Stata, SPSS, SAS, and R software packages for illustrative purposes. The following code is to subset the second category for each variable RACEALLP_A and SEX_A, which happens to be the value “2” in both cases in this example. These are Sample Adult variables for race and sex where RACEALLP_A=2 is Black or African American only and SEX_A=2 is female.

Example using SUDAAN

SUDAAN has a SUBPOPN statement that allows the targeting of a subpopulation while using the full (unsubsetting) data file containing the design information for the entire sample.

Strategy 1 (recommended)

Use the SUBPOPN statement with the SUDAAN method described above for the full Sample Adult dataset:

```
PROC <add procedure, file name, filetype> .DESIGN WR;
  NEST PSTRAT PPSU;
  WEIGHT WTFA _A;
  SUBGROUP <add variable names>;
  LEVELS <add levels of variables>;
  SUBPOPN RACEALLP_A=2 & SEX_A=2 / NAME="Analysis of Black or African American women;"
```

Using the full dataset with the SUBPOPN statement in this example would constrain this analysis to Black or African American women only (RACEALLP_A = 2 for Black or African American and SEX_A = 2 for female). Use of the SUBPOPN statement is equivalent to subsetting the dataset, except that any resulting variance estimates are based on the full design structure for the complete dataset.

Strategy 2 (not recommended, except when Strategy 1 is infeasible)

Use the MISSUNIT option on the NEST statement with the method described above for subsetting data:

```
NEST PSTRAT PPSU / MISSUNIT;
```

In a WR design, when some PSUs are removed from the database through the listwise deletion of records outside the population of interest, leaving only one PSU in one or more strata, the MISSUNIT option in SUDAAN “fixes” the estimation to avoid errors due to the presence of strata with only one PSU. In the special case of a WR design with exactly two PSUs per stratum, using the MISSUNIT option with subsetting data gives the same variance estimate as using Strategy 1. However, except for this special case, there is no guarantee that the

variance estimates obtained by this method are equivalent to those obtained using Strategy 1. Other calculations, such as those for design effects, degrees of freedom, standardization, etc., may need to be carried out differently.

Example using SAS

*SAS proc surveymeans or surveyfreq

*One must first define an indicator variable, e.g.:

```
IF RACEALLP_A=2 & SEX_A=2 THEN SUBGRP=1; ELSE SUBGRP=0;
```

*And then refer to the indicator variable in proc surveymeans using the DOMAIN statement, e.g.:

```
PROC SURVEYMEANS DATA <file name>; DOMAIN SUBGRP;
```

*Proc surveyfreq does not have a DOMAIN statement. Instead, include the indicator variable in the TABLES specification:

```
PROC SURVEYFREQ <file name>;
```

```
TABLES SUBGRP*<name of variable to be analyzed>;
```

*This will produce tables for all values of the SUBGRP variable. It is very important that the indicator variable is defined for all data records, otherwise an invalid result can occur.

Example using STATA

**Use Stata svy

*Add SUBPOP to the SVY statement and must first define an indicator variable for the subpopulation.

```
svy, subpop(<name of variable for suppopulation>): mean <name of variable to be analyzed>
```

Example using R

R (including the “survey” add-on package)

After applying the svydesign function to a data frame that contains the entire NHIS sample file being analyzed, specify the criteria that define the subgroup of interest in the subset function and apply the function to the R “object” created by the svydesign function to create a new R object. Note that the syntax that follows specifies the subgroup of interest without using an equality test.

```
# subset for raceallp_a=2 & sex_a=2
```

```
subgrp <- subset(nhisvy, raceallp_a==2 & sex_a==2)
```

```
svymean(~name of variable to be analyzed, design=subgrp)
```

Note that users may want to recode variables such that missing values (which have numeric codes greater than 1) are not treated as real values. For example, sex>1 would include missing codes 7, 8 and 9 (don’t know, refused, not ascertained, respectively).

Example using SPSS

SPSS csdescriptives or cstabulate

One must first define an indicator variable, e.g.:

DO IF (RACEALLP_A EQ 2 AND SEX_A EQ 2).

COMPUTE SUBGRP=1.

ELSE.

COMPUTE SUBGRP=0.

END IF.

And then refer to the indicator variable in csdescriptives or cstabulate, e.g.: CSDESCRIPTIVES (or CSTABULATE)

/SUBPOP TABLE=SUBGRP

It is very important that the indicator variable be defined for all data records. Otherwise, an invalid result can occur.

Questionnaire and Codebook Section Acronyms

Table 11. Acronym definition of sections in the questionnaire and codebooks: 2025 NHIS

Section	Section Description
ADH	Attention Deficit/Hyperactivity Disorder
ADO	Age of Disability Onset
ANX	Anxiety
ASP	Aspirin
AST	Asthma
BCK	Back
BEH	Behavior
BMI	Current pregnancy, height, weight
BSC	Baby Pediatric Symptom Checklist
CAN	Cancer
CAR	Caregiving
CHL	Cholesterol
CIG	Cigarettes and E-cigarettes
CIV	Civic Engagement
COG	Cognition
COM	Communication
CON	Other Chronic Conditions
COV	Coverage
CVC	Cardiovascular Conditions
CVL	Long-COVID
CVV	COVID-19 vaccination
DEP	Depression
DIB	Diabetes
DLD	Developmental and Learning Disabilities
DNC	Dental Care
EMP	Employment
EPI	Epilepsy
FAM	Family composition
FEM	Employment of family members
FLG	Flag
FOO	Food Related Programs
FRT	Front
GAD	GAD-7 Anxiety
GEN	General
HEA	Hearing
HHC	Household composition
HIS	Health Status
HIT	Internet Access & Health Information Technology

Table 11 continued. Acronym definition of sections in the questionnaire and codebooks: 2025 NHIS

Section	Section Description
HOU	Housing
HYP	Hypertension
IDN	Identifier
IMM	Immunization (Sample Child)
IMS	Immunization (Sample Adult)
INC	Family Income
INS	Health Insurance
LNG	Language Spoken at Home
LNK	Linkage
LSF	Life Satisfaction
MAR	Marital Status
MHC	Mental health care
MNP	Menopause
MOB	Mobility
NAT	Nativity
ORN	Sexual Orientation
OTB	Other Tobacco
PAI	Chronic Pain
PAR	Parent Demographics
PAY	Difficulty Paying for Health Care
PHQ	PHQ-8 Depression
PMD	Prescription Medication
PRV	Preventive screening
PTC	Physical and other therapeutic care
REC	Child's full name
REL	Relationship of children to parents
ROS	Roster
SCH	Schooling
SCI	Social Connectivity and Isolation
SDQ	Child Mental Health - SDQ Questionnaire
SLE	Stressful Life Events
SOC	Social Functioning
SPS	Social Parenting Support
SSL	Social Support and Loneliness
TBI	Concussions – past 12 months
TEL	Telephone use (Sample Adult)
TEL	Telephone ownership (Sample Child)
THX	Thanks
TLH	Telehealth
UCF	Unit control file

Table 11 continued. Acronym definition of sections in the questionnaire and codebooks: 2025 NHIS

Section	Section Description
UPP	Upper body, motor skills and self-care (Sample Child)
UPP	Self-care and upper body (Sample Adult)
UTZ	Utilization
VET	Veterans Status
VFY	Sample Adult verification
VFY	Verification and demographic details (Sample Child)
VIS	Vision
VOL	Volunteer Activities
VSL	Voice, Swallowing, Speech, and Language Communication Disorders
WPH	Whole Person Health

Sample Adult's Health

I. Health Status and Conditions

Annual Core

Several sections throughout the Sample Adult module measure the health of U.S. adults. Sample Adults were asked to self-report their height and weight, self-perceived health status, current pregnancy status for females ages 18–49 years, and whether a doctor or other health care professional had told them that they had series of selected conditions. Estimates derived from questions that ask about specific health conditions diagnosed by a doctor or health care professional may underestimate the true burden of these conditions in the population due to the undiagnosed status of the condition during its detectable pre-clinical and clinical phase and from reporting bias. For a list of health conditions measured in the annual core, by questionnaire section and reference periods of its respective questions, see Table 12. Sample Adults were also asked about the intake of medication to treat diabetes, hypertension and high cholesterol, type of diabetes, and visits to an emergency room due to asthma, if ever diagnosed with these respective conditions. Age of diagnosis was collected from Sample Adults who were ever diagnosed with diabetes and cancer.

Table 12. Annual core content measures of health conditions about the Sample Adult, by questionnaire section and reference periods.

Topic	Section	Reference period in available questions
Angina pectoris	CVC	Ever
Anxiety disorder	CON	Ever
Arthritis, rheumatoid arthritis, gout, lupus, or fibromyalgia	CON	Ever
Asthma	AST	Ever; Past 12 months; Current
Cancer and cancer kind	CAN	Ever
Chronic obstructive pulmonary disease (COPD)	CON	Ever
Coronary heart disease	CVC	Ever
Dementia, including Alzheimer's disease	CON	Ever
Depression	CON	Ever
Diabetes	DIB	Ever
Gestational diabetes	DIB	Ever
Health status	HIS	Current
Height and weight	BMI	Current; If pregnant, before pregnancy
Hyperlipidemia	CHL	Ever; Past 12 months
Hypertension	HYP	Ever; Past 12 months
Myocardial infarction	CVC	Ever
Pre-diabetes	DIB	Ever
Pregnancy status	BMI	Current
Stroke	CVC	Ever

Sponsored Content

Attention Deficit/Hyperactivity Disorder. NCBDD sponsored five questions in the Sample Adult ADH section about Attention-Deficit/Hyperactivity Disorder (ADHD). Sample Adults were asked whether a doctor or other health professional had told them that they had ADHD and if they currently have it. Adults with ADHD were also asked their age at diagnosis, whether they currently take prescription medication for ADHD, and whether they received counseling, therapy, or behavioral treatment for ADHD in the past 12 months. These questions allow for estimates of both lifetime and current ADHD among adults and can be used to examine associations with other health outcomes. Similar questions are also asked of Sample Children to allow total population estimates of both lifetime and current ADHD.

Communication Disorders. NIDCD sponsored content in the VSL section related to sensory or communication disorders to help track Healthy People 2030 Objectives and to update national estimates. Sample Adults were asked about difficulty in the past 12 months with voice (i.e., hoarse, strained voice, or trouble speaking loud enough), swallowing, speech (i.e., stuttering, trouble pronouncing words), and language (i.e., using, learning, or understanding words). For each of the communication disorders experienced, follow-up questions asked for the number of days they had this and the level of difficulty experienced. An additional question asked those with a speech problem if this included stuttering, trouble pronouncing words correctly, or both. Sample Adults who experienced any of the communication problems were also asked if these difficulties followed a brain injury or stroke, if they had seen a speech-language pathologist or specialist, and if the disorder had improved or gotten worse in the past 12 months. Those who had seen a speech-language pathologist or specialist were also asked how the health care services received had affected their lives. These questions were previously fielded in 2022. Similar content is also available for the Sample Child.

COVID-19 Infection. The CVL section includes items about COVID infection and Long COVID symptoms sponsored by NCIRD. Sample Adults were asked about ever having COVID-19. Adults with a history of COVID-19 were asked if they had COVID-19 symptoms lasting for three or more months that they didn't have before the infection (Long COVID), whether they currently had symptoms, and if Long COVID symptoms had reduced their ability to carry out day-to-day activities compared with the time before they had COVID-19. These questions have been fielded since 2023, and in 2024, the variable name for the question on Long COVID symptoms was updated to reflect a minor change in the "read if necessary" statement about examples of symptoms. Similar questions about COVID-19 infection were asked of Sample Children.

Diabetes (insulin). NIH/NIDDK and NCCDPHP sponsored three questions in the Sample Adult DIB section. Sample Adults diagnosed with diabetes and are now taking insulin were asked how long after diagnosis they began taking insulin, whether it was discontinued for more than 6 months after initiating it, and whether this occurred during the first year of diabetes diagnosis. These insulin questions can inform about patterns of insulin use and have been used in algorithms to classify diabetes type (Casagrande et al., 2020). These questions have been fielded since 2019.

Diabetes -Other injectable medication and feeling overwhelmed. NCCDPHP sponsored two questions in the DIB section. Sample Adults diagnosed with prediabetes or diabetes were asked if they are currently taking any injectable medications to lower their blood sugar or to lose weight. A "read if necessary" statement included the names of injectable GLP-1 medications. This question was also fielded in 2024. GLP-1s are new types of medicine for people with type 2 diabetes to help manage blood sugar, help with weight loss and protect the heart and kidneys (CDC, 2024). Sample Adults who reported having been told by a doctor or health professional that they had

diabetes were asked how often they felt overwhelmed by the demands of living with diabetes. This question was previously fielded in 2021.

Epilepsy. NCCDPHP sponsored four questions on epilepsy in the Sample Adult EPI section. Sample Adults were asked whether they had ever been told by a doctor or health professional that they had a seizure disorder or epilepsy, and those diagnosed with the disorder were also asked, whether they were currently taking antiseizure medication, the number of seizures in the past year, and whether they saw a neurologist or epilepsy specialist in the past year. These questions were fielded during 2021-2022, and 2025 to provide national and subpopulation estimates of epilepsy prevalence.

Live birth. NCIRD sponsored a question for female Sample Adults between 18 and 49 years old (or whose age was not known) that asked if they had a live birth in the past 12 months. This question was fielded in the IMS section in 2019–2024 and moved to the BMI section in 2025.

Menopause. NIH/ORWH sponsored four questions in the MNP section to provide national estimates of women who experience symptoms associated with menopause and use of prescription medication for these symptoms, and to reduce misclassification of menopause. These questions were asked of women age 40-70 who previously reported having intact uterus and cervix, and among those age 40-49 also reported no present pregnancy or not having a live birth in the past 12 months or not knowing or refusing to answer if these four conditions applied to them. An introduction statement listed symptoms commonly associated with menopause: hot flashes, night sweats, insomnia or sleep disturbances, irregular vaginal bleeding or menstrual cycles, and vaginal dryness. Women were asked whether they had these symptoms in the past 12 months, took medication for them, had a menstrual period in the past 12 months, and whether their menstrual period had stopped because of medical treatment for irregular vaginal bleeding. Hormonal changes associated with menopause transition may result in systemic health effects, including the increasing the risk of CVD, diabetes mellitus, cancers associated with central adiposity, and bone loss and fragility fracture (Davis et al, 2023).

Oophorectomy history. NIH/ORWH sponsored a question in the PRV section that asked women age 18 and older whether they have had both ovaries removed as one or more separate surgeries. For women who had previously reported history of hysterectomy, the question's ending was rephrased to "as a part of hysterectomy or as one or more separate surgeries." Hormonal changes associated with surgical menopause may be linked to systemic health effects (Davis et al, 2023).

Whole Person Health (WPH). WPH is an integrative approach to supporting and understanding a person's well-being and health across interconnected biological, behavioral, social and environmental domains (Langevin, 2021; Hibben et al 2026; Pitcher et al 2026). The WPH concept has been used in healthcare settings to integrate treatments and lifestyle changes (NCCIH, 2021). WPH, as measured by the WPH Index, can also be used in research studies to assess and track the health of the "whole person" over time (NCCIH, 2025). Questions on WPH are fielded in NHIS to collect information about WPH domains at the population level, and to facilitate examining associations between WPH and conditions such as chronic pain, diabetes, cardiovascular disease and mental health conditions. NCCIH sponsored eight questions in the WPH section that asked for subjective ratings on different WPH domains: 1) quality of life, 2) social and family connections, 3) diet, 4) physical activity, 5) ability to manage stress, 6) sleep, 7) ability to find meaning and purpose in daily life, and 8) ability to manage personal health. These eight questions can be used with the annual core question on self-rated health from the HIS section to complete the nine domains of the WPH. Response options used a five-point scale, ranging from "excellent" to "poor."

Major Recodes

Age of ADHD/ADD diagnosis. The age when first told by a doctor or health professional that the Sample Adult had had ADHD or ADD is top-coded at age 85.

Age of diabetes diagnosis. The age when first told by a doctor or health professional that the Sample Adult had diabetes is top-coded at age 85.

Age of cancer diagnosis. The age when first told by a doctor or health professional that the Sample Adult had a specific kind of cancer is available for 27 of the 29 different kinds of cancers and 'other' kind of cancer collected in NHIS. To protect confidentiality, Sample Adults mentioning kidney or testicular cancer were suppressed along with the age of diagnosis for these specific cancers. The age of cancer diagnosis for those mentioning kidney and testicular cancer are included in the recode for age of diagnosis for 'other' kind of cancer. The age of cancer diagnosis was also calculated for combined cancer types colorectal (which combines colon and rectal cancer) and for head and neck cancers (which combines larynx-tracheal, mouth/tongue/lip, and throat (pharyngeal) cancers). The youngest age provided for the respective cancer type was assigned for Sample Adults with combined cancers, those who identified the same kind of cancer as the second or third kind of cancer or mentioned 'other kind of cancer' more than once. Sample Adults who reported 'don't know' or 'refused' when asked about the type of cancer but provided an age of diagnoses for the unidentified cancer are included in the variable for age of cancer diagnosis for 'other kind of cancer.' Variables for age of cancer diagnosis are top-coded at age 85 for all cancers, and bottom-coded at age 18 for cancers of the breast, cervix, prostate, and skin (nonmelanoma, melanoma and unknown type). When calculating years since diagnosis, by subtracting age of cancer diagnosis from respondent's current age, note that age is also top-coded at 85, and differences may be underestimated or incorrect among those with bottom and top-coded values.

Body Mass Index (BMI). Two recodes for BMI were created using unrestricted height and weight values which contain the greater range of height and weight values than are available on the public-use file. The first recode classifies Sample Adults into underweight, healthy weight, overweight, and obese, and a second recode further classifies obesity to include underweight, healthy weight, overweight, class 1 obesity, class 2 obesity, and class 3 obesity. BMI was calculated using the formula: $BMI = \text{kilograms} / \text{meters}^2$ where 1 kilogram = 2.20462 pounds and 1 meter = 39.37008 inches. In 2024, the conversion into metric values were rounded to 0 decimals for weight and to 3 decimals for height, and the BMI values used to create BMI categories were rounded to 2 decimals. The categorical measure of BMI was classified as follows: underweight is BMI < 18.5; healthy weight is BMI 18.5 to <25; overweight is BMI ≥ 25 to <30; and obese is BMI ≥ 30. The three classes of obesity were classified as follows: class 1 obesity is BMI 30 to <35; class 2 obesity 35 to <40; and class 3 obesity is 40 or greater (CDC, 2024). Sample Adults who answered don't know or refused for height or weight, reported values for either height or weight that were outside the limits for public data release, or for whom height or weight values for public release were set to 96 and 996 due to missing sex information were coded as 99 in the categorical BMI measure.

Cancer types. Sample Adults could name up to three kinds of cancer. Twenty-nine kinds of cancer could be identified in NHIS, with the option for 'other' not listed. Responses from up to three kinds of cancers were recoded during editing into "mentioned" or "not mentioned" variables for each cancer type. Due to confidentiality concerns, recodes that identified whether kidney cancer or testicular cancer were mentioned were suppressed, and Sample Adults reporting these cancers were included in the recode 'other cancer type' mentioned. Recodes were also created that combine specific kinds of cancers available in the public-use file. Sample Adults reporting colon or

rectal cancer were combined into a separate variable indicating whether colorectal cancer was “mentioned” or “not mentioned.” Similarly, Sample Adults reporting larynx-tracheal, mouth, tongue, lip, or throat (pharyngeal) cancer were recoded into a separate variable indicating whether a head and neck cancer was “mentioned” or “not mentioned.” Sample Adults who reported having had cancer but reported ‘don’t know’ or ‘refused’ when asked about the type of cancer are recoded as “don’t know” or “refused” in these recodes, respectively. Respondents who reported a combination of “don’t know” and “refused” to type of cancers are recoded as “don’t know” in these recodes.

Height and Weight. Sample Adults had the option to report height and weight using the U.S. customary system (pounds: feet and inches) or the metric system (kilograms: meters and centimeters). Metric responses on height and weight were converted into the U.S. Customary system using the following conversion scale: 1 meter=39.37008 inches and 1 kilogram=2.20462 pounds (lbs.). The public-use height variable reflects total height in inches (e.g., 65 inches is 5ft 5in), with height ranges 63–76 inches for men and 59-70 inches for women. The public-use weight variable reflects total pounds rounded to whole integers with weight ranges 126–299 lbs. for men and 100-274 lbs. for women. Pregnant women ages 18–49 years were asked to report weight before pregnancy. Sample Adults who reported values outside the public-use limits for either height or weight had data for both variables recoded to “96” or “996” (“Not available”) on the public-use data file to protect the confidentiality of those who might be identifiable by their unusual physical characteristics. In addition, due to the sex-specific height and weight limits for public data release, all Sample Adults for whom sex was answered as don’t know or refused were also coded as 96 for height and 996 for weight. Don’t know and refused responses to either height or weight were retained in the height and weight public-use recodes.

Number of cancers. A summary recode was created that indicates the number of kinds of cancer mentioned by the Sample Adult. Sample Adults who were never told that they had cancer were assigned the value 0. The number of kinds of cancers were derived from responses to the type of cancer they were told they have and whether they had a second, third and other (additional) types of cancer (assigned values 1–4, respectively, where 4 indicates four or more). This recode reflects the number of cancers mentioned even when the same kind of cancer was mentioned more than once.

Number of seizures. The response categories for the number of seizures in the past 12 months (0, 1, 2-3, 4-10 and more than 10) was top-coded at “4 or more” seizures, and values 7 (refused), 8 (not ascertained), and 9 (don’t know) were retained.

Years since diabetes diagnosis. This recode is the calculation of the Sample Adult’s current age (in years and top-coded at 85) minus the age (in years and top-coded at 85) when first told they have diabetes. This difference may be underestimated for Sample Adults with a top-coded age. The highest number of years since diabetes diagnosis available in the public-use data release is 84 years. Due to confidentiality concerns, Sample Adults age 85 and older who were diagnosed at age 85 and over, are coded as 96. This recode updates the recode available in previous years for years since diabetes diagnoses.

Whole Person Health Index (WPHI). The WPHI was developed jointly by NCCIH and NCHS to use in analysis as a continuous variable for measuring the health of the whole person (NCCIH, 2025). Responses from the eight questions in the WPH section along with the self-reported health status question from the HIS section were summed to generate an index score. The responses to the nine questions were reversed to have higher values correspond to higher ratings: 1 for “poor”, 2 for “fair”, 3 for “good”, 4 for “very good”, and 5 for “excellent”. The

total index score ranges from 9 to 45 with higher scores indicating better health. Sample Adults with the responses “don’t know” or “refused” are excluded from the calculation and coded as 99 for not ascertained.

II. Functioning and Disability

Annual Core

Functioning and Disability. The questions on functioning and disability found in the annual sections VIS, HEA, MOB, COM, COG, UPP, ANX, DEP, and the rotating sections PAI and FGE of the Sample Adult module are part of sets of international standard measures developed, tested and endorsed by the Washington Group on Disability Statistics (WG). All questions are fielded annually, except for sections PAI and FGE and selected questions within the sections HEA and COM that are rotating content. The WG is a city group established in 2001 under the United Nations Statistical Commission to address the need for population-based measures of disability by promoting and coordinating international cooperation in the area of health statistics focusing on disability data collection tools suitable for censuses and national surveys. The major objective is to provide necessary information on disability that is comparable throughout the world by identifying individuals with functional limitations in basic actions, regardless of nationality or culture. The questions reflect advances in the conceptualization of disability and use the World Health Organization’s International Classification of Functioning, Disability, and Health (ICF) as a conceptual framework. These questions do not capture all aspects of difficulty in functioning but rather focus on domains of functioning that are likely to identify the majority of adults at risk of participation restrictions in an unaccommodating environment.

The questions included for Sample Adults are from the WG Extended Set on Functioning (WG-ES). Two additional question sets developed by the WG are subsets of the WG-ES – the WG Short Set on Functioning (WG-SS) and the WG Short Set on Functioning – Enhanced (WG-SS Enhanced), comprised of 6 and 12 questions, respectively. For a list of questions included in each set and their respective questionnaire sections, see Table 13.

Questions ask about the Sample Adults’ level of difficulty (no difficulty, some difficulty, a lot of difficulty, or cannot do at all) in basic domains of functioning including seeing, hearing, mobility, communication, cognition, self-care, and upper body functioning and about the frequency and intensity of experiencing anxiety and depression, pain, and fatigue. In addition to questions about level of difficulty, several of the domains have questions to provide information on the use of accommodations. More information on the Washington Group and the question sets may be obtained by request to the WG Secretariat at WG_Secretariat@cdc.gov or found on the WG website at: <http://www.washingtongroup-disability.com/>.

The WG questions can be analyzed separately, by domain, or combined across domains. A disability status indicator is available to data users (See Major Recodes below) that identifies Sample Adults who are at greater risk than the general population for experiencing restrictions in participation because of difficulties doing certain universal, basic actions. This recode classifies Sample Adults with disability as those reporting “have a lot of difficulty” or “cannot do at all” for at least one of the six domains included in the WG Short Set on Functioning. Use of the functioning and disability data should be tailored to meet the needs of the analysis. Other disability indicators can be created from the WG Extended Set on Functioning and the WG Short Set – Enhanced. Consult the WG website for guidance on the creation of these indicators. Changing the threshold for either the number of domains the respondent identifies having difficulty with or the degree of difficulty can create different identifiers that will capture different populations. For example, a recode that includes respondents who have “some difficulty” with any of the domains will capture a greater proportion of the population than a recode limited to include only those who report they “cannot do at all” to any of the domains. In this example, the

functional abilities of the larger group will be much more heterogeneous than that of the smaller group. Analytic guidelines, including recommended disability identifiers, written for each of the WG questions sets may be obtained from the WG website: <https://www.washingtongroup-disability.com/analysis/analysis-overview/>.

Table 13. Functioning limitations included in the Sample Adult module, by topic, section and Washington Group Question Set.

Topic	Section	WG-SS	WG-SS Enhanced	WG-ES
Wear glasses or contacts	VIS			✓
Have difficulty seeing	VIS	✓	✓	✓
Use a hearing aid	HEA			✓
How often use hearing aid	HEA			✓
Have difficulty hearing	HEA	✓	✓	✓
Difficulty walking or climbing steps	MOB	✓	✓	✓
Use equipment or receive help for getting around	MOB			✓
Type of equipment	MOB			✓
Difficulty walking 100 yards	MOB			✓
Difficulty walking a third of mile	MOB			✓
Difficulty walking up or down 12 steps	MOB			✓
Difficulty communicating	COM	✓	✓	✓
Difficulty remembering or concentrating	COG	✓	✓	✓
Difficulty remembering, concentrating, or both	COG			✓
How often have difficulty remembering	COG			✓
Difficulty remembering few things, a lot, everything	COG			✓
Difficulty with self-care	UPP	✓	✓	✓
Difficulty raising a 2-liter bottle from waist to eye level	UPP		✓	✓
Difficulty using hands and fingers	UPP		✓	✓
How often feel worried, nervous, or anxious	ANX		✓	✓
Take prescription medication for these feelings	ANX			✓
Level of feelings of worried, nervous, anxious	ANX		✓	✓
How often feel depressed	DEP		✓	✓
Take prescription medication for these feelings	DEP			✓
Level of feelings of depressed	DEP		✓	✓
How often have pain*	PAI			✓
How much pain*	PAI			✓
How often felt very tired or exhausted*	FGE			✓
How long tired or exhausted feelings last*	FGE			✓
Level of tiredness or exhaustion*	FGE			✓

NOTES: WG-SS is WG Short Set on Functioning, WG-SS Enhanced is WG Short Set on Functioning – Enhanced, and WG-ES is WG Extended Set on Functioning.

*PAI is the chronic pain section, and it is fielded on odd survey years including 2025. FGE is the fatigue section, and it is fielded on even survey years, and was last fielded in the 2024 NHIS.

A cognitive testing report is available for selected WG questions at

<https://wwwn.cdc.gov/QBank/Report.aspx?1216>. For a report that examines differences in survey responses of disability between the set of disability questions from Short Set on Functioning (WG-SS) and set of disability questions developed for the American Community Survey, see, <https://www.cdc.gov/nchs/data/nhsr/nhsr161-508.pdf>.

Participation. The SOC section in the Sample Adult module contains three questions about difficulty with participation in everyday life activities that are not part of the WG questions on functioning. These questions ask the Sample Adult whether, because of a physical, mental, or emotional condition, they are limited in the kind or amount of work they can perform, have difficulty doing errands alone, and have difficulty participating in social activities. The degree of difficulty (none, some, a lot, cannot do at all) is captured for the errands and social activities questions. Analysts can analyze each question separately or can combine the questions into an indicator to meet the needs of their analysis.

Sponsored Content

The Administration for Community Living (ACL) sponsored a question in the ADO section. Sample Adults reporting having "a lot of difficulty" or "cannot do at all" to any one of five functioning questions (walking or climbing stairs, communicating, remembering or concentrating, self-care, or doing errands alone) were asked whether the difficulty began before age 22. The purpose of the question was to assist analysts who wish to identify adults with intellectual or developmental disability. This question has been fielded since 2020.

Major Recodes

Functioning difficulties status. Beginning in 2024, an additional indicator based on the WG Short Set on Functioning (WG-SS) was included. This indicator identifies Sample Adults who are at greater risk than the general population for experiencing restrictions in participation because of at least some difficulty doing certain universal, basic actions. This recode classifies Sample Adults with functioning difficulties if they reported "some difficulty", "a lot of difficulty", or "cannot do at all" for at least one of the six domains of functioning: seeing (even if wearing glasses), hearing (even if wearing hearing aids), mobility (walking or climbing stairs), communication (understanding or being understood by others), cognition (remembering or concentrating), and self-care (such as washing all over or dressing). Sample Adults who responded "no difficulty" to all six domains of functioning are classified as without functioning difficulties. Those responding "don't know" or "refused" to all six questions are excluded.

Disability status. An indicator of disability status based on the WG Short Set on Functioning (WG-SS) identifies Sample Adults who are at greater risk than the general population for experiencing restrictions in participation because of a lot of difficulty doing certain universal, basic actions. This recode classifies Sample Adults with disability as those reporting "a lot of difficulty" or "cannot do at all" for at least one of six domains of functioning: seeing (even if wearing glasses), hearing (even if wearing hearing aids), mobility (walking or climbing stairs), communication (understanding or being understood by others), cognition (remembering or concentrating), and self-care (such as washing all over or dressing). The remaining Sample Adults, that is those who responded "some difficulty" or "no difficulty" to at least one domain (and did not report "a lot of difficulty" or "cannot do at all" for any of the six domains of functioning) are classified as without disability. Those responding "don't know" or "refused" to all six questions are excluded.

III. Chronic Pain

Rotating Core

Rotating every other year since 2019 in the PAI section of the Sample Adult module are questions about frequency and severity of pain in the past 3 months, and pain impact or interference (how often pain limited their life or work activities and how often their pain had an effect on their family and significant others in the past 3 months). These questions are an extension of the work of the National Pain Strategy, an Interagency Pain Research Coordinating Committee at the National Institutes of Health. For more information, visit <https://www.ninds.nih.gov/current-research/trans-agency-activities/interagency-pain-research-coordinating-committee-iprcc>. Questions about pain frequency and severity were also fielded in 2020 as sponsored content, and pain impact was fielded during July–December 2020 as COVID-19 related content.

Rotating every other year are also questions about the amount of pain experienced in the following body locations: back; hands, arms or shoulders; hips, knees, or feet; headache or migraine; abdominal, pelvic, or genital pain; and toothache or jaw pain. The universe for the questions about pain impact or interference, and pain location are restricted to Sample Adults who reported having pain on some days, most days, or every day in the past 3 months. Analysts can analyze the pain questions separately. Two of the pain questions, frequency of pain in the past 3 months and amount of pain experienced, are also measures of the pain domain in the WG extended set of disability identifiers and can be analyzed as part of the WG Extended Set on Functioning (see Table 13).

Sponsored Content

Pain management. NCIPC sponsored several questions in the PAI section on pain management techniques. Sample Adults who reported experiencing pain some days, most days, or every day in the past 3 months were asked about their use of various techniques to manage their pain within the past 3 months. The methods to manage pain fielded were: 1) taking over-the-counter medications such as aspirin, Tylenol, Advil, or Aleve, 2) taking an opioid pain reliever, 3) taking a pain reliever other than an opioid, 4) physical therapy, rehabilitative therapy, or occupational therapy, 5) spinal manipulation or other forms of chiropractic care, 6) talk therapies such as cognitive-behavioral therapy (CBT), 7) Yoga, Tai Chi, or Qi Gong, 8) Other forms of exercise, such as walking, swimming, bike riding, stretching, or strength training, 9) massage, 10) meditation, guided imagery, or other relaxation techniques, and 11) any other approaches to manage pain. Content on pain management has been previously fielded in 2019–2020 in the PAI and OPN sections. Pain management technique questions are an extension of the work of the National Pain Strategy, an Interagency Pain Research Coordinating Committee at the National Institutes of Health. For more information, visit <https://www.ninds.nih.gov/current-research/trans-agency-activities/interagency-pain-research-coordinating-committee-iprcc>

IV. Health Care Access and Health Service Utilization

Annual Core

Several sections throughout the Sample Adult module measure access to and use of health services, as well as affordability of care. For a list of measures on these topics asked in the annual core, by questionnaire section and reference period of its respective questions, see Table 14. During 2019–2023, the number of urgent care and retail health clinic visits were asked in a single question. Beginning in 2024, two separate questions are used to better assess the prevalence of each service. Similar content is also available for the Sample Child.

Table 14: Annual core content measures of health care access, service use, and affordability of care asked of the Sample Adult, by questionnaire section and reference periods.

Measure (section)	Reference period
Immunizations (IMS)	
Flu vaccine	Last 12 months, Month and Year
Pneumonia vaccine	Ever
Number of pneumonia vaccines received	Ever
Mental Health Care (MHC)	
Took medication for emotions or mental health	Last 12 months
Received therapy or counseling	Last 12 months; current
Delayed getting therapy or counseling due to cost	Last 12 months
Needed but did not get therapy/counseling due to cost	Last 12 months
Prescription Medication (PMD)	
Took prescription medication	Last 12 months
Skipped doses to save money	Last 12 months
Took less medication to save money	Last 12 months
Delayed filling prescription to save money	Last 12 months
Needed but did not get prescription due to cost	Last 12 months
Problems Paying Medical Bills (PAY)	
Anyone in family having problems paying medical bills*	Last 12 months
Have bills unable to pay at all*	Current
Level of worry about paying medical bills if sick or accident	Current
Telehealth (TLH)	
Medical appointment by video or by phone	Last 12 months

Table 14 continued: Annual core content measures of health care access, service use, and affordability of care asked of the Sample Adult, by questionnaire section and reference periods.

Measure (section)	Reference period
Utilization (UTZ)	
Saw a doctor for medical care	Last time interval
Medical and wellness visit combined	Last time interval
Wellness visit	Last time interval
Usual place to go for medical care	Current
Kind of place for medical care	Current
Number of retail health clinic visits	Last 12 months
Number of urgent care visits	Last 12 months
Number of emergency department visits	Last 12 months
Any overnight hospitalization	Last 12 months
Delayed medical care due to cost	Last 12 months
Needed but did not get medical care due to cost	Last 12 months

* These are family-level replicate questions asked once per family.

Rotating Content

Additional content about the use of selected health services is included in 2025–2026, as part of rotating content previously fielded in 2019–2020 and 2022–2023. These questions ask Sample Adults about receiving dental, eye, home, and physical or other therapeutic care, and about affordability of dental care. Table 15 lists rotating core measures of health care use, and affordability, by questionnaire section and reference period of its respective questions.

Table 15. Measures of health care access, service use, and affordability of care asked of the Sample Adult in the rotating core, by questionnaire section and reference periods.

Measure (Section)	Reference period
Dental Care (DNC)	
Received a dental cleaning or exam	Last time interval
Delayed dental care due to cost	Last 12 months
Needed but did not get dental care due to cost	Last 12 months
Physical and Other Therapeutic/Specialist Care (PTC)	
Received eye exam from eye specialist	Last 12 months
Received physical, speech, rehabilitative, or occupational therapy	Last 12 months
Received home care	Last 12 months

Emerging Content

Internet Access and Health Care. Five questions on internet access and use of the internet for health and medical purposes have been fielded since July 2022 in the HIT section as emerging content. These questions aim to capture data on individuals' use of the internet to access health information, communicate with healthcare providers, and obtain medical test results. Internet access may enable access to telemedicine, promote health literacy, and support public health prevention (Benda et al., 2020; Early and Hernandez, 2021), and may have a positive impact on mental well-being, physical activity, and social isolation. (Kearns and Whitley, 2019, Duplaga and Szulc, 2019; Khalaila and Vitman-Schorr, 2018).

Sponsored Content

Caregiving. NCCDPHP sponsored one question on caregiving in the CAR section. Sample Adults were asked if they provided regular care or assistance to a friend or family member with a health problem or disability in the past 30 days. Data on caregiving can be used for national estimates of the extent of caregiving in the U.S. and to examine the health and well-being of caregivers.

COVID-19 vaccination: In 2025, the questions about COVID vaccination changed from ever having received a vaccine, to having received a vaccine in the past 12 months. Month and year of most recent vaccination were collected, but specific to a vaccine received in the past 12 months. As a result of these changes, variable names have changed.

Immunization. The IMS section includes several questions sponsored by NCIRD regarding CDC recommended vaccinations for flu, shingles, tetanus, pertussis, COVID-19, and RSV (see adult vaccination schedule at: <https://www.cdc.gov/vaccines-adults/index.html>). Table 16 lists immunization measures by population, reference period and additional details collected.

Table 16. Sponsored content about immunization measures asked of Sample Adults, by population, reference period, and additional information collected: 2025 NHIS.

Vaccine	Population	Reference Period
COVID-19*	Adults age 18 and older	Past 12 months
Shingles†	Adults age 50 and older	Ever
Tdap (tetanus including pertussis)	Female adults ages 18–49 or age unknown who had a live birth in the past 12 months	During pregnancy
Tetanus	Adults who responded no, don't know, or refused, or who were not asked the question about Tdap vaccination during pregnancy	Past 10 years
RSV	Adults age 50 and older	Ever

* Asked about month and year. Similar content is available for Sample Children.

† In 2024, shingles vaccination was also asked of adults age 18 and older who have a weakened immune system.

V. Preventive Care and Services

Rotating Core

Aspirin Use. Rotating every other year starting in 2019 are questions about aspirin use. In the ASP section, Sample Adults age 40 and older were asked if a doctor or other health professional ever told them to take a low-dose aspirin each day to prevent or control heart disease and whether they were following that advice. In April 2022, the United States Preventive Services Task Force (USPSTF) updated the 2016 recommendation on the use of low-dose aspirin for the prevention of cardiovascular disease (CVD) (USPSTF, 2022). The USPSTF concluded that low-dose aspirin has a small net benefit in reducing the risk for cardiovascular events in adults ages 40 to 59 who have a 10% or greater 10-year CVD risk, and there is no net benefit to initiating aspirin use for the primary prevention of CVD events in adults age 60 and older (USPSTF, 2022).

Preventive Services. Rotating every other year in the PRV section is content about clinical checks of blood glucose, cholesterol, arterial blood pressure, and screenings for colorectal cancer, and breast and cervical cancer among women (see Table 17). Questions about cholesterol and blood pressure checks were asked of all adults and check for blood glucose was asked of adults who have not been told they have diabetes. In 2025, questions about colorectal cancer screening were asked of adults age 35 and older, a change from prior rotations when asked of adults age 40 and older. The variable names for questions on colorectal cancer screening were changed in 2025 to reflect the change in universe. Questions about cervical cancer screening were asked among adults who reported their sex as female, and questions about breast cancer screening were asked among adults who reported their sex as female and age as 30 and older.

The USPSTF recommends screening for colorectal cancer screening for adults ages 45-75, and selective screening for adults ages 76-85 (USPSTF, 2021). There are multiple factors that influence which tests clinicians and patients may use —frequencies of screenings, home-based or clinical setting, stool-based or direct visualization, preparation procedures, sedation during the test, and follow-up procedures for abnormal findings.

The USPSTF recommends screening women ages 21-65 for cervical cancer every three years with pap smear test (cervical cytology) alone, and in women ages 30-65 every five years with high-risk human papillomavirus (hrHPV) testing alone or dual testing of hrHPV and pap smear (USPSTF, 2018.).

The USPSTF recommends mammography screening for breast cancer every two years in women ages 40-74 (USPSTF, 2024).

Sponsored Content

Diabetes checks. NCCDPHP sponsored two questions asked in the PRV section that asked Sample Adults with diabetes about the last time they had their hemoglobin A1C level checked by a doctor, nurse, or other health professional, and the number of times monitored if checked in the past 12 months.

Cancer screening. NCCDPHP and NCI sponsored several questions about breast, cervical, colorectal, and prostate cancer screenings in the PRV section. See Table 18 for the specific topics by population and reference period. Cancer screening content was previously fielded in 2023, 2021, and 2019. Analysts comparing measures about cancer screening between prior years should note variable name updates which are indicative of changes in universe (i.e., who was asked the question), response options or question text. For information about periodicity of

sponsored cancer control content, see National Health Interview Survey (NHIS) Cancer Control Supplement (CCS) at <https://healthcaredelivery.cancer.gov/nhis/>.

Table 17. Rotating content about cardiovascular and cancer screening measures asked of Sample Adults, by population asked and period measured.

Topic	Population asked	Period measured
Cardiovascular checks		
Arterial blood pressure	Adults age 18 and older	When last
Blood cholesterol	Adults age 18 and older	When last
Blood Glucose	Adults age 18 and older without diabetes	When last
Breast cancer screening		
Mammogram	Females age 30 and older	Ever; When last
Cervical cancer screening		
Pap smear or Human Papillomavirus (HPV) test	Females age 18 and older	Ever; When last, type of test
Hysterectomy*	Females age 18 and older	Ever
Colorectal cancer screening		
Colonoscopy†§	Adults age 35 and older	Ever; When last
Sigmoidoscopy†§	Adults age 35 and older	Ever; When last
CT colonography or virtual colonoscopy†	Adults age 35 and older	Ever; When last
Fecal immunochemical test (FIT) or fecal occult blood test†	Adults age 35 and older	Ever; When last

* Women who reported a history of a hysterectomy as a reason for not having had cervical cancer screening were not asked this question but were auto-coded as having had a hysterectomy.

† This question has a new variable name in 2025 to reflect the change in age range for this question.

§ Sample Adults who had a colonoscopy or sigmoidoscopy but did not know which one were only asked for the most recent time of this exam.

Major Recodes

Number of A1Cs. Responses 5 to 95 times, and "96 or more" for the number of A1C tests in the past 12 months were top-coded as "5 or more," and responses corresponding to "refused," "not ascertained," and "don't know" were retained with values 7–9, respectively.

Table 18. Sponsored content on cancer screening, by population and reference period: 2025 NHIS.

Topic	Population asked	Reference period
Breast cancer screening		
Main reason for mammogram	Females age 30 and older who ever had a mammogram	At most recent screening
Most important reason for not having mammogram	Females age 30 and older who never had a mammogram or had a mammogram more than 5 years ago	Not applicable
Dense breast tissue information	Females age 30 and older who ever had a mammogram	At most recent screening
Breast exam by a doctor	Females age 25 and older	Ever; When last
Main Reason for breast exam by a doctor	Females age 25 and older who ever had a breast exam by a doctor	At most recent screening
Cervical cancer screening		
Main reason for cervical test	Females age 18 and older who had a cervical cancer test	At most recent screening
Pap test	Females age 18 and older who had a cervical cancer test	At most recent screening
HPV test	Females age 18 and older who had a cervical cancer test	At most recent screening
Told if had Pap or HPV test	Females age 18 and older who had a cervical cancer test	At most recent screening
Follow-up to check for cancer or precancerous cells	Females age 18 and older who had a cervical cancer test	Past 5 years
Main reason for no cervical cancer screening	Female age 18 and older who never had a cervical cancer test or had their last cervical cancer test more than 5 years ago	Not applicable
Colorectal cancer screening		
Cologuard*	Adults age 35 and older	Ever; When last
Colonoscopy reason	Adults age 35 and older who ever had a colonoscopy	At most recent screening
Payment for colonoscopy	Adults age 35 and older who ever had a colonoscopy	At most recent screening
Doctor recommended colorectal screening	Adults age 35 and older who were never tested for colorectal cancer, or whose last test exceeded the recommended time interval between testing, or is unknown†	Past 12 months
Type of tests recommended by doctor/health professional	Adults age 35 and older who had a colorectal cancer test recommended	Past 12 months

Table 18 continued. Sponsored content on cancer screening, by population and reference period: 2025 NHIS.

Topic	Population asked	Reference period
Prostate cancer screening		
Prostate-specific antigen (PSA) test	Males age 40 and older	Ever; When last
Reason for PSA	Males age 40 and older who ever had a PSA test	At most recent screening
Number of PSA tests	Males age 40 and older who ever had a PSA test but not more than 10 years ago or refused or don't know	Past 5 years

* Sample Adults who reported “yes” to having had a Cologuard test and “yes” to the previous question to ever having had a blood stool or Fecal immunochemical test (FIT) were also asked whether the Cologuard test was part of the Fecal immunochemical test (FIT) reported in an earlier. Only those reporting “no,” “don’t know,” or “refused” were asked for the last time they had a Cologuard test.

† See the universe description in the Questionnaire or Sample Adult codebook for more information.

VI. Health-Related Behaviors

Annual Core

Sample Adults were asked about cigarette smoking and the use of electronic cigarettes (e-cigarettes) in the CIG section. All adults were asked if they had smoked at least 100 cigarettes in their entire life. Those who had done so were asked whether they now smoke cigarettes every day, some days, or not at all. Every day smokers were asked about the number of cigarettes smoked every day, while someday smokers were asked the number of days that they smoked in the past 30 days and the average number of cigarettes smoked on those days. All Sample Adults were also asked about the use of e-cigarettes or other electronic vaping products, even one time in their entire life, and if so, whether they now use this product every day, some days, or not at all.

Sponsored Content

Other Tobacco Products. The FDA sponsored five questions about the use of cigars and smokeless tobacco products in the OTB section. In separate questions, Sample Adults were asked about whether they had ever smoked a cigar or used smokeless tobacco products. Questions included examples and descriptions of these products. Those who said “yes” to each respective tobacco product were asked whether they now use it every day, some days or not at all. Sample Adults who had ever smoked any type of cigar were also asked about the number of days they smoked cigars in the past 30 days.

Major Recodes

Cigarette smoking status. Sample Adults were classified in terms of their lifetime and current cigarette smoking status. The recode includes the following categories: current every day smoker; current someday smoker; former smoker; never smoker; smoker, current status unknown; and unknown if ever smoked. Former smokers smoked at

least 100 cigarettes in their entire life and currently do not smoke at all. Never smokers have never smoked any cigarettes or have smoked less than 100 cigarettes in their entire life. “Smoker, current status unknown” is a person who has smoked 100 cigarettes in their entire life and the question about current smoking practices was answered as don’t know or refused, or it was not ascertained. “Unknown if ever smoked” includes those whose response to ever having smoked at least 100 cigarettes in their entire life was don’t know or refused, or it was not ascertained.

E-cigarette use status. Sample Adults were classified in terms of their ever and current e-cigarette use. The recode includes the following categories: current e-cigarette user; used e-cigarette, not current user; never e-cigarette user; e-cigarette user, current status unknown; and unknown if ever used e-cigarettes. Current e-cigarette use include those who use e-cigarettes every day or somedays. Not current e-cigarette use include those who have used an e-cigarette at least one time and who currently do not use them at all. Never e-cigarette use include those who have never used an e-cigarette. User, current status unknown is defined as person who has used an e-cigarette at least one time and the question about current use was answered as don’t know or refused, or it was not ascertained. Adults whose e-cigarette use is unknown include those whose response to ever having used an e-cigarette in their entire life was answered as don’t know or refused, or it was not ascertained.

VII. Mental Health

Rotating Core

Rotating every three years, and previously fielded in 2022 and 2019, are two mental health scales measuring symptoms of generalized anxiety disorder (GAD) and depression.

Symptoms of generalized anxiety disorder are measured using the 7-item Generalized Anxiety Disorder scale (GAD-7) (Spitzer et al., 2006). This is a brief scale to screen for GAD symptoms in the past two weeks and assess its severity in clinical settings and the general population (Rutter and Brown 2017; Löwe et al., 2008). GAD-7 has also been found to have moderately good operating characteristics for three other anxiety disorders – panic disorder, social anxiety disorder, and post-traumatic stress disorder (Kroenke et al., 2007). The GAD-7 was developed based on the most correlated items with a 13-item scale that included 9 items from the criteria for GAD in the Diagnostic and Statistical Manual for Mental Disorders, Fourth Edition (DSM-IV) and 4 items on the basis of review of existing anxiety scales (Spitzer et al., 2006).

Symptoms of depression are measured using the 8-item Patient Health Questionnaire depression scale (PHQ-8) (Kroenke et al., 2009). This is a valid diagnostic and severity measure for current depressive disorders derived using the nine-item criteria for depressive disorders in the DSM-IV. Symptoms of depression are asked in reference to the past two weeks. PHQ-8 is used in clinical settings and in population-based studies to screen for symptoms indicative of a possible clinically significant depression and assess severity of depressive disorders (Kroenke et al., 2009). PHQ-8 is an abbreviated version of the nine-item PHQ-9 scale (Kroenke et al., 2001) that excludes the question about thoughts of death and self-injury, an indicator of possible suicide risk.

The set of questions in the GAD-7 and PHQ-8, available in the GAD and PHQ sections of the questionnaire, ask Sample Adults to assess how often they have been bothered over the last 2 weeks by a set of specific symptoms. Response options to the GAD-7 and PHQ-8 questions are the same: 1) not at all, 2) several days, 3) more than half the days and 4) nearly every day. These response categories correspond to 0 to 3 points, respectively, when scoring each question. The points for each question are summed to produce a total score between 0 and 21 in GAD-7 and between 0 and 24 for PHQ-8. Total scores may be used in analysis, and pre-determined cut-off values of the total score are available for assessing severity of symptoms (Kroenke et al., 2010). Variables classifying the total score

into four categories of severity of symptoms of GAD and depression (i.e., none or minimal, mild, moderate, and severe) are available to data users (see major recodes, below). The recode for severe symptoms of depression is based on a PHQ-8 total score of 15 or greater. The PHQ-8 total score can also be classified into five categories of severity, where those scoring 15 or greater may be further categorized into moderately severe (point values 15 to 19) and severe symptoms (point values 20 to 24) (Kroenke et al., 2009). For both GAD-7 and PHQ-8, a total score of 10 or greater is indicative of a possible clinically significant condition.

Some other surveys do not include all 8 questions of the PHQ-8 or 7 questions of the GAD-7 but instead include only the first two items of each scale. These abbreviated measures are known as the PHQ-2 and GAD-2, respectively (Kroenke, et al., 2003; Kroenke et al., 2007). PHQ-2 and GAD-2 scores can be calculated from the NHIS for comparison with other surveys. The score of the two questions per scale are summed to produce a total score between 0 and 6. A score of 3 points or greater identifies possible cases for major depressive disorder based on the PHQ-2 and for generalized anxiety disorder based on the GAD-2.

Emerging Content

Social Parenting Support. In 2025, three questions related to connectedness and stressors experienced by parents of children under 18 years residing at home were fielded as emerging content in the SPS section. Sample Adults identified as parents were asked about the frequency of receiving the social and emotional support they need for parenting and raising children, level of difficulty handling the day-to-day demands of parenting, and frequency of connecting with other parents about their children. Only Sample Adults with biological, step, or foster child in the home identified in rostering were included to receive this question. Sample Adults who told the interviewer at the first question that they were not a parent or were not raising children in the home were not asked additional questions in this section. These questions can assist with monitoring parental social connectedness and stress in the U.S. The U.S. Surgeon General Advisory on the Mental Health and Well-Being of Parents emphasized the importance of understanding the unique challenges of raising children in the modern age, emphasizing the stressors that parents face in their daily lives, and finding ways to foster a culture of connection among parents to combat loneliness and isolation (OSG, 2024).

For researchers interested in examining the relationship between connectedness and stressors experienced by the Sample Adult parent and health outcomes of their child (Sample Child), see section “Parent-Child Pair Analysis.” Additional content about social and emotional support was also fielded in the SCI and SSL sections of the Sample Adult module.

Sponsored Content

Life Satisfaction. NCCDPHP and ODP sponsored a question about life satisfaction in the LSF section. Sample Adults were asked how satisfied they were with their life using response options of very satisfied, satisfied, dissatisfied, and very dissatisfied. Life satisfaction is an aspect of general well-being that represents people’s subjective assessments of their lives overall and reflects factors such as health, work, and income, among others (OECD, 2013). The American Psychological Association defines life satisfaction as “the extent to which a person finds life rich, meaningful, full, or of high quality” (APA, 2022). Measures of life satisfaction, including the items used in NHIS, have been shown to be valid and reliable (Cheung and Lucas, 2014; OECD, 2013). Life satisfaction will be used as a Healthy People 2030 Overall Health and Well-being Measure to estimate and track adult well-being at the national level (Ochiai et al., 2021). This question has been fielded since 2021. In 2021, this question was fielded as part of an experiment; see the 2021 *Survey Description* for more information.

Social Connection. In 2025, NCCDPHP sponsored a question on social support and a question on loneliness in the SSL section—which continued from 2024—as well as four new questions on social isolation in the SCI section. In SSL, Sample Adults were asked about how often they receive the social and emotional support they need and how often they feel lonely. In SCI, Sample Adults were asked about how many times they get together with and talk on the telephone or by video with people they care about and feel close to in a typical week. They were also asked about the frequency of attending religious services and attending meetings of clubs or organizations they belong to in the past 12 months. These four questions were adapted from the Berkman-Syme Social Network Index (SNI) (Berkman and Syme, 1979). The NHIS questions were revised to instruct Sample Adults to exclude people living with them when responding to frequency of social interactions in a typical week, and to include video calls to capture a newer alternative to telephone calls. In addition, response options were categorized for ease of interviewing. The SNI scoring includes marital status, and in the NHIS, marital status is available in the MAR section. For more information about the SNI scoring method, see <https://www.nationalacademies.org/read/18951/chapter/7#198> and Pantell et al., 2013.

While the questions in SSL focus on functional features of social connection and perceived or subjective feelings of loneliness and connectedness, the questions in SCI focus on structural features of social connection and provide objective, quantitative measures of social isolation through the number of social interactions (Holt-Lunstad, 2021). Studies suggest that both social isolation and loneliness are associated with health outcomes, with loneliness having more psychological effects and social isolation having stronger effects on mortality (Hong et al., 2023). These questions can assist with monitoring social connectedness in the U.S.

Major Recodes

Severity of GAD symptoms. Severity of GAD symptoms were categorized into no or minimal symptoms, mild symptoms, moderate symptoms, and severe symptoms for Sample Adults with responses to at least 6 of the 7 questions, using the cut-offs of the total score at <5, <10, <15, and 15 or greater. Instances when one question was refused or answered as don't know, or it was not ascertained, the average of the 6 ascertained answers (with point values 0-3) was calculated and added to the total score from the six ascertained questions. Total scores with decimal values were not rounded up to the next whole integer for determining category of severity. Sample Adults missing responses to two or more questions were categorized as having unknown severity of symptoms and coded as 8.

Severity of depression symptoms. Severity of depression symptoms were categorized into no or minimal symptoms, mild symptoms, moderate symptoms, and severe symptoms for Sample Adults with responses to at least 7 of the 8 questions, using the cut-offs of the total score at <5, <10, <15, and 15 or greater. Instances when one question was refused or answered as don't know, or it was not ascertained, the average of the 7 ascertained answers (with point values 0-3) was calculated and added to the total score from the seven ascertained questions. Total scores with decimal values were not rounded up to the next whole integer for determining category of severity. Sample Adults missing responses to two or more questions were categorized as having unknown severity of symptoms and coded as 8.

VIII. Injuries

Sponsored Content

Traumatic Brain Injury. In 2023–2025, NCIPC sponsored content about Traumatic Brain Injury (TBI). Mild traumatic brain injuries are known as concussions. Concussions and TBIs are injuries caused by a bump, blow, or jolt to the head or penetrating head injury that results in disruption of normal brain function.

The TBI section began with an introductory statement that asked Sample Adults to think about all head injuries, for example, from playing sports, car accidents, falls, or being hit by something or someone that occurred in the past 12 months. Two separate questions asked whether the Sample Adult had experienced concussion symptoms as a result of a head injury such as: 1) been knocked out or lost consciousness, dazed or confused, gap in memory; and 2) headaches, sensitivity to light or noise, balance problems, or changes in mood or behaviors. Sample Adults reporting concussion symptoms were asked whether they were evaluated for a concussion or brain injury by a health professional, whether the injury occurred when playing sports or participating in physical or recreational activities, and if so, whether it was while participating in organized team or league sports settings. These questions were also asked of Sample Children ages 0–17 years.

IX. Community Engagement and Social Context

Emerging Content

Civic Engagement and Volunteering. Questions that asked Sample Adults about their participation in local elections and public meetings, and volunteer activities over the past 12 months, have been fielded since July 2022 in the CIV and VOL sections as emerging content. Higher levels of civic engagement, as measured by voter participation, volunteering, and community organizing, have been linked to higher levels of well-being, self-reported physical health, and lower levels of chronic disease and depression (Danso, 2017; Nelson et al., 2019; Stopka et al., 2022).

Language Spoken at Home. Beginning in quarter 2 of 2023, questions related to speaking a language other than English at home were added as emerging content in the LNG section. Primary use of a language other than English has been related to differences in health, access to care, and use of services (Dubard & Gizlice, 2008).

Sample Adults responding to the interview in English were asked if they speak a language other than English at home, and if so, to specify the other language spoken most often at home. Sample Adults responding to the interview in a language other than English were coded as speaking another language at home and the language used during the interview was coded as the language spoken most often at home. All those who spoke another language at home were also asked whether they use English or other language when they see a doctor and consume media. Adults who spoke a language other than English at home and indicated earlier in the interview that they were able to participate in social activities were also asked whether they use English or other language when socializing.

Major Recodes

Language Spoken at Home. Sample Adults who spoke a language at home other than English were asked to specify the language. Responses were categorized based on a predetermined list, or recorded verbatim for languages that did not appear on the list. To protect confidentiality, almost all responses to language spoken at home were suppressed from the public-use file. A public-use recode is available in the LNG section that identifies whether the language spoken at home was Spanish or another language.

Sample Child's Health

I. Health Status and Conditions

Annual Core

The Sample Child module includes questions about the perceived health status of the child and whether a doctor or other health care professional had diagnosed the child with asthma, diabetes, selected developmental conditions, and whether a representative from a school or a health professional had stated that the child had a learning disability. Questions about asthma also asked whether the Sample Child still asthma has; had an episode of asthma or an asthma attack during the past 12 months; and had to visit an emergency room or urgent care center because of asthma during the past 12 months. Table 19 shows the topic, section, age of child and reference period for the question.

Table 19. Annual core content measures of health conditions about the Sample Child, by questionnaire section, age asked, and reference period.

Topic	Section	Age	Reference period
Asthma	AST	0–17 years	Ever, past 12 months, current
Autism, Asperger's disorder, pervasive developmental disorder, or autism spectrum disorder	DLD	2–17 years	Ever, current
Attention-Deficit/Hyperactivity Disorder (ADHD) or Attention-Deficit Disorder (ADD)	DLD	2–17 years	Ever, current
Diabetes	DLD	0–17 years	Ever
Health status	HIS	0–17 years	Current
Intellectual disability	DLD	0–17 years	Ever, current
Learning disability	DLD	2–17 years	Ever, current
Other developmental delay	DLD	0–17 years	Ever, current
Prediabetes or borderline diabetes	DIB	0–17 years	Ever

Sponsored Content

Communication Disorders. NIDCD sponsored content in the VSL section related to sensory or communication disorders in children ages 3-17 years to help track Healthy People 2030 Objectives and to provide national estimates. Questions about communication disorders asked whether the Sample Child had difficulty in the past 12 months with voice (i.e., hoarse, strained voice, or trouble speaking loud enough), swallowing, speech (i.e., stuttering, trouble pronouncing words), and language (i.e., using, learning, or understanding words). For each of the communication problems experienced, follow-up questions asked for the number of days they had this and the level of difficulty experienced. An additional question asked if the speech problem included stuttering, trouble pronouncing words correctly, or both. For sample children who experienced any of the communication problems, a question was asked if these difficulties followed a head or brain injury, if the child had seen a speech-language pathologist or specialist, and if the disorder had improved or gotten worse in the past 12 months. Those who had seen a speech-language pathologist or specialist were also asked how the health care services received had affected their lives. Similar content is also available for the Sample Adult.

COVID-19 Infection. The CVL section includes items about COVID infection and Long COVID symptoms sponsored by NCIRD. The Sample Child respondent was asked if the child ever had COVID-19. For children with a history of infection, respondents were asked if the child had COVID-19 symptoms lasting for 3 or more months that they didn't have before the infection (Long COVID), whether they currently had symptoms, and if Long COVID symptoms had reduced their ability to carry out day-to-day activities compared with the time before they had COVID-19. These questions have been fielded since 2023 and in 2024, the variable name for the question about Long COVID symptoms was updated to reflect a minor change in the "read if necessary" statement about examples of symptoms. Similar questions about COVID-19 infection were asked of Sample Adults.

Major Recodes

Missed days due to asthma. A recode was created of the number of missed days to daycare or school due to asthma during the past 12 months, top-coded at 10 or more days.

II. Functioning and Disability

Annual Core

The questions on functioning and disability found in sections VIS, HEA, MOB, UPP, COM, COG, ANX, DEP, and BEH of the Sample Child module compose the Child Functioning Module (CFM) that is part of a set of international standard measures developed, tested and endorsed by the Washington Group on Disability Statistics (WG). The WG is a city group established in 2001 under the United Nations Statistical Commission to address the need for population-based measures of disability by promoting and coordinating international cooperation in the area of health statistics focusing on disability data collection tools suitable for censuses and national surveys. The major objective is to provide necessary information on disability that is comparable throughout the world by identifying individuals with functional limitations in basic actions, regardless of nationality or culture. The questions reflect advances in the conceptualization of disability and use the World Health Organization's International Classification of Functioning, Disability, and Health (ICF) as a conceptual framework. The CFM was developed jointly with the United Nations Children's Fund (UNICEF). There are questions about children 2–4 years of age (CFM 2–4) and questions about children 5–17 years of age (CFM 5–17). These questions do not capture all aspects of difficulty in functioning, but rather focus on domains of functioning that are likely to identify the majority of children at risk of participation restrictions in an unaccommodating environment.

Questions ask about the Sample Child's level of difficulty (no difficulty, some difficulty, a lot of difficulty, or cannot do at all) in basic domains of functioning including seeing, hearing, mobility, dexterity, self-care, communication, cognition, playing, learning, relationships, and behavior and about the frequency of experiencing anxiety and depression as well as kicking/biting/hitting others. The CFM 2–4 and CFM 5–17 are designed to include domains of functioning relevant for each age group. For a list of questions asked in each set, and the associated functioning domain, questionnaire section, and respective age range, see Table 20. In addition, several of the domains have questions to provide information on the use of accommodations. Questions about the use of equipment or assistance with walking were asked about children who had difficulty walking both with and without equipment or assistance. More information may be obtained by request to the WG Secretariat at WG_Secretariat@cdc.gov or found on the WG website at: <http://www.washingtongroup-disability.com/>.

The WG questions can be analyzed separately, by domain, or combined across domains. CFM disability status indicators are available to data users (see Major Recodes below). The disability composite indicators for Sample Children ages 2–4 and 5–17 identify children who are at greater risk than the general population for experiencing restrictions in participation because of difficulties doing certain universal, basic actions. Consult the WG website for guidance on the creation of these indicators. Changing the threshold for either the number of domains the respondent identifies the child having difficulty with or the child's degree of difficulty can create different identifiers that will capture different populations. For example, a recode that includes Sample Children with the response "some difficulty" to any of the domains will capture a greater proportion of the population than a recode that includes only Sample Children with the response "cannot do at all" to any of the domains. In this example, the functional abilities of the larger group will be much more heterogeneous than that of the smaller group. Analytic guidelines written for each of the CFM questions sets, including recommended disability identifiers, may be obtained from the WG website.

Table 20. Annual core content measures of functioning limitations measured in the Sample Child module, by domain, question topic, questionnaire section and age range of question

Domain	Question topic	Section	Age range for question
Seeing	Wear glasses or contacts	VIS	2–17
Seeing	Have difficulty seeing (with glasses, if worn)	VIS	2–17
Hearing	Use a hearing aid	HEA	2–17
Hearing	Have difficulty hearing sounds (with hearing aid, if used)	HEA	2–17
Mobility	Use equipment or assistance for walking	MOB	2–17
Mobility	Difficulty walking	MOB	2–4
Mobility	Difficulty walking 100 yards	MOB	5–17
Mobility	Difficulty walking a third of a mile	MOB	5–17
Dexterity	Difficulty picking up small objects	UPP	2–4
Self-care	Difficulty with self-care	UPP	5–17
Communication	Difficulty understanding you	COM	2–4
Communication	Difficulty understanding Sample Child	COM	2–4
Communication	Difficulty being understood by people inside of household	COM	5–17
Communication	Difficulty being understood by people outside of household	COM	5–17
Learning	Difficulty learning things	COG	2–17
Cognition	Difficulty remembering things	COG	5–17
Affect	How often seem very anxious, nervous, or worried	ANX	5–17
Affect	How often seem very sad or depressed	DEP	5–17
Playing	Difficulty playing	BEH	2–4
Behavior	Kick, bite, or hit other children or adults	BEH	2–4
Behavior	Difficulty controlling behavior	BEH	5–17
Cognition	Difficulty concentrating	BEH	5–17
Behavior	Difficulty accepting changes in routine	BEH	5–17
Relationships	Difficulty making friends	BEH	5–17

Major Recodes

Disability status, ages 2–4. An indicator of disability status based on the WG Child Functioning Module that identifies Sample Children ages 2–4 years who are at greater risk than the general population for experiencing restrictions in participation because of difficulties doing certain universal, basic actions. This recode classifies children with disability as those with the responses “a lot of difficulty” or “cannot do at all” for at least one of the questions asking about the Sample Child’s difficulty seeing, hearing, walking, dexterity, communication, learning, and playing, or with the response “cannot do at all” to the question about controlling behavior. The remaining Sample Children, with the responses “some difficulty” or “no difficulty” to at least one question (and who do not have the responses “a lot of difficulty” or “cannot do at all” for any of the questions), are classified as without disability. Sample Children with a response of “don’t know” or “refused” to all questions are excluded.

Disability status, ages 5–17. An indicator of disability status based on the WG Child Functioning Module that identifies Sample Children ages 5–17 years who are at greater risk than the general population for experiencing restrictions in participation because of difficulties doing certain universal, basic actions. This recode classifies children with disability as those with the responses “a lot of difficulty” or “cannot do at all” for at least one of the questions asking about the Sample Child’s difficulty seeing, hearing, walking, self-care, communication, learning, remembering, concentrating, accepting change, controlling behavior, and making friends or the response “daily” to questions asking how often the Sample Child feels anxious, nervous, or worried or feels depressed. The remaining Sample Children, with the responses “some difficulty” or “no difficulty” to at least one question (and who do not have the responses “a lot of difficulty” or “cannot do at all” or “daily” for any of the questions), are classified as without disability. Sample children with a response of “don’t know” or “refused” to all questions are excluded.

III. Health Care Access and Health Service Utilization

Annual Core

Several sections throughout the Sample Child module measure access to and use of health services, as well as affordability of care. For a list of measures on these topics asked in the annual core, by questionnaire section and reference period of its respective questions, see Table 21. Similar content is also available for Sample Adults. Beginning in 2024, the Mental Health Care section became annual core content for Sample Children. During 2019–2023, the number of urgent care and retail health clinic visits were asked in a single question. Beginning in 2024, two separate questions are used to better assess the prevalence of each service.

Table 21. Annual core content measures of health care access, service use, and affordability of care asked of the Sample Child, by questionnaire section and reference periods.

Measure (Section)	Reference period
Immunizations (IMM)	
Flu vaccine (any, number of vaccines up to 2)	Last 12 months, Month and Year
Mental Health Care (MHC)*	
Took medication for emotions/mental health	Last 12 months
Received therapy or counseling from mental health professional	Last 12 months
Delayed getting therapy/counseling due to cost	Last 12 months
Needed but did not get therapy/counseling due to cost	Last 12 months
Prescription Medication (PMD)	
Took prescription medication	Last 12 months
Delayed filling prescription to save money	Last 12 months
Needed but did not get prescription due to cost	Last 12 months
Problems Paying Medical Bills (PAY)	
Anyone in family having problems paying medical bills†	Last 12 months
Have bills unable to pay at all†	Current
Level of worry about paying medical bills if sick/accident	Current
Telehealth (TLH)	
Medical appointment by video or by phone	Last 12 months
Utilization (UTZ)	
Saw a doctor for medical care	Last time interval
Medical and wellness visit combined	Last time interval
Wellness visit	Last time interval
Usual place to go for medical care	Current
Kind of place for medical care	Current

Table 21 continued. Annual core content measures of health care access, service use, and affordability of care asked of the Sample Child, by questionnaire section and reference periods.

Measure (Section)	Reference period
Utilization (UTZ) continued	
Number of retail health clinic visits	Last 12 months
Number of urgent care visits	Last 12 months
Number of emergency department visits	Last 12 months
Any overnight hospitalization	Last 12 months
Delayed medical care due to cost	Last 12 months
Needed but did not get medical care due to cost	Last 12 months

* MHC became annual core content for Sample Children in 2024

† These are family-level replicate questions asked once per family.

Rotating Core

Additional content about the use of selected health services is included in 2025–2026, as part of rotating content previously fielded in 2022–2023 and 2019–2020. These questions ask Sample Children about receiving dental, eye, home, and physical or other therapeutic care, and about affordability of dental and mental care. Table 22 lists rotating core measures, by questionnaire section and reference period of its respective questions.

Table 22. Measures of health care access, service use, and affordability of care asked of the Sample Child in the rotating core, by questionnaire section and reference periods: 2025–2026NHIS.

Measure	Reference period
Dental Care (DNC)	
Received a dental cleaning/exam	Last time interval
Delayed dental care due to cost	Last 12 months
Needed but did not get dental care due to cost	Last 12 months
Physical and Other Therapeutic/Specialist Care (PTC)	
Received eye exam from eye specialist	Last 12 months
Received physical/speech/rehabilitative/occupational therapy	Last 12 months
Received home care	Last 12 months

Sponsored Content

COVID-19 Vaccination. NCIRD sponsored questions about COVID-19 vaccination in the CVV section of the Sample Child module. In 2025, the questions about COVID vaccination changed from ever having received a vaccine, to having received a vaccine in the past 12 months. Month and year of most recent vaccination were collected, but specific to a vaccine received in the past 12 months. As a result of these changes, variable names have changed.

IV. Behavioral and Mental Health

Annual Core

The Baby Pediatric Symptom Checklist (BPSC) is a 12-item validated screening tool used for assessing social and emotional difficulties among children ages 0–17 months (Sheldrick, 2013). The BPSC is one component of the larger Survey of Well-being of Young Children (SWYC), a screening instrument designed for use in a clinical setting, such as pediatric primary care. Information about SWYC was formerly available online from the Tufts Medicine’s Department of Pediatrics. In 2025, Tufts Medical Center transferred the SWYC to the TEAM UP Scaling and Sustainability Center at Boston Medical Center (<https://www.teamupforchildren.org/parts-of-the-swyc/swyc-bpsc/>).

In the BSC section of the Sample Child module, parents or adults knowledgeable and responsible for the child’s health rated a series of behaviors related to irritability, inflexibility, and difficulty with routines that may be used to identify risk for social and emotional difficulties. Each item can be rated as “not at all”; “somewhat”; or “very much” and responses are assigned point values of 0, 1 and 2 respectively. Items with missing responses will count as 0 points. The BPSC is constructed of three subscales (irritability, inflexibility and difficulty with routines) and each subscale is composed of 4 items. Any summed score of three or more on any of the three subscales indicates that a child is at risk and in practice will prompt further evaluation with a health care professional. For analysis, users may sum responses to operationalize risk as a continuous variable. As thresholds have not yet been developed to operationalize a categorical variable for the general population, no cut-off scores have been provided. Although the BPSC instrument was developed for children under age 18 months, questions were fielded among Sample Children ages 0–23 months. Further, original validation of the BPSC was confirmed using NHIS data (Zablotsky, Black, Sheldrick, Perrin, Blumberg, 2022).

Rotating Core

The Strengths and Difficulties Questionnaire in the SDQ section is a series of questions designed to be used as a screening tool for clinical assessment of mental disorders and for epidemiological research of emotional and behavioral problems in children and adolescents aged 4-17 years. The parent respondent version of the SDQ was used and this instrument is copyrighted by Robert Goodman, Ph.D., FRCPsych, MRCP (Goodman, 1997; Goodman 2009).

Parents or adults knowledgeable and responsible for the child’s health were asked to answer, on behalf of the Sample Child, 25 questions that assess emotional symptoms, conduct problems, hyperactive behavior, peer relationships and prosocial behaviors. The instrument is intended to be scored by summing items within each subscale, and four of the five subscales can be summed to create a total score which may be used in analysis as a continuous variable. Higher scores to items assessing emotional symptoms, conduct problems, hyperactive behavior, and peer relationships indicate more problematic attributes. See Table 23 for a list of SDQ subscales by item and scoring values. Recodes of summary scores are available to data users (see Major Recodes below). Additionally, the SDQ section houses questions that collect information about the duration and impact of symptoms, which can be useful for assessing the child’s problem, the impact that the problem has on the child and his/her family. See Table 24 for a list of the SDQ impact subscale scale items and scoring values. These questions were previously fielded during 2019 and 2022. Further information about the SDQ, is available at <http://www.sdqinfo.com>.

Table 23. Strengths and Difficulties Questionnaire subscales, items in each subscale, and point values for scoring

Subscale and Question Topic	Not true	Somewhat true	Definitely true
EMOTIONAL SUBSCALE			
Often complains of headaches, stomach-aches, or sickness	0	1	2
Many worries, often seems worried	0	1	2
Often unhappy, depressed, or tearful	0	1	2
Nervous or clingy in new situations	0	1	2
Many fears, easily scared	0	1	2
CONDUCT SUBSCALE			
Often has temper tantrums or a hot temper	0	1	2
Generally obedient, usually does what parents want	2	1	0
Often fights with other children or bullies	0	1	2
Often lies or cheats	0	1	2
Steals from home, school, or elsewhere	0	1	2
HYPERACTIVITY SUBSCALE			
Restless, overactive, cannot stay still for long	0	1	2
Constantly fidgeting or squirming	0	1	2
Easily distracted, concentration wanders	0	1	2
Thinks things out before acting	2	1	0
Sees task through to the end, good attention span	2	1	0
PEER RELATIONSHIP SUBSCALE			
Rather solitary, tends to play alone	0	1	2
Has at least one good friend	2	1	0
Generally liked by other children	2	1	0
Picked on or bullied by other children	0	1	2
Gets on better with adults than other children	0	1	2
PROSOCIAL SUBSCALE			
Considerate of other people's feelings	0	1	2
Shares readily with other children	0	1	2
Helpful if someone is hurt, upset, or feeling ill	0	1	2
Kind to younger children	0	1	2
Often volunteers to help others	0	1	2

Source: Goodman R. The Strengths and Difficulties Questionnaire: a research note. Journal of child psychology and psychiatry, and allied disciplines. 1997; 38(5):581-6. 1997.

Table 24. Strengths and Difficulties Questionnaire, impact subscale items and point values for scoring

Question Topic	Not at all	Only a little	A medium amount	A great deal
Difficulties upset or distress child	0	0	1	2
Difficulties interfere with home life	0	0	1	2
Difficulties interfere with friendships	0	0	1	2
Difficulties interfere with classroom learning	0	0	1	2
Difficulties interfere with leisure activities	0	0	1	2

Source: Goodman R. The extended version of the Strengths and Difficulties Questionnaire as a guide to child psychiatric caseness and consequent burden. *J Child Psychol Psychiatry*. 1999;40(5):791-799.

Major Recodes

SDQ subscales. Summary scores of each of the five SDQ subscales (emotional, conduct, hyperactive, peer relationships and prosocial), have been calculated and provided as a continuous variable ranging from 0-10 with higher scores indicating more problematic attributes for emotional, conduct, hyperactive, and peer relationship subscales and lower scores indicating more problematic attributes for the prosocial subscale. The five subscales can be analyzed separately to look at specific psychological problems. In order to calculate the SDQ subscales, variables with original answer codes of 1, 2, 3 in the instrument which correspond to point values of 0, 1, 2 (when question is worded negatively) or 2, 1, 0 (when question is worded positively) were summed. See Table 23 for subscales with items and point values. SDQ subscale recodes were assigned a code of 88 if more than two items needed to calculate the subscale were answered as “don’t know,” “refused,” or were not ascertained.

SDQ total. The SDQ total score is based on the sum of the first four subscales (emotional, conduct, hyperactive, and peer relationships). This recode is a continuous variable ranging from 0-40 with higher scores indicating more problematic attributes. For the SDQ total score, a code of 88 has been assigned if more than one subscale score is missing.

SDQ impact score. The SDQ impact score is based on the sum of the impact questions, which are asked only among Sample Children ages 4-17 years who had responded as having had minor, definite or severe difficulties with emotions, concentration, behavior, or being able to get along with other people. This recode is a continuous variable ranging from 0–10, with higher scores indicating that difficulties have a greater impact on the child’s and or family’s life (see Table 24). For the SDQ impact total score, a code of 88 is assigned if more than one item is missing.

Prosocial subscale. A fifth subscale that describes children’s positive behaviors. These items are excluded from the overall SDQ score when using the SDQ to identify children who have or are at increased risk of having psychological disorders.

I. Stressful Life Events

Rotating Core

Stressful life events have been associated with early and lifelong health conditions that may extend into adulthood, including diabetes, mental health disorders, and cardiovascular problems (Felitti et al., 1998). Previously fielded in 2023, 2021 and 2019, the SLE section includes four rotating core questions that asked the Sample Child respondent whether the child has witnessed or experienced neighborhood violence, lived with someone with a mental illness, lived with someone with a drug or alcohol problem, and had a parent who was incarcerated after the child was born. The questions were adapted from the Adverse Childhood Experience (ACE) questions that have previously been fielded in the [National Survey of Children's Health](#).

Given the sensitive nature of these questions, an introductory paragraph precedes the first question and informs the respondent that these events can happen in any family and that any question can be skipped. It is possible that some respondents may choose to skip some of these questions, particularly if other individuals in the household are present at the time of the interview.

V. Injuries

Sponsored Content

Traumatic Brain Injury. Traumatic Brain Injury (TBI) questions were sponsored by CDC/NCIPC in 2023–2025. Mild traumatic brain injuries are known as concussions. Concussions and TBIs are injuries caused by a bump, blow, or jolt to the head or penetrating head injury that results in disruption of normal brain function.

The TBI section began with an introductory statement that asked the Sample Child respondent to think about all head injuries, for example, from playing sports, car accidents, falls, or being hit by something or someone that occurred in the past 12 months. Two separate questions asked whether the Sample Child had experienced concussion symptoms as a result of a head injury, such as: 1) been knocked out or lost consciousness, dazed or confused, gap in memory; and 2) headaches, sensitivity to light or noise, balance problems, or changes in mood or behaviors. For children who had experienced concussion symptoms, the Sample Child respondent was asked whether the child had been evaluated for a concussion or brain injury by a health professional, whether the child's injury occurred when playing sports or participating in physical or recreational activities, and if so, whether it was while participating in organized team or league sports settings. These questions were also asked of Sample Adults. Similar questions about head injuries or concussions that happened at any time in the child's lifetime were fielded as emerging content in the 2020–2022 NHIS in the TBI section.

Health Insurance

Annual Core

The health insurance sections (INS) of the Sample Adult and Sample Child modules have a full range of items addressing health insurance such as coverage status, sources of coverage, characteristics of coverage, and reasons for no coverage. The flow and content of the questions pertaining to health insurance programs covered in the INS sections are similar to questions covered in the 1997–2018 NHIS Family Core. The main difference starting in 2019 and continuing through 2025 is that instead of asking about health insurance for all family or household members, one adult and one child (if present) are selected from each household to receive these questions. The Sample Adult and Sample Child receive a similar set of questions with a few exceptions that will be outlined below.

Health Insurance Coverage Status

An individual is considered currently insured if they currently have coverage through private health insurance, Medicare, Medicaid, Children’s Health Insurance Program (CHIP), military (TRICARE, Veterans Administration (VA), and CHAMP-VA), other state-sponsored health plans, or other government program. Individuals without any of the aforementioned types of coverages, with only Indian Health Service coverage, or a non-comprehensive plan that covers only dental, vision, or prescription drugs are considered uninsured.

For ease of analysis two recodes are available — NOTCOV_A (on the Sample Adult file) and NOTCOV_C (on the Sample Child file) — that reflect this definition of noncoverage as used in *Health, United States* (in which persons with *only* Indian Health Service coverage or a single service plan that covers only dental, vision, or prescription drugs are considered uninsured). In addition, recodes that categorize coverage into hierarchies are available to identify sources of coverage (including no coverage) among persons ages 18–64 and age 65 and older in the Sample Adult file and ages 0–17 years in the Sample child file, see sources of hierarchical coverage recodes, below. These recodes are in addition to the specific source of coverage recodes.

Sources of Coverage

Sample Adult and Sample Child respondents could identify one or more sources of medical care coverage, and single service plans were asked as separate questions. The following sources of healthcare coverage were collected in the interview:

- Private health insurance: Coverage obtained through employment or directly purchased (including Medigap plans).
- Medicare: The federal health insurance program for adults who are 65 and older, certain younger people with disabilities, and people with End-Stage Renal Disease (permanent kidney failure requiring dialysis or a transplant, sometimes called ESRD).
- Medicaid: A joint federal and state program that provides free or low-cost healthcare coverage to Americans, including some low-income people, families and children, pregnant women, the elderly, and people with disabilities.

- CHIP (Children’s Health Insurance Program): A joint federal and state program that provides low-cost health coverage to children in families that earn above the income threshold to qualify for Medicaid.
- Military: TRICARE, VA or CHAMP-VA
- Other state-sponsored health plans.
- Other government program
- Indian Health Service: A part of the federal government that delivers direct medical and public health services to federally recognized Native American Tribes and Alaska Native people.
- Single service dental plan
- Single service vision plan
- Single service prescription drug plan

Sources of Coverage Hierarchical Recodes

Source of coverage recodes use a health insurance hierarchy developed at the National Center for Health Statistics for the National Health Interview Survey. Tabulation of estimates using sources of coverage recodes have been used in Summary Health Statistics ([interactive 2019-present](#), [online tables 2013-2018](#) and [reports 1997-2012](#)), and other [NCHS publications using NHIS](#). For people under age 65, recode COVER_A is included in the Sample Adult file (age 18-64) and COVER_C is included in the Sample Child file (age 0-17). For adults age 65 and older, recode COVER65_A is included in the Sample Adult file. Two separate hierarchies by age group were developed because adults age 65 and older are often using two or more sources of coverage to pay for their healthcare. Medicare is usually the primary source of payment, and another type of coverage is a secondary source of payment for healthcare. In addition, with a significant proportion of adults age 65 and older on a Medicare Advantage plan, the age specific hierarchy takes this into account.

For people under age 65, the hierarchy has four mutually exclusive categories. The recode is coded as system missing for adults age 65 and older as it is not applicable to them. People under age 65 with more than one type of health insurance coverage were assigned to a category using the following hierarchy:

- 1) Private coverage—Includes people who had any comprehensive private insurance plan (including health maintenance organizations, preferred provider organizations, and exchange-based coverage).
- 2) Medicaid—Includes people who do not have private coverage, but who have Medicaid or other State-sponsored health plans including CHIP.
- 3) Other coverage—Includes people who do not have private insurance, Medicaid, or other public coverage, but who have any type of military coverage or Medicare. This category also includes people who are covered by other government programs.
- 4) Uninsured—Includes people who have not indicated that they are covered at the time of the interview under private health insurance, Medicare, Medicaid, CHIP, a state-sponsored health plan, other government programs, or military coverage. This category also includes people who are covered by Indian Health Service only or who only have a plan that pays for one type of service such as dental or vision care.

For adults age 65 and older (older adults), the recode has six mutually exclusive categories (COVER65_A). The recode is coded as system missing for adults under 65 years as it is not applicable to them. This hierarchy recode de-duplicates the report of private health insurance and Medicare Advantage. Older adults who report both Medicare Advantage and private coverage are placed in the Medicare Advantage category. In Addition, older adults with a report of Medicare and Medicaid, referred to as the “dual-eligible” are placed in their own category. Those with more than one type of health insurance coverage were assigned to a category using the following hierarchy.

- 1) Private coverage—Includes older adults who have both Medicare and any comprehensive private insurance plan (including health maintenance organizations, preferred provider organizations, and Medi-gap plans). This category also includes older adults with private insurance only.
- 2) Medicare and Medicaid—Includes older adults who do not have any private coverage but have Medicare and Medicaid or other state-sponsored health plans including CHIP.
- 3) Medicare Advantage—Includes older adults who have not been classified into one of the above two categories and have signed up for a Medicare Advantage Plan, sometimes called “Part C” or “MA Plans.” These plans are offered by private companies approved by Medicare. Medicare pays these companies to cover Medicare benefits. For those who join a Medicare Advantage Plan, the plan provides all of Medicare Part A (Hospital Insurance) and Medicare Part B (Medical Insurance) coverage. This is different than a Medicare Supplement Insurance (Medi-gap) policy.
- 4) Medicare only excluding Medicare Advantage—Includes older adults who receive their coverage only through Medicare and who do not have private coverage, Medicaid, or Medicare Advantage. The government pays for Medicare benefits when the person uses services.
- 5) Other coverage—Includes older adults who have not been previously classified as having private, Medicare and Medicaid, Medicare Advantage or Medicare only excluding Medicare Advantage. This category also includes older adults who have only Medicaid, other state-sponsored health plans, or CHIP, as well as people who have any type of military coverage with or without Medicare.
- 6) Uninsured—Includes older adults who have not indicated that they are covered at the time of the interview under private health insurance, Medicare, Medicaid, CHIP, a state-sponsored health plan, other government programs, or military coverage. This category also includes older adults who are covered by Indian Health Service only or who only have a plan that pays for one type of service such as dental care or vision care.

Characteristics of Coverage

For all coverage types, except for the Indian Health Service, additional follow-up questions specific to the type of coverage are asked. Some of these characteristics of coverage are broad and are relevant to more than one coverage type, whereas other characteristics are very specific to a particular type of coverage. In summary, the INS section includes detailed follow-up questions in the following areas:

- Health Insurance Marketplace, state exchanges, or Healthcare.gov as to how coverage (private, Medicaid, CHIP, state-sponsored plans, other government programs) was obtained.
- Enrollment in a high-deductible health plan (private, Medicaid, CHIP, state-sponsored plans, or other government programs).

- Premium (private, Medicaid, CHIP, state-sponsored plans, or other government programs)
- Characteristics of private insurance (up to two plans per person)
 - exchange-based
 - policyholder
 - coverage of individuals other than the policyholder
 - relationship to the policyholder (only Sample Adult)
 - how plan was obtained (e.g., work, directly purchased)
 - who pays for the plan (e.g., self or family, work)
 - annual amount of premium paid by individual or family
 - prescription drug benefit
 - dental benefit
 - vision benefit
 - health savings account
- Characteristics of Medicare
 - Medicare Parts (A, B, C, D)
 - Medicare Advantage plan or Medicare managed care
 - Dental care coverage among those with Medicare Advantage plans
 - Vision care coverage among those with Medicare Advantage plans
- Types of military healthcare
 - VA (only Sample Adult)
 - TRICARE
 - CHAMP-VA

Continuity of Coverage

For people with coverage, they were asked if there was any time in the past 12 months when they did not have coverage. If they answered “yes,” they were asked for how many months they did not have coverage. For people who were currently uninsured, they were asked when the last time was that they had coverage. If it was less than a year, they were asked for how many months they did not have coverage.

Reasons for No Coverage

There are two sets of questions concerning the reasons for not having healthcare coverage. The first set focuses on reasons for no longer being enrolled in health coverage among those individuals who currently lacked coverage for less than three years. Reasons measured included the following: policyholder retired; lost a job or changed employers; a deadline was missed for signing up or paying for coverage; became ineligible due to age or leaving school; the cost of the coverage increased; and had Medicaid or other public coverage but were no longer eligible. The second set of questions focuses on reasons for not obtaining health coverage among those individuals who currently do not have healthcare coverage. Reasons measured included the following: currently uninsured because coverage is not affordable; do not need or want coverage; not eligible for coverage; process of signing up is too difficult or confusing; cannot find a plan that meets needs; has applied for coverage but it has not started yet; and other reason. Based on coding open-ended responses, two additional categories are available as recoded variables. These additional reasons captured were retired, lost a job or changed employers, and missing a deadline for signing up for coverage.

New questions added in quarter 4 of 2025

Beginning in quarter 4 of 2025, Sample Adult and Sample Child interviews added three questions for people who indicated they had coverage and three questions for people who were uninsured for less than 12 months prior to the interview. Among those who had coverage, the questions were fielded to determine whether the respondent had any coverage in the past 12 months obtained through: 1) a current or former employer or union; 2) Healthcare.gov, the health insurance Marketplace or state-based health insurance exchange; and 3) Medicaid or CHIP. Among those who were uninsured for less than 12 months prior to the interview, the questions were fielded to determine whether the respondent had previous coverage obtained through these same three sources. Due to data quality concerns and because these questions were fielded only in the last quarter of 2025 when a lapse in appropriations impacted data collection, these six questions for Sample Adults and Sample Children are not included in the public-use file and are available through the RDC (<https://www.cdc.gov/rdc/index.html>).

Replicate

To reduce respondent burden, under certain circumstances a family who shared the same private plans were only asked about detailed characteristics of shared plans once, either in the Sample Adult interview or Sample Child interview, whichever occurred first. To be eligible for this replicate, the Sample Adult and Sample Child must be from the same family, the private plan must cover more than one person, the private plan must have a “valid” plan name (it cannot have a refused or not known as the name of the plan), and the plan has to have information as to either where the plan was obtained or who pays for it (it cannot have refused or not known to either of these fields). In addition, if either the Sample Adult or Sample Child indicated that they have Medigap coverage through HIKIND03_A or HIKIND03_C, they were not eligible for the replicate. Families may share up to two private plans. It is important to note that due to confidentiality concerns HIKIND03_C is not available on the Sample Child file. Responses to HIKIND03_C have been combined with responses to HIKIND01_C and are made available in HIKIND01R_C.

Processing Health Insurance Responses

The INS sections use responses to follow-up questions to evaluate the reliability of the reported health insurance coverage and to adjudicate conflicting information. For many survey respondents, health insurance is a complex topic and some inconsistencies in survey responses are expected. If the responses to follow-up questions are inconsistent with the original health insurance coverage indicated, the original responses are edited. As a result, a portion of the Sample Adults and Sample Children are reassigned to a different type of coverage or reclassified from insured to uninsured (or vice versa). Conversely, follow-up responses in agreement with the original health insurance response are not edited and are included in the recodes. Therefore, it is best to use the recodes created, and listed in Table 25 below, for specific types of healthcare coverage and noncoverage because of the complicated editing process that takes place in the INS sections.

Table 25. Annual core content of health insurance recoded variables for the Sample Adult and Sample Child

Type of health insurance coverage	Sample Adult file	Sample Child file
Private health plans	PRIVATE_A	PRIVATE_C
Medicare	MEDICARE_A	OTHGOVR_C*
Medicaid	MEDICAID_A	MEDICAID_C
Children's Health Insurance Program (CHIP)	CHIP_A	CHIP_C
Military health plans	MILITARY_A	MILITARY_C
Indian Health Service	IHS_A	IHS_C
Other government programs	OTHGOV_A	OTHGOVR_C*
State-sponsored health plans	OTHPUB_A	OTHPUB_C
Uninsured	NOTCOV_A	NOTCOV_C
Health insurance hierarchy, age <65	COVER_A	COVER_C
Health Insurance hierarchy 65+	COVER65_A	Not applicable

* This recode combines Sample Children covered by Medicare, other government programs or both.

Characteristics about the Sample Adult and Sample Child

Annual Core

Sex, Age, Hispanic origin, and Race

The NHIS collects information across different modules and sections that describe the Sample Adult and Sample Child's sex, age, Hispanic origin, and race. Table 26 lists the variables that summarized the final public-use variables on sex, age, Hispanic origin, and race available for them. These variables are described in the HHC section of the Codebooks.

Table 26. Annual core content on public-use variables for sex, age, Hispanic origin, and race variables

Description	Sample Adult variable	Sample Child variable
Sex	SEX_A	SEX_C
Age*	AGEP_A	AGEP_C
Hispanic origin	HISP_A	HISP_C
Hispanic group detail	HISDETP_A	HISPDETP_C
Single and multiple race groups	RACEALLP_A	RACEALLP_C
Single and multiple race groups combined with Hispanic origin	HISPALLP_A	HISPALLP_C

* During rostering, Household Respondents who refused or don't know the age of a household member are then asked a set of age range questions about the household member. The public-use data file includes information from the household respondent on whether the person selected to be the Sample Adult is under 65 years of age or age 65 or older (AGE65). In the Sample Adult and Sample Child modules, the respondent can then provide the missing age, correct it, or also decline to provide this information. AGEP_A and AGEP_C are based on the final age information.

Starting in 2019, responses of "refused" or "don't know" to the sex and age questions are allowed. To preserve confidentiality, the ages of adults over age 85 are top-coded at 85 on the Sample Adult public-use data file.

In accordance with the Office of Management and Budget's Revisions to the Standards for the Classification of Federal Data on Race and Ethnicity (OMB, 1997) for the collection of ethnicity and race in federal data systems, separate questions are asked about Hispanic origin and race. Persons of Hispanic origin may be of any race or combination of races. Hispanic origin includes persons of Mexican/Mexican American/Chicano, Central American, South American, Puerto Rican, Cuban, Dominican, or other Hispanic origin. Race is based on the Sample Adult's description of his or her own racial and ethnic identity, and an adult knowledgeable and responsible for the child's health provides this information for the Sample Child. More than one race can be reported for a Sample Adult and Sample Child.

Public-use race and Hispanic origin variables are imputed when unknown using hot deck imputation methods (Andridge and Little, 2010) where donor information was provided from either within the household or the local geographic area. Less than 1% of NHIS cases have imputed values for race or Hispanic origin. Race was not imputed for Hispanic persons who did not identify with any of the OMB race categories and thus race was coded as 'not ascertained' for these persons. Imputation was implemented to determine whether the person was of any Hispanic or Latino origin but not to determine the country or geography of origin.

The confidentiality of respondents and their families could be compromised if there were extensive details available about the Sample Adult, Sample Adult's spouse or partner, Sample Children and their parents, and other family members. Since 2019, detailed information about race and ethnicity that was previously available in public-use files is suppressed as more detailed geographic information (e.g., region and urbanization level) is included in the public-use file. The following four single-race categories are available for Sample Adult and Sample Children in the public-use files: 1) White; 2) Black or African American; 3) Asian; and 4) American Indian or Alaska Native (AIAN). The only multiple race category available in the public-use files is AIAN and another race. Sample Adult and Sample Child respondents indicating a single race other than the four mentioned or reporting more than one race, other than including AIAN, were combined into the "other single and multiple races" category. Detailed Hispanic origin information available in the public-use files includes Mexican and Mexican American only.

Estimates, as three-year averages, of selected health topics for adults by detailed race and ethnicity are available at the NHIS website (https://wwwn.cdc.gov/NHISDataQueryTool/SHS_adult3yr/index.html).

For additional information about the historical context of race and ethnicity data collection in the NHIS, including editing, references for NHIS race and Hispanic origin, and background documents related to race and ethnicity data collection in federal data systems, see Appendix II of the 2012 NHIS *Survey Description*.

Nativity

Information on place of birth (U.S. state or territory, or outside of the U.S.) and citizenship status is collected in the NAT section of the Sample Adults and Sample Child modules. To protect confidentiality, information on U.S. state or territory of birth and detailed citizenship status is not available in the public-use data files. Information about whether the Sample Adult and Sample Child was born in the U.S. and is a U.S. citizen, and for those born outside the U.S., the number of years they have been in the U.S. (categorical variable) is available in the public-use files.

Schooling

The schooling sections (SCH) of the Sample Adult and Sample Child modules contain questions to determine the number of school days missed during the past 12 months. Persons responding for Sample Children age 5–17 years, and Sample Adults age 18 and older who were enrolled in or attending school at the time of interview, were asked about the number of school days missed because of illness, injury, or disability during the 12 months prior to the interview. Persons responding for the Sample Children ages 0–17 years were asked if the child ever had a special education or early intervention plan, such as an Individualized Education Plan (IEP), or an Individualized Family Service Plan (IFSP). Those who responded "yes" were then asked if the child currently (sometime in the past school year) had a special education or early intervention plan, and if he/she received these services to help with their emotions, concentration, behavior, or mental health.

Education

Information on educational attainment, measured as the highest level of school or highest degree completed, is asked about all adults in the household during the Household Roster module. The variable EDUCP_A describes the highest educational attainment of the Sample Adult, top-coded to combine professional school degree (e.g., MD, DDS, DVM, JD) with doctoral degree (e.g., PhD, EdD). Additionally, similar recodes have been created that describe the level of educational attainment for the spouse or partner of the Sample Adult (described under

“Characteristics about the Spouse or Partner of the Sample Adult”), and the parents of the Sample Child (described under “Characteristics about the Sample Child’s Parents”). Persons responding for Sample Children are not asked about highest school grade or education the Sample Child has completed.

Employment

The Sample Adult EMP section contains information regarding the Sample Adult’s work status in the week before the interview, main reason for not working for pay, when they last worked for pay, and among those who worked for pay in the past year, usual number of hours worked, work benefits, and days of sick leave taken.

Sample Adults were first asked whether they worked for pay at a job or business last week; if not, they were asked if they had a job or business last week, but were temporarily absent due to illness, vacation, or family or maternity leave. Sample Adults who were not working last week or were not temporarily absent from a job or business last week were asked the main reason they were not working for pay at a job or business last week. Sample Adults who were unemployed, laid off, looking for work, performing seasonal or contract work, retired, unable to work for health reasons or disability, taking care of the house or family, going to school, or had some other reason, as well as refused or don’t know responses were then asked when was the last time they worked for pay at a job or business, even if only for a few days.

Employed Sample Adults — those who 1) were working last week, 2) were temporarily absent from work last week, 3) performed seasonal or contract work in the past 12 months, and 4) were working at a family-owned job or business not for pay last week — were asked about the typical total number of hours worked at all jobs or businesses, and those who refused or didn’t know how many hours they usually work or worked were asked whether they usually work 35 or more hours. Employed Sample Adults and those who were working anytime within the past 12 months were asked about paid sick leave, job-sponsored health coverage, and workdays missed due to illness or injury.

Beginning in 2021, new variable names, from those used in 2019–2020, were implemented in the annual content of the EMP section due to changes in question order and universe. The new question set asked the reason for not working for pay last week and the last time the Sample Adult worked for pay earlier in the set of questions, which then informed the universe of subsequent questions. For example, Sample Adults who performed seasonal work are now included in the question about when last worked for pay, which used to be previously skipped. Hours worked and full-time status used to be asked of those working last week or temporarily absent from work last week, and since 2021 it is also asked of those working not for pay at a family business or who had seasonal work in the past 12 months. The question about paid sick leave and job-sponsored health coverage used to exclude those who had some period of employment in the past 12 months but did not work last week and were not temporarily absent from work last week. See the current and 2020 questionnaires for more information.

Employment Recodes

Employment status. To ensure that the variables in this section are internally consistent with one another, a summary recode, EMPWRKLSW1_A, identifies all employed Sample Adults, including those who were working during the last week; those with a job or business but temporarily absent the last week; those performing seasonal or contract work in the past 12 months; and those doing unpaid work.

Hours Worked. Recode EMPWKHRS3_A provides the hours worked last week top-coded at 95 for confidentiality. Recode EMPWRKFT1_A identifies all employed Sample Adults who usually worked 35 or more hours per week. These recodes include seasonal or contract workers if they worked in the past 12 months.

Last worked. EMPLSTWOR1_A indicates the last time that Sample Adults who were not working in the last week or temporarily absent from their usual job or business had worked for pay, and Sample Adults performing unpaid work are coded as “8” or “not ascertained” on this recode.

Workdays missed. To protect confidentiality, information on the number of workdays missed due to health reasons, EMPDYSMSS3_A, is top-coded at 130 days. This recode includes seasonal/contract workers who worked in the past 12 months.

Marital Status

The MAR section contains information on the marital status of the Sample Adult. Sample Adults are first asked if they are “now married, living with a partner together as an unmarried couple, or neither.” Sample Adults who responded they are married are asked if their spouse lives in the same residence. If not living in the same residence, they are asked if this is because the Sample Adult and their spouse are legally separated. Sample Adults who are married or living with a partner are asked to verify the sex of their spouse or partner that was obtained during rostering. Sample Adults whose answer to the initial marriage and cohabitation question was other than “married” are asked if they have ever been married. Sample Adults who are currently living with a partner and have been married are asked their current legal marital status – that is, whether they are currently married, widowed, divorced, or separated. Sample Adults who are neither married nor living with a partner but have been married are asked if they are now widowed, divorced, or separated.

Additional information about the spouse or partner of the Sample Adult is described under “Characteristics about the Spouse or Partner of the Sample Adult.”

Parental Status

The MAR section also includes recodes which describe whether the Sample Adult is a parent of a child residing in the family (PARSTAT_A) and the Sample Adult’s relationship to Sample Child (SAPARENTSC_A).

Sexual Orientation

Sample Adults were asked about their sexual orientation in the ORN section. This question was asked before determining marital status of the Sample Adult and sex of the spouse or partner living in the household. An exploratory question was added since 2022 to examine what Sample Adults meant when they answered not knowing the answer or something else (other than gay, lesbian, bisexual, and straight). These verbatim responses are not available in the public-use file. Sexual orientation was not asked about the Sample Child.

Proxy Status

Generally, Sample Adults provide information for themselves during the Sample Adult interview. However, in a small number of cases, proxy responses are allowed if the Sample Adult had a physical or mental condition that prevented them from responding. Since 2024, variables PROXYFLAG_A and PROXYREL2_A replace previous indicators for the use of proxy and the relationship of the proxy to the Sample Adult respondent, respectively.

Veteran Status

The VET section contains information about the Sample Adult's military veteran status and use of Veteran Affairs (VA) services. Specifically, it includes information on whether the Sample Adult ever served in the U.S. Armed Forces, military Reserves, or National Guard, if served in active duty for training in the Reserves or National Guard, or if served in a foreign country during a time of armed conflict or on a humanitarian or peace-keeping mission, and whether has received a VA service-connected disability rating. Information about use of VA services focused on whether the Sample Adult sought care at a Veteran's Health Administration facility or receive any other health care paid for by the VA in the past 12 months, and whether they have ever enrolled in or used VA healthcare.

Characteristics about the Parents of the Sample Child

Annual Core

Starting in 2019, the PAR section includes variables and recodes describing the demographic characteristics of parents residing with the Sample Child, type of relationship (biological, adoptive, step, foster) between the child and his or her parent(s), each parent's current and legal marital status, and information on nativity (whether the parent was born in the U.S. or a U.S. territory).

Detailed information on the type of relationship between Sample Child and parent is suppressed in the public-use data due to disclosure risks. Specifically, adoptive and biological children are combined in a single category, and foster children cannot be identified. In addition, while the redesigned NHIS allows for the collection of information on up to four parents as long as they all reside in the same household as the child, only information on the first two are released in the public-use file. Instances where a Sample Child had three or four residential parents are rare. Detailed information for the parents residing with the Sample Child is available through the NCHS RDC. For a list of restricted variables, see the PAR section of the Sample Child Codebook for restricted variables.

Several recodes are available in the PAR section of the public-use data that describe the demographic characteristics of up to two parents residing with the Sample Child. The information for these recodes was obtained from questions asked in various sections of the Sample Child module (i.e., PAR, GEN, HHC). Table 27 lists the available recodes about the parent residing with the child, which include parental sex, age (bottom-coded at 20 and top-coded at 70 in 2025), education of the parent with the highest educational attainment (top-coded to combine professional school degree and doctoral degree), employment status (including full- versus part-time work), current marital status, legal marital status, and whether the Sample Child's parents are of the same sex or of opposite sex.

To protect confidentiality, detailed information about race and ethnicity of the Sample Child's resident parents is suppressed. To assist data users interested in studying interracial families, the PAR section includes four recodes to indicate (1) whether the Sample Child's race is the same as all parents in the household, (2) whether the Sample Child and all the parents in the household are of same Hispanic or Latino origin category (i.e., Hispanic, non-Hispanic), (3) whether the Sample Child's parents are of the same race to each other, and (4) whether the Sample Child's parents are of the same Hispanic or Latino origin category to each other. These recodes are "yes" and "no" answers. The "yes" response means that the Sample Child and all resident parents are in the same racial category, or the Sample Child and all resident parents are in the same Hispanic or Latino origin category. A "no" response means that either the Sample Child and at least one of the parents is of a different racial category, or that either the Sample Child and at least one of the parents are not of the same Hispanic or Latino origin category, respectively. Similarly, same race and same Hispanic or Latino origin category between parents is based on a common racial and Hispanic or Latino origin category. Same race is in reference to the racial categories available in the public-use file, that is White, Black or African American, Asian, AIAN, AIAN and another race, and all other single and multiple races. Same Hispanic or Latino origin category is based on whether the child and all parents, and whether all the parents, are of any Hispanic or Latino ethnicity (regardless of country or area of origin). Sample children residing with one parent in the household are categorized as missing (not in universe) in the variables categorizing whether the Sample Child's parents are of the same race to each other, and whether the Sample Child's parents are in the same Hispanic or Latino origin category.

Additional information about the family of the Sample Child is described under "Characteristics about the Family and Household of the Sample Adult and Sample Child."

Table 27. Annual core content of public-use recodes in the PAR section of the Sample Child codebook describing demographic characteristics of the parents residing with the Sample Child.

Description	Residential parent 1 variable	Residential parent 2 variable	Variable for both residential parents
Relationship type to Sample Child	RELCHPARENTP1_C	RELCHPARENTP2_C	
Sex	PARSEX1_C	PARSEX2_C	
Age	PARAGETC1_C	PARAGETC2_C	
Education of the Sample Child's parent with the highest education			MAXPAREDUP_C
Working last week	PARWORK1_C	PARWORK2_C	
Working full-time last week (35+ hours)	PARWKFT1_C	PARWKFT2_C	
Current marital status	MARSTAT1_C	MARSTAT2_C	
Legal marital status	LEGMSTAT1_C	LEGMSTAT2_C	
Two parents of same or of opposite sex			PARSAMEOPP_C
Sample child's Hispanic ethnicity is the same as both parents			SCPARHISP_C
Sample child's race is the same as both parents			SCPARRAC_C
Sample child's parents are of the same Hispanic ethnicity			HISPPARSC_C
Sample child's parents are of the same race			RACPARSC_C

NOTES. Parental information is from the parent(s) residing with the child at the time of interview. The Sample Child's parents are labeled as parent 1 and parent 2 based on the order that this information was provided by the respondent and entered by the interviewer during the interview for questions WHOPAR and WHOFOST. All information in the recodes about parent 1 (e.g., sex, age) are about the same person, and similarly for parent 2.

Characteristics about the Spouse or Partner of the Sample Adult

Annual Core

Starting in 2019, the MAR section includes several recodes describing the demographic characteristics of the spouse or partner living with the Sample Adult, if married or cohabiting. The information for these recodes was obtained from questions asked in various sections of the Sample Adult module (i.e., MAR, GEN, HHC). These recodes are shown in Table 28 and include the spouse or partner's sex, age (bottom-coded at 20 and top-coded at 85 years), education (top-coded to combine professional school degree and doctoral degree), current work status, and whether the spouse or partner worked full-time (35 or more hours per week).

For data users interested in studying interracial families, recodes of the race and ethnicity of the spouse or partner in the household are available in reference to the Sample Adult's race and ethnicity. These recodes are "yes" and "no" answers, where a "yes" response means that Sample Adult and the spouse or partner are in the same racial category, or the same Hispanic or Latino origin category (i.e., Hispanic, non-Hispanic), and a "no" response means that the Sample Adult and the spouse or partner are of a different racial category, or Hispanic or Latino origin category, respectively. Same race is in reference to the racial categories available in the public-use file, that is White, Black or African American, Asian, AIAN, AIAN and another race, and all other single and multiple races. Same Hispanic ethnicity is based on any Hispanic or Latino ethnicity (regardless of country or area of origin).

Additional information about the family of the Sample Adult is described under "Characteristics about the Family and Household of the Sample Adult and Sample Child."

Table 28. Annual core content of public-use recoded variables in the MAR section of the codebook describing demographic characteristics of the spouse or partner residing with the Sample Adult

Description	Variable for spouse	Variable for cohabiting partner
Sex	SPOUSESEX_A	PRTNRSEX_A
Age	SPOUSAGETC_A	PRTNRAGETC_A
Hispanic ethnicity of Sample Adult and spouse/partner are the same	SASPPHISP_A	SASPPHISP_A
Race of Sample Adult and spouse/partner are the same	SASPPRACE_A	SASPPRACE_A
Education	SPOUSEDUCP_A	PRTNREDUCP_A
Working last week	SPOUSWRK_A	PRTNRWRK_A
Working full-time last week (35+ hours)	SPOUSWKFT_A	PRTNRWKFT_A

Characteristics about the Family and Household of the Sample Adult and Sample Child

Annual Core

Family and Household Composition variable

Table 29 provides a list of various recodes included on the public-use Sample Adult and Sample Child data files that describe the families and households in which the Sample Adult and Sample Child live. Flag variables indicate source respondent for each module and whether the sample unit was a single or multiple family household. Also listed are recodes that indicate the level of education attained by the adult with the highest education in the Sample Adult's and Sample Child's family. In addition, several top-coded counters provide the number of family members in various age groups in the household.

Table 29. Annual core content of public-use recodes of family and household composition and counter variables available for Sample Adult and Sample Child

Description	Codebook section	Variable name for Sample Adult's family	Variable name for Sample Child's family
Sample Adult is the Household Respondent or the proxy who lives in the household	FLG	HHRESPSA_FLG	Not applicable
Sample child respondent is the Household Respondent	FLG	Not applicable	HHRESPSC_FLG
Number of adults in the Sample Adult's/Sample Child's family (top-coded)	FAM	PCNTADLT_A	PCNTADLT_C
Number of children in the Sample Adult's and Sample Child's family (top-coded)	FAM	PCNTKIDS_A	PCNTKIDS_C
Indicator for at least one person is age 65 and older in the Sample Adult's and Sample Child's family	FAM	OVER65FLG_A	OVER65FLG_C
Highest level of education of all the adults in the Sample Adult's and Sample Child's family (top-coded)	FAM	MAXEDUCP_A	MAXEDUCP_C
Flag indicating Sample Adult and Sample Child lives in households containing more than one family (multifamily households)	FAM	MLTFAMFLG_A	MLTFAMFLG_C
Count of persons ages 0–17 years in the Sample Adult's and Sample Child's households (top-coded)	HHC	PCNTLT18TC	PCNTLT18TC
Count of persons age 18 years and older in the Sample Adult's and Sample Child's households (top-coded)	HHC	PCNT18UPTC	PCNT18UPTC

Notes. NHIS defines a family as an individual or a group of two or more people residing together who are related by birth, marriage, or adoption. It also includes any unrelated children who are cared for by the family and any unmarried cohabiting partners and their children. Count and indicator variables include the Sample Adult, or Sample Child where applicable.

Family Employment

The family employment section (FEM) contains employment information for all related adults in the Sample Adult's and Sample Child's families. To reduce respondent burden, these questions were asked once per family. Responses to these questions were used to create recodes that list the number of persons age 18 and older who work and who work full-time (i.e., 35 or more hours per week) in the Sample Adult's and Sample Child's family. These recodes are top-coded for confidentiality. Recode names were updated in 2023 because categories were also added to indicate whether the Sample Adult or Sample Child had any family members whose work or full-time status could not be ascertained. See Table 30 for a list of family employment variables available for public-use.

Table 30. Annual core content of employment counter variables available in the FEM section of the Sample Adult or Sample Child files.

Description	Sample Adult's Family	Sample Child's Family
Number of adults who are working	PCNTADWKP1_A	PCNTADWKP1_C
Number of adults who are working full-time (35 or more hours per week)	PCNTADWFP1_A	PCNTADWFP1_C

Notes. Top-coded at 3. Categories 5-8 indicate any unknown work status or unknown full-time work status. Count variables include the Sample Adult.

Family Income

The family income section (INC) collects information regarding a variety of income sources, and total combined family income. All questions are asked once per family, using the family-level-replicate interviewing approach. Respondents are told at the start of the family income section that all questions are seeking information about possible income sources in the previous calendar year, and the names of all family members (collected earlier) to consider when responding. No personal earnings information is collected as part of the redesigned questionnaire.

Respondents were asked whether anyone in the family received income from a variety of sources (e.g., wages, salary from self-employment, social security, railroad retirement, government assistance). Respondents are also asked to report their "best estimate" of their family's total income (in dollars) from all sources for all family members living in the household before taxes in the last calendar year. Because nonresponse to this question tends to be relatively high, the NHIS includes a series of follow-up questions utilizing an unfolding bracket methodology that obtains additional income information. The unfolding bracket method asked a series of closed-ended income range questions (e.g., "is it less than \$75,000, or \$75,000 or more?") if the respondent did not provide an estimated total family income. These closed-ended income range questions were constructed so that each successive question established a smaller range for the amount of the family's income. In addition to asking respondents about the family's income relative to specific dollar values (i.e., \$75,000, \$100,000, and \$150,000), these respondents were also asked about the family's income relative to the federal poverty threshold (100%, 138%, 200%, 250% and 400%) and take into account each family's size (collected earlier in the interview).

The poverty thresholds used in the questionnaire, and shown in Table 31, are intended to approximate the U.S. Census Bureau's weighted average poverty thresholds for 2024. Because these values were not available when the 2025 NHIS instrument was created, the poverty thresholds used in the 2025 instrument were derived by NCHS from the 2023 weighted average poverty thresholds by family size, the average Consumer Price Index for

all urban consumers (CPI-U) from 2023, the forecasted annual growth rate of the CPI-U for 2024, actual monthly CPI values (all urban consumers) for January-July 2024, and projected CPI values (all urban consumers) for August-December 2024.

Table 31. Poverty thresholds used in the 2025 NHIS Instrument, by family size.

Family Size	100% of the federal poverty level	138% of the federal poverty level	200% of the federal poverty level	250% of the federal poverty level	400% of the federal poverty level
1 person < 66 years	\$17,000	\$23,000	\$34,000	\$42,000	\$68,000
1 person ≥ 66 years	\$16,000	\$22,000	\$31,000	\$39,000	\$63,000
2 persons, both < 66	\$22,000	\$30,000	\$44,000	\$55,000	\$88,000
2 persons, 1 is ≥ 66	\$20,000	\$27,000	\$39,000	\$49,000	\$79,000
3 persons	\$26,000	\$36,000	\$52,000	\$65,000	\$104,000
4 persons	\$33,000	\$46,000	\$67,000	\$84,000	\$134,000
5 persons	\$40,000	\$55,000	\$79,000	\$99,000	\$158,000
6 persons	\$45,000	\$62,000	\$90,000	\$112,000	\$179,000
7 persons	\$51,000	\$70,000	\$102,000	\$128,000	\$204,000
8 persons	\$57,000	\$78,000	\$113,000	\$142,000	\$226,000
9 or more persons	\$67,000	\$93,000	\$135,000	\$168,000	\$269,000

When the questions about income relative to poverty threshold are asked during the course of the interview, the appropriate poverty threshold relative to the family's size (in a dollar amount) is displayed on the interviewer's screen, so that the respondent is asked if the family's income in the previous year was less than the applicable poverty threshold, or if the family's income was greater than or equal to that same poverty threshold.

In 2025, 78.0% of Sample Adults and 83.3% of Sample Child respondents provided their family income. Missing family income for Sample Adults (22.0%) and Sample Children (16.7%) were imputed using family income bracket responses or other survey information. Missingness on family income is not completely at random, and excluding observations with missing income information can result in biased analyses. Reported and imputed family income information are used to create poverty ratio (see, Recodes of Family Income and Imputed Family Income, below).

To protect confidentiality, continuous and categorical family income, reported and imputed, as well as the variables obtained from the income bracketing questions, are not available on the NHIS public-use data files. See Appendix for availability of restricted income questions.

Recodes of Family Income and Imputed Family Income

Missing data on family income and earnings in the NHIS are imputed using a multiple imputation methodology. Imputation is the process of replacing missing data with substituted values based on information collected from

other observations in the dataset. Multiple imputation accounts for the extra variability due to imputation in statistical analyses.

Ten multiply imputed income data values are created for the Sample Adult and the Sample Child's families. Recent literature on multiple imputation analysis suggests that increasing the number of imputations (e.g., to 10 or higher) produces more precise estimates for a wide variety of analyses (van Buuren, 2012). The family income is imputed within the lower and upper bound when the unfolding income bracketing questions are answered.

Ten sets of top-coded continuous poverty ratios and grouped poverty ratios are available for the Sample Adult and Sample Child families. These recodes incorporate information from reported and imputed total family income and are included in the respective income files (Table 32) as a continuous ratio of total family income and family size relative to the poverty threshold and as a finite number of categories.

Table 32. Top-coded variables for family poverty ratio, and related flags available in the Sample Adult and Sample Child files

Description	Sample Adult file	Sample Child file
Top-coded poverty ratio	POVRATTC_A	POVRATTC_C
Grouped poverty ratio	RATCAT_A	RATCAT_C
Imputed income flag	IMPINCFLG_A	IMPINCFLG_C
Imputation number	IMPNUM_A	IMPNUM_C

In cases where the Sample Adult and Sample Child are in the same family, these corresponding values are identical. The poverty ratio variable is top-coded to the mean of the top 95th percentile of the 20 distinct imputations from the Sample Adult and Sample Child inhouse poverty ratio values (i.e., 10 poverty ratio values from the Sample Adult and 10 poverty ratio values from the Sample Child).

All 10 imputations are stacked in a single file with a variable (IMPNUM_A or IMPNUM_C) indicating the imputation number in each file. Stacking the 10 multiple imputation datasets into one allows for fewer steps in data preparation for analyses in SAS and Stata. (For sample code on how to use multiply imputation in analysis, see the section 'Merging Survey Data and Imputed Income Files.')

For the convenience of analyses that don't need or use multiple imputed data, the same variables — POVRATTC_A and RATCAT_A (for the Sample Adult's family) and analogously for the Sample Child's family — from a single imputation are also available in the Sample Adult and Sample Child files.

Analysts are reminded that imputed income files for each year should be merged with the relevant data files for that year before concatenating data files for multiple years.

For technical information about the imputation model and for more about the drawbacks of single imputation, please refer to the "Imputed income technical document" available with the annual file releases on the "NHIS Data, Questionnaires and Documentation" website, under "Imputed Income" (<https://www.cdc.gov/nchs/nhis/documentation/index.html>)

Food Related Programs

The food related programs (FOO) section includes questions to ascertain past 12-month participation in three food assistance programs: Supplemental Nutrition Assistance Program (SNAP), free or reduced-price breakfasts or lunches at school, and the Special Supplemental Nutrition Program for Women, Infants, and Children (WIC). Since 2023, a different variable name is used for the question about meals at school, compared to 2019–2022, due to a wording change in the question (from “reduced-cost” to “reduced-price”).

These are family-level replicate questions asked once per family. The universes for the questions in this section broadly coincide with program eligibility. Sample Adult and Sample Child respondents were asked about SNAP participation. Sample Adult and Sample Child respondents living in families with females ages 12–55 or children ages 0–5 years were asked about WIC program participation. Sample Adult and Sample Child respondents living in families with children ages 5–17 years were asked about free or reduced-price breakfasts or lunches at school.

Housing

The housing section (HOU) collects information on housing tenure, length of residence, and participation in Federal, State, or local government housing assistance programs among renters. To reduce respondent burden, these questions were asked once per family.

Region and urbanization level

Geographical classification of the U.S. population is provided on the NHIS in two ways: region and urban-rural classification. In the geographical classification, states are grouped into four regions used by the U.S. Census Bureau: Northeast (Maine, Vermont, New Hampshire, Massachusetts, Connecticut, Rhode Island, New York, New Jersey, and Pennsylvania), Midwest (Ohio, Illinois, Indiana, Michigan, Wisconsin, Minnesota, Iowa, Missouri, North Dakota, South Dakota, Kansas, and Nebraska), South (Delaware, Maryland, District of Columbia, West Virginia, Virginia, Kentucky, Tennessee, North Carolina, South Carolina, Georgia, Florida, Alabama, Mississippi, Louisiana, Oklahoma, Arkansas, and Texas), and West (Washington, Oregon, California, Nevada, New Mexico, Arizona, Idaho, Utah, Colorado, Montana, Wyoming, Alaska, and Hawaii).

Since 2024, the 2023 NCHS Urban-Rural Classification Scheme for Counties replaced the 2013 NCHS Urban-Rural Classification Scheme for Counties. It is based on OMB’s July 2023 delineations of metropolitan statistical areas (MSA) and micropolitan statistical areas (derived using the 2020 OMB standards for defining MSAs and micropolitan statistical areas) and the 2022 postcensal estimates of the resident U.S. population (NCHS, 2024). The urban-rural classification scheme groups U.S. counties and county-equivalent entities into six urban-rural categories: large central metro, large fringe metro, medium metro, small metro, micropolitan, and non-core (Ingram and Franco, 2014). The categorization included on the NHIS public-use files combine medium and small metropolitan areas into a single group and micropolitan and non-core areas into a single group to yield the following four categories: large central metro, large fringe metro, medium and small metro, and nonmetropolitan. Additional information about the NCHS Urban-Rural Classification Scheme is available at: <https://www.cdc.gov/nchs/data-analysis-tools/urban-rural.htm>.

Telephone Use

The telephone use section (TEL) contains information regarding the availability of a working non-cellular telephone (landline) and of a cellular (wireless, mobile) telephone in the home, and for adults with both types of working telephones, whether the calls received are answered “all or almost all,” “some,” or “very few or none” on the cell phone. The purpose of the telephone questions is to track the percentages of wireless-only adults and of children living in homes with only wireless telephones. The telephone use data are in the Paradata file. Variable information is found in the TEL sections (adult and child) of the Paradata codebook.

Telephone Use Recodes

Recodes that classify the telephone status of the household (HH_PLASS) and the Sample Adult (AD_PLASS) are included in the Paradata file. The response categories in the recodes correspond to the classification presented in the NCHS report “Wireless Substitution: Early Release of Estimates from the National Health Interview Survey” <https://www.cdc.gov/nchs/nhis/early-release/wireless-substitution.html>

NCHS Data Linkage Program

The Data Linkage Program in the Division of Analysis and Epidemiology (DAE) at NCHS is a cross-cutting program which aims to maximize the scientific value of the Center's population-based surveys by linking NCHS survey data with vital and other administrative records. Linked data files create new data resources that can support a wide range of studies on health care utilization and outcomes, public health, and policy evaluation to improve the health of the U.S. population.

For more information on the NCHS Data Linkage Program, please visit the NCHS Data Linkage website:
<https://www.cdc.gov/nchs/linked-data/about/index.html>

Data Sources Linked

The Data Linkage Program has linked NHIS data with:

National Death Index (NDI) death certificate data, including cause of death
(<https://www.cdc.gov/nchs/linked-data/mortality-files/index.html>)

Centers for Medicare & Medicaid Services (CMS) enrollment and claims data for:
Medicare (<https://www.cdc.gov/nchs/linked-data/medicare/index.html>)
Medicaid/CHIP (<https://www.cdc.gov/nchs/linked-data/medicaid/index.html>)

Department of Housing and Urban Development (HUD) housing assistance program data
(<https://www.cdc.gov/nchs/linked-data/hud/index.html>)

United States Renal Data System (USRDS) data on End Stage Renal Disease (ESRD)
(<https://www.cdc.gov/nchs/linked-data/esrd/index.html>)

Department of Veterans Affairs (VA) VA benefits data (<https://www.cdc.gov/nchs/linked-data/va/index.html>)

For a list of linked surveys and administrative data by year, please see
https://www.cdc.gov/nchs/data/datalinkage/LinkageTable_1.pdf.

Linkage Methodology

All linkages of data for NHIS participants with administrative data are approved by the NCHS Ethics Review Board (ERB). Only NHIS participants who meet the linkage consent criteria established by the ERB for a particular linkage and provide the personally identifiable information (PII) needed to conduct the linkage are considered *linkage-eligible*. *Linkage eligibility* refers to the potential ability to link data obtained from an NHIS participant with administrative data. *Linkage-eligibility* is distinct from program eligibility, which refers to whether a person meets eligibility criteria for a benefits program such as Medicare or HUD housing assistance. Survey participants are informed of NCHS' intent to conduct data linkage activities through a variety of procedures such as "advance letters," participant brochures, and during the interview when verbal consent is requested. As part of the consent process, NHIS participants selected to be the Sample Adult or Sample Child (by proxy respondent) are asked for the last four digits of their Social Security Number (SSN) and Medicare Beneficiary Identifier (MBI) for participants who reported Medicare coverage. Additionally, those who refused

to provide the last four digits of their SSN or MBI are asked if they would consent to linkage based on their other identifying information.

Questions to determine linkage eligibility are collected in the LNK sections of the Sample Adult and Sample Child interview, and associated variables are stored in the public-use Paradata file.

The person-level linkages are conducted using both deterministic and probabilistic techniques. PII, such as SSN, name, and date of birth, are used in the linkage algorithms. Please refer to the appropriate linkage documentation for further information on methodology and analytic considerations (for example, for the linked NDI data, <https://www.cdc.gov/nchs/media/pdfs/2026/01/lmf2022-methodology-analytic-considerations.pdf>).

Geocoded Data

Sample addresses from NHIS are also geocoded to standard Census geocoded areas. This enables researchers to merge contextual data (e.g., county-level data, air quality data) with NHIS data. The geocodes are considered restricted-use and only available through the NCHS Research Data Center (RDC) network, including Federal Statistical Research Data Centers (FSRDCs) (<https://www.cdc.gov/rdc/index.html>).

Public-Use Linked Data

Public-use Linked Mortality Files (LMFs) with a limited set of mortality variables for adult NHIS participants are available for download from the NCHS website. Statistical disclosure limitation methods were applied to the public-use versions of the LMFs to reduce the risk of participant re-identification. For select records, synthetic data were substituted for follow-up time or underlying cause of death. Information regarding vital status was not changed. For more information on the public-use LMFs, please visit the NCHS LMF webpage: https://www.cdc.gov/nchs/linked-data/mortality-files/index.html#cdc_data_surveillance_section_3-2019-public-use

Restricted-Use Linked Data

All other linked data files are restricted-use and available only through the NCHS or Federal Statistical RDCs. For more information about the restricted-use linked data, including the file contents and methods used for linkage and analytic consideration, follow the links provided for each of the following data linkages:

National Death Index (NDI), Restricted-Use Linked Mortality Data

https://www.cdc.gov/nchs/linked-data/mortality-files/index.html#cdc_data_surveillance_section_5-2022-restricted-use

NCHS-CMS Medicare

<https://www.cdc.gov/nchs/linked-data/medicare/restricted.html> and <https://www.cdc.gov/nchs/linked-data/medicare/methods.html>

NCHS-CMS Medicaid

<https://www.cdc.gov/nchs/linked-data/medicaid/index.html>

NCHS-HUD

<https://www.cdc.gov/nchs/linked-data/hud/index.html>

NCHS-USRDS ESRD

<https://www.cdc.gov/nchs/linked-data/esrd/index.html>

NCHS-VA

<https://www.cdc.gov/nchs/linked-data/va/index.html>

Geocoded data

https://www.cdc.gov/rdc/geocodes/geowt_nhis.htm

For more information about accessing the restricted-use linked data, please visit the NCHS RDC website:

<https://www.cdc.gov/rdc/>.

Medical Expenditure Panel Survey (MEPS)

NHIS interviewed households also serve as a sampling frame for the Medical Expenditure Panel Survey (MEPS). MEPS, conducted by the Agency for Healthcare Research and Quality (AHRQ), collects data on the specific health services that Americans use, how frequently they use them, the cost of these services, and how they are paid for, as well as data on the cost, scope, and breadth of health insurance held by and available to U.S. workers.

MEPS Household Component collects data from a nationally representative subsample of households that participated in the prior year's NHIS. Crosswalks that will allow data users to merge the MEPS full-year population characteristics public-use data files with the NHIS person-level public-use data files are available from AHRQ:

https://meps.ahrq.gov/mepsweb/data_stats/more_info_download_data_files.jsp#hc-nhis

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Appendix: Availability of Health, Health Insurance and Selected Demographic Questions Not Included in the Sample Adult and Sample Child Public-use Files

List of questions not available in the public-use files and availability through the Research Data Center (D), as a recode (R), or not available (NA): 2025 NHIS

Module	Section Acronym	Questionnaire Variable	Description	Availability	Public-use Recode
Roster	HHC	EDUC	Highest level of education completed	D R	EDUCP_A MAXPAREDUP_C SPOUSEDUCP_A PRTNREDUCP_A MAXEDUCP_A MAXEDUCP_C
Roster, Sample Adult, Sample Child	HHC, VFY	RACE, RACE_SP, RACE_VRBAT, NEWRACE_A, NATORG, NEWNATORG_A, HISPTYPE_A, HISPOTHER_A, HISPVRBAT_A, PITYPE_A, PIOTHER_A, PIVRBAT_A, ASIANTYPE_A, ASIANOTHER_A, ASIANVRBAT_A, RACEOTHER_A, RACEVRBAT_A, NEWNATORG_C, HISPTYPE_C, HISPOTHER_C, HISPVRBAT_C, PITYPE_C, PIOTHER_C, PIVRBAT_C, ASIANTYPE_C, ASIANOTHER_C, ASIANVRBAT_C, RACEOTHER_C, RACEVRBAT_C,	Race and Ethnicity	D R	RACEALLP_A, HISPALLP_A, HISP_A, HISDETP_A, RACEALLP_C, HISPALLP_C, HISP_C, HISDETP_C, SASPPRACE_A, SASPPHISP_A, SCPARRAC_C, RACPARSC_C, SCPARHISP_C, HISPPARSC_C
Sample Adult	ADH	ADHDAGE_A	Age diagnosed-first ADHD	D R	ADHDAGETC_A
Sample Adult	BMI	HEIGHTFT_A, HEIGHTIN_A, HEIGHTM_A, HEIGHTCM_A	Height	D R	HEIGHTTC_A, BMICAT_A, BMICATD_A
Sample Adult	BMI	WEIGHTLB_A, WEIGHTKG_A	Weight	D R	WEIGHTLBTC_A, BMICAT_A, BMICATD_A
Sample Adult	CAN	CANKIND1_A-CANKIND3_A	Kind of cancer	D R	BLADDCAN_A, BLOODCAN_A, BONECAN_A, BRAINCAN_A, BREASCAN_A, CERVICAN_A, COLONCAN_A, ESOPHCAN_A, GALLBCAN_A, LARYNCAN_A, LEUKECAN_A, LIVERCAN_A, LUNGCAN_A, LYMPHCAN_A, MELANCAN_A, MOUTHCAN_A, OVARYCAN_A, PANCRCAN_A, PROSTCAN_A, RECTUCAN_A, SKNMCMCAN_A, SKNMCMCAN_A, SKNDKCAN_A, STOMACAN_A, THROACAN_A, THYROCAN_A, UTERUCAN_A, HDNCKCAN_A, COLRCCAN_A, OTHERCANP_A
Sample Adult	CAN	CANKIND1_A-CANKIND3_A	Kind of cancer (kidney, testicular)	D	
Sample Adult	CAN	CANAGE1_A-CANAGE3_A	Age of cancer diagnosis	D R	BLADDAGETC_A, BLOODAGETC_A,

					BONEAGETC_A, BRAINAGETC_A, BREASAGETC_A, CERVAGETC_A, COLONAGETC_A, ESOPHAGETC_A, GALLBAGETC_A, LARYNAGETC_A, LEUKEAGETC_A, LIVERAGETC_A, LUNGAGETC_A, LYMPHAGETC_A, MELANAGETC_A, MOUTHAGETC_A, OVARYAGETC_A, PANCRAGETC_A, PROSTAGETC_A, RECTUAGETC_A, SKNMAGETC_A, SKNNMAGETC_A, SKNDKAGETC_A, STOMAAGETC_A, THROAAGETC_A, THYROAGETC_A, UTERUAGETC_A, HDNCKAGETC_A, COLRCAGETC_A, OTHERAGETC_A
Sample Adult	CAN	CANAGE1_A-CANAGE3_A	Age of cancer diagnosis (kidney, testicular)	D	
Sample Adult	CAN	CANMORE_A	More than three kinds of cancer	D R	NUMCAN_A
Sample Adult	DIB	DIBAGE_A	Age of diabetes diagnosis	D R	DIBAGETC_A, DIFRSTC1_A
Sample Adult	EMP	EMPHOURS_A	Hours worked per week	D R	EMPWKHRS3_A, EMPWRKFT1_A
Sample Adult	EMP	EMPFULLTIM_A	Work 35+ hours per week	D R	EMPWRKFT1_A
Sample Adult	EMP	EMPDAYMISS_A	Days missed work, past 12 months	D R	EMPDYSMSS3_A
Sample Adult	EPI	EPINUMSEIZ_A	Number of seizures in the past 12 months	D R	EPINUMSEZP_A
Sample Adult	FEM	FEMWORK_A	Employment status of other adults	R	PCNTADWKP1_A
Sample Adult	FEM	FEMWKFT_A	Other adult family member works 35+ hours per week	R	PCNTADWFP1_A
Sample Child	FEM	FEMWORK_C	Employment status of adults	R	PCNTADWKP1_C, PCNTPARWKP_C
Sample Child	FEM	FEMWKFT_C	Adult family member works 35+ hours per week	R	PCNTADWFP1_C, PCNTPARWFP_C
Sample Adult	INC	INCTOTAL_A, INC100FILL_A, INC250FILL_A, INC138FILL_A, INC75K_A, INC100K_A, INC138K_A, INC150K_A, INC250K_A, INC400K_A, INC100PCT_A, INC138PCT_A, NC200PCT_A, INC250PCT_A, INC400PCT_A	Total family income as poverty ratio	D R	POVRATTC_A, RATCAT_A
Sample Child	INC	INCTOTAL_C, INC100FILL_C, INC250FILL_C, INC75K_C, INC100K_C, INC138K_C, INC150K_C, INC250K_C, INC400K_C, INC100PCT_C, INC138PCT_C	Reported and imputed Sample Child family income	D R	POVRATTC_C, RATCAT_C

		INC200PCT_C, INC250PCT_C, INC400PCT_C			
Sample Adult	INS	HIKIND_A	Kinds of health insurance	R	MEDICARE_A, MEDICAID_A, PRIVATE_A, CHIP_A, OTHPUB_A, OTHGOV_A, MILITARY_A, IHS_A, NOTCOV_A, COVER_A, COVER65_A, HIKIND01_A, HIKIND02_A, HIKIND03_A, HIKIND04_A, HIKIND05_A, HIKIND06_A, HIKIND07_A, HIKIND08_A, HIKIND09_A, HIKIND10_A
Sample Adult	INS	HICHANGE_A	Verification of insurance coverage	NA	
Sample Adult	INS	MCANAME_A	Verbatim response to name of Medicare Advantage or Medicare HMO plan	R	MCADVR_A
Sample Adult	INS	MACHMN_A	Verbatim response to name of Medicaid managed care plan	NA	
Sample Adult	INS	PLANNAME1_A	Adult shares child's plan 1	R	PRIVATE_A
Sample Adult	INS	POLHLD1_A	Policyholder for adult who shares child's plan 1	R	POLHLD1_A, POLHLD2_A, PRPLCOV1_A, PRPLCOV2_A, PRPOLH1_A, PRPOLH2_A
Sample Adult	INS	PRPOLHP1_A	Relationship to policyholder for adult who shares child's plan 1	R	POLHLD1_A, POLHLD2_A, PRPLCOV1_A, PRPLCOV2_A, PRPOLH1_A, PRPOLH2_A
Sample Adult	INS	PLANNAME2_A	Adult shares child's plan 2	R	PRIVATE_A
Sample Adult	INS	POLHLD2_A	Policyholder for adult who shares child's plan 2	R	POLHLD1_A, POLHLD2_A, PRPLCOV1_A, PRPLCOV2_A, PRPOLH1_A, PRPOLH2_A
Sample Adult	INS	PRPOLHP2_A	Relationship to policyholder for adult who shares child's plan 2	R	POLHLD1_A, POLHLD2_A, PRPLCOV1_A, PRPLCOV2_A, PRPOLH1_A, PRPOLH2_A
Sample Adult	INS	HIPNAM1_A	Verbatim response to name of Sample Adult's first private plan	R	EXCHPR1_A
Sample Adult	INS	MORPLAN_A	Any other plans	NA	
Sample Adult	INS	HIPNAM2_A	Verbatim response to name of Sample Adult's second private plan	R	EXCHPR2_A
Sample Adult	INS	POLHLD_A	Policyholder for private plan	R	POLHLD1_A, POLHLD2_A
Sample Adult	INS	PRPLCOV_A	Plan cover others	R	PRPLCOV1_A, PRPLCOV2_A
Sample Adult	INS	PRPOLH_A	Relationship to policyholder	R	PRPOLH1_A, PRPOLH2_A
Sample Adult	INS	PLNWRK_A	How adult's plan was obtained	R	PLNWRKR1_A, PLNWRKR2_A
Sample Adult	INS	PLNWKSP_A	Verbatim response to how plan was obtained	R	PLNWRKR1_A, PLNWRKR2_A
Sample Adult	INS	PLNEXCHG_A	Plan obtained through the Marketplace	R	PLNEXCHG2_A, PLNEXCHG1_A
Sample Adult	INS	PLNPAY_A	Who pays for this plan	R	PLN1PAY1_A, PLN1PAY2_A, PLN1PAY3_A, PLN1PAY4_A, PLN1PAY5_A, PLN1PAY6_A, PLN2PAY1_A, PLN2PAY2_A, PLN2PAY3_A, PLN2PAY4_A, PLN2PAY5_A, PLN2PAY6_A
Sample Adult	INS	HICOSTN_A, HICOSTT_A	Premium amount that family or adult pays for plan	R	HICOSTR1_A, HICOSTR2_A
Sample Adult	INS	PRDEDUC_A	Plan has a deductible	R	PRDEDUC1_A, PRDEDUC2_A
Sample Adult	INS	PRHDHP_A	Annual deductible	R	PRHDHP1_A, PRHDHP2_A
Sample Adult	INS	HSAHRA_A	Health savings account	R	HSAHRA1_A, HSAHRA2_A

Sample Adult	INS	PRRXCOV_A	Plan has prescription drug coverage	R	PRRXCOV1_A, PRPXCOV2_A
Sample Adult	INS	PRDNCOV_A	Plan has dental coverage	R	PRDNCOV1_A, PRDNCOV2_A
Sample Adult	INS	PRVSCOV_A	Plan has vision coverage	R	PRVSCOV1_A, PRVSCOV2_A
Sample Adult	INS	CHNAME_A	Verbatim response to name of Sample Adults' Children's Health Insurance Program (CHIP) plan	NA	
Sample Adult	INS	OPNAME_A	Verbatim response to name of Sample Adults' state-sponsored plan	R	PLEXCHOP_A
Sample Adult	INS	OGNAME_A	Verbatim response to name of Sample Adults' other government plan	R	PLEXCHOGR_A
Sample Adult	INS	MILSPC_A	Type of military related health care	R	MILSPC1_A, MILSPC1R_A, MILSPC2_A, MILSPC3_A
Sample Adult	INS	RSNHIOHSP_A	Verbatim response to reasons for not getting coverage	R	RSNHICOST_A, RSNHIWANT_A, RSNHIELIG_A, RSNHICONF_A, RSNHIMEET_A, RSNHIWAIT_A, RSNHIOTH_A, RSNHIJOB_A, RSNHIMISS_A
Sample Adult	INS	PREPRIV1_A and PREPRIVU_A	Any time in the past 12 months, health insurance through a current or former employer or union	D	
Sample Adult	INS	PREEXCHU_A and PREEXCHI_A	Any time in the past 12 months, health insurance purchased through Healthcare.gov, the Health Insurance Marketplace, or a state-based health insurance exchange	D	
Sample Adult	INS	PREMCAIDU_A and PREMCAIDI_A	Any time in the past 12 months, had Medicaid, Medical Assistance, or the Children's Health Insurance Program	D	
Sample Child	INS	HIKIND_C	Kinds of health insurance	R	MEDICAID_C, PRIVATE_C, CHIP_C, OTHPUB_C, OTHGOVR_C, MILITARY_C, IHS_C, NOTCOV_C, COVER_C, HIKIND01R_C, HIKIND04_C, HIKIND05_C, HIKIND06_C, HIKIND07_C, HIKIND08_C, HIKIND09R_C, HIKIND10_C
Sample Child	INS	HICHANGE_C	Verification of insurance coverage	NA	
Sample Child	INS	MCANAME_C	Verbatim response to name of Medicare Advantage or Medicare HMO plan	D R	
Sample Child	INS	MCPART_C	Type of Medicare coverage	D	
Sample Child	INS	MCCHOICE_C	Enrolled in Medicare Advantage Plan	D	
Sample Child	INS	MCHMO_C	Medicare HMO	D	
Sample Child	INS	MCDNCOV_C	Dental care coverage for those enrolled in Medicare Advantage plans	D	
Sample Child	INS	MCVSCOV_C	Vision care coverage for those enrolled in Medicare Advantage plans	D	
Sample Child	INS	MCPARTD_C	Medicare Part D	D	
Sample Child	INS	MACHMN_C	Verbatim response to name of Medicaid managed care plan	NA	
Sample Child	INS	PLANNAME1_C	Child shares adults' plan 1	R	PRIVATE_C
Sample Child	INS	POLHLD1_C	Policyholder for child who shares adult's plan 1	R	POLHLD1_C, POLHLD2_C, PRPLCOV1_C, PRPLCOV2_C, PRPOLH1_C, PRPOLH2_C

Sample Child	INS	PLANNAME2_C	Child shares adults' plan 2	R	PRIVATE_C
Sample Child	INS	POLHLD2_C	Policyholder for child who shares adult's plan 2	R	POLHLD1_C, POLHLD2_C, PRPLCOV1_C, PRPLCOV2_C, PRPOLH1_C, PRPOLH2_C
Sample Child	INS	HIPNAM1_C	Verbatim response to name of Sample Child's first private plan	R	EXCHPR1_C
Sample Child	INS	MORPLAN_C	Any other plans	NA	
Sample Child	INS	HIPNAM2_C	Verbatim response to name of Sample Child's second private plan	R	EXCHPR2_C
Sample Child	INS	POLHLD_C	Policyholder for private plan	R	POLHLD1_C, POLHLD2_C
Sample Child	INS	PRPLCOV_C	Plan cover others	R	PRPLCOV1_C, PRPLCOV2_C
Sample Child	INS	PLNWRK_C	How plan was obtained	R	PLNWRKR1_C, PLNWRKR2_C
Sample Child	INS	PLNWKSP_C	Verbatim response to how plan was obtained	R	PLNWRKR1_C, PLNWRKR2_C
Sample Child	INS	PLNEXCHG_C	Plan obtained through the Marketplace	R	PLNEXCHG2_C, PLNEXCHG1_C
Sample Child	INS	PLNPAY_C	Who pays for this plan	D R	PLN1PAY1_C, PLN1PAY2_C, PLN1PAY3_C, PLN1PAY5_C, PLN1PAY6R_C, PLN2PAY1_C, PLN2PAY2_C, PLN2PAY3_C, PLN2PAY5_C, PLN2PAY6R_C
Sample Child	INS	HICOSTN_C, HICOSTT_C	Premium amount that family pays for plan	R	HICOSTR1_C, HICOSTR2_C
Sample Child	INS	PRDEDUC_C	Plan has a deductible	R	PRDEDUC1_C, PRDEDUC2_C
Sample Child	INS	PRHDHP_C	Annual deductible	R	PRHDHP1_C, PRHDHP2_C
Sample Child	INS	HSAHRA_C	Health savings account	R	HSAHRA1_C, HSAHRA2_C
Sample Child	INS	PRRXCOV_C	Plan has prescription drug coverage	R	PRRXCOV1_C, PRPXCOV2_C
Sample Child	INS	PRDNCOV_C	Plan has dental coverage	R	PRDNCOV1_C, PRDNCOV2_C
Sample Child	INS	PRVSCOV_C	Plan has vision coverage	R	PRVSCOV1_C, PRVSCOV2_C
Sample Child	INS	CHNAME_C	Verbatim response to name of Sample Child's Children's Health Insurance Program (CHIP) plan	NA	
Sample Child	INS	PREPRIVU_C and PREPRIVI_C	Any time in the past 12 months, health insurance through a current or former employer or union	D	
Sample Child	INS	PREEXCHU_C and PREEXCHI_C	Any time in the past 12 months, health insurance purchased through Healthcare.gov, the Health Insurance Marketplace, or a state-based health insurance exchange	D	
Sample Child	INS	PREMCAIDU_C and PREMCAIDI_C	Any time in the past 12 months, had Medicaid, Medical Assistance, or the Children's Health Insurance Program	D	
Sample Adult	LNG	LANGSPEC_A, LANGSPECSP_A	Language at home	D R	LANGSPECR_A
Sample Adult	MAR	SPOUSWHO_A	Person number for spouse	D	
Sample Adult	MAR	SPOUSSEX_A, SPOUNWSEX_A	Confirm spouse's sex; Correct spouse's sex	D R	SPOUSESEX_A
Sample Adult	MAR	PARTNERWHO_A	Person number for partner	D	
Sample Adult	MAR	PARTNERSEX_A, PARTNEWSEX_A	Confirm partner's sex; Correct partner's sex	D R	PRTNRSEX_A
Sample Adult	MAR	LEGALSTAT_A	Legal marital status	D	LEGMARSTAT_A

Sample Adult	MAR	WIDIVSEP_A	Widowed/Divorced/Separated	D R	MARSTAT_A
Sample Adult	NAT	CITIZEN_A, NATCTZN_A	Citizen status; How Sample Adult became U.S. citizen	D R	CITZNSTP_A
Sample Adult	NAT	NATSTBORN_A	Place of birth	D	
Sample Adult	NAT	NATUSYR_A	Year came to U.S.	D R	YRSINUS_A
Sample Child	NAT	CITIZEN_C, NATCTZN_C	Citizen status; How Sample Child became U.S. citizen	D R	CITZNSTP_C
Sample Child	NAT	NATSTBORN_C	Place of birth	D	
Sample Child	NAT	NATUSYR_C	Year came to U.S.	D R	YRSINUS_C
Sample Adult	ORN	ORIENTSP_A	Verbatim to sexual orientation	NA	
Sample Child	PAR	RELCHPAR_C	Biological/Adoptive/Step/Other to type of parent-child relationship	D R	RELCHPARENTP1_C-RELCHPARENTP21_C
Sample Child	PAR	MARITAL_C	Married/Living with partner as unmarried couple/neither to questions about marital status of Sample Child's parents	D R	MARITAL1_C-MARITAL2_C
Sample Child	PAR	SPOUSLIV_C	Spouse of first-fourth parent lives there	D	
Sample Child	PAR	SPOUSEP_C	First-fourth parents are legally separated	D	
Sample Child	PAR	SPOUSWHO_C	Person number of first-fourth residential parent's spouse	D	
Sample Child	PAR	SPOUSSEX_C; FIXSPOUSSEX_C_	Confirming sex of first-fourth parent's spouse; Correcting sex of first-fourth parent's spouse	D	
Sample Child	PAR	PARTNERWHO_C	Person number of first-fourth residential parent's unmarried partner	D	
Sample Child	PAR	PARTNERSEX_C; FIXPARTSEX_C	Confirming sex of first-fourth parent's unmarried partner; Correcting sex of first-fourth parent's unmarried partner	D	
Sample Child	PAR	EVRMARRIED_C, WIDIVSEP_C	Ever been married	D R	MARSTAT1_C-MARSTAT2_C
Sample Child	PAR	LEGALSTAT_C	Married/Widowed/Divorced/Separated to questions about legal marital status	D R	LEGMSTAT1_C-LEGMSTAT2_C
Sample Child	PAR	PARBORN_C	Sample child's first-second parent born in the US/US territory	D	
Sample Child	PAR	FOSTPAR	Sample Child currently in foster care	D	
Sample Adult	PRV	DIBA1CNUM_A	Number of A1C tests past 12 months	D R	DIBA1CNMT_A
Sample Adult	SCH	SCHDYSMSS_A	Number of school days missed, past 12 months	D R	SCHDYSMSTC_A
Sample Child	SCH	SCHDYSMSS_C	Number of school days missed, past 12 months	D R	SCHDYSMSTC_C
Sample Adult	UTZ	EMERGE12M_A	Number of times visited hospital emergency room, past 12 months	D R	EMERG12MTC_A
Sample Adult	UTZ	RETAILHC12M_A	Number of times visited retail health clinic, past 12 months	D R	RETAILHC12MTC_A
Sample Adult	UTZ	URGCC12M_A	Number of times visited urgent care, past 12 months	D R	URGCC12MTC_A
Sample Child	UTZ	EMERGE12M_C	Number of times visited hospital emergency room, past 12 months	D R	EMERG12MTC_C
Sample Child	UTZ	RETAILHC12M_C	Number of times visited retail health clinic, past 12 months	D R	RETAILHC12MTC_C
Sample Child	UTZ	URGCC12M_C	Number of times visited urgent care, past 12 months	D R	URGCC12MTC_C

NOTE: The Research Data Center (RDC) is a data enclave established to provide a mechanism whereby researchers can access detailed data files in a secure environment without jeopardizing the confidentiality of survey participants. Information about RDC access options and application procedures is available at: <https://www.cdc.gov/rdc/>.